COMPREHENSIVE MEDICAL KNOWLEDGE BASE

Complete Health Guidance for All Ages

Children • Adults • Pregnant Women • Men's Health • Women's Health

Edition v3 — 200+ Diseases — RAG Chatbot Reference

Symptoms | Medications | Home Remedies | Diet & Nutrition
Precautions | When to Go to Hospital | Recovery Time | Applicable Age Group

193	16	8	All Ages
Total Diseases	Categories	Fields Per Disease	Coverage

■■ **MEDICAL DISCLAIMER:** This document is for educational purposes and for use in AI health chatbot systems only. Information here is general health guidance and does **NOT** constitute medical advice, clinical diagnosis, or treatment. Always consult a qualified healthcare professional for any medical concern. In emergencies, call your local emergency services immediately (India: 108 / 102).

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- Morning Sickness (Nausea and Vomiting of Pregnancy) [Pregnant Women (especially 1st trimester)]
- Gestational Diabetes [Pregnant Women (usually 24-28 weeks)]
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- Iron Deficiency Anemia in Pregnancy [Pregnant Women]
- Gestational Hypertension [Pregnant Women (after 20 weeks)]
- Hyperemesis Gravidarum [Pregnant Women (1st trimester, sometimes persists)]
- Postpartum Depression [New Mothers (within first year postpartum)]
- Urinary Tract Infections in Pregnancy [Pregnant Women]
- Placenta Previa [Pregnant Women (usually 2nd-3rd trimester)]
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- Antenatal Depression [Pregnant Women]
- Round Ligament Pain in Pregnancy [Pregnant Women (typically 2nd trimester)]

Men's Health Conditions (9 diseases)

- Benign Prostatic Hyperplasia (BPH) [Men (over 50)]
- Erectile Dysfunction [Men (any age, more common 40+)]
- Prostate Cancer [Men (risk increases after 50)]
- Testicular Disorders (Torsion, Cancer, Epididymitis) [Males (all ages)]
- Male Infertility [Men of Reproductive Age]
- Kidney Stones in Men [Men (2-3 times more common than women)]
- Male Pattern Baldness (Androgenetic Alopecia) [Men (can start from late teens onward)]
- Epididymo-orchitis [Males (All Ages)]

- Hypogonadism (Low Testosterone) [Adult Men (especially over 40)]

Women's Health Conditions (11 diseases)

- Polycystic Ovary Syndrome (PCOS) [Women of reproductive age]
- Endometriosis [Women of reproductive age]
- Breast Cancer [Women (and rarely men)]
- Menopause [Women (typically 45-55 years)]
- Cervical Cancer [Women]
- Ovarian Cysts [Women of Reproductive Age]
- Fibrocystic Breast Disease [Women (20-50 years, rarely postmenopausal)]
- Vulvodynia / Chronic Pelvic Pain [Women (Any Age)]
- Premenstrual Syndrome (PMS) and PMDD [Women of Reproductive Age]
- Vulvovaginal Candidiasis (Vaginal Thrush) [Women (3 in 4 women will have at least one episode)]
- Dysmenorrhea (Painful Periods) [Women (most common gynecological condition globally)]

Mental Health Disorders (12 diseases)

- Depression (Major Depressive Disorder) [All Ages]
- Anxiety Disorder (Generalized) [All Ages]
- Panic Disorder [Adults, Teens]
- Insomnia [All Ages]
- Bipolar Disorder [Adults, Teens]
- OCD (Obsessive-Compulsive Disorder) [Adults and Children (often starts in childhood/early adulthood)]
- PTSD (Post-Traumatic Stress Disorder) [Adults and Children (after trauma exposure)]
- Eating Disorders (Anorexia and Bulimia) [Adolescents and Young Adults (predominantly women)]
- Substance Use Disorder (Alcohol Addiction) [Adults]
- Schizophrenia [Adults (typically onset late teens to early 30s)]
- Dementia (Vascular Dementia) [Elderly (usually 65+)]
- Chronic Fatigue Syndrome (ME/CFS) [Adults (predominantly women, often after infection)]

Neurological Conditions (9 diseases)

- Epilepsy [All Ages]
- Alzheimer's Disease / Dementia [Elderly (usually 65+)]
- Parkinson's Disease [Adults (usually 60+)]
- Multiple Sclerosis (MS) [Adults (usually 20-40 years, women more than men)]
- Migraine with Aura [Adults (Women more than Men)]
- Peripheral Neuropathy [Adults (common in Diabetics)]
- Bell's Palsy [Adults (All Ages)]
- Carpal Tunnel Syndrome [Adults (Women 3x more than Men, Pregnant Women, Diabetics)]
- Trigeminal Neuralgia [Adults (usually over 50, Women more than Men)]

Skin Conditions (10 diseases)

- Eczema (Atopic Dermatitis) [All Ages (most common in children)]
- Psoriasis [Adults (teens and children less common)]
- Acne Vulgaris [Teens, Young Adults (all ages possible)]
- Ringworm / Fungal Skin Infections (Tinea) [All Ages]
- Vitiligo [All Ages]
- Urticaria (Hives) [All Ages]
- Seborrheic Dermatitis (Dandruff) [All Ages (Infants as Cradle Cap, Adults Common)]
- Shingles (Herpes Zoster) [Adults (especially over 50, Immunocompromised)]
- Rosacea [Adults (30-60 years, Fair-skinned Women)]
- Alopecia Areata [All Ages]

Bone & Joint Diseases (10 diseases)

- Arthritis (Osteoarthritis) [Adults (especially 45+)]
- Rheumatoid Arthritis [Adults (women more than men)]
- Osteoporosis [Postmenopausal women, Elderly men]
- Gout [Adults (men more than women)]
- Lower Back Pain (Lumbar) [Adults (most common cause of disability worldwide)]

- Frozen Shoulder (Adhesive Capsulitis) [Adults (40-60 years, Women more than Men, Diabetics high risk)]
- Plantar Fasciitis [Adults (especially Runners, Overweight, People who Stand Long Hours)]
- Fibromyalgia [Adults (Women 7x more than Men)]
- Scoliosis [Children and Adolescents (also Adults with degenerative scoliosis)]
- Tendinitis (Tendinopathy) [Adults (Athletes and Overuse Workers)]

Eye, Ear & Nose Conditions (8 diseases)

- Conjunctivitis (Pink Eye) [All Ages]
- Cataracts [Elderly (usually 60+)]
- Glaucoma [Adults (especially over 60, High risk: family history, Diabetes, Myopia, Afro-Caribbean ethnicity)]
- Macular Degeneration (AMD) [Elderly (usually 55+)]
- Tinnitus [Adults (especially with noise exposure or hearing loss)]
- Vertigo (BPPV — Benign Paroxysmal Positional Vertigo) [Adults (most common cause of dizziness)]
- Otitis Externa (Swimmer's Ear) [All Ages (Common in Swimmers)]
- Presbyopia (Age-Related Vision Changes) [Adults (typically over 40)]

Infectious Diseases (12 diseases)

- Malaria [All Ages (high risk: children, pregnant women, travelers)]
- Dengue Fever [All Ages]
- COVID-19 [All Ages]
- Typhoid Fever [All Ages (especially children and young adults)]
- Hepatitis A [All Ages]
- HIV/AIDS [All Ages]
- Cholera [All Ages]
- Leptospirosis (Weil's Disease) [All Ages (High risk: farmers, sewer workers, flood exposure)]
- Chickenpox Prevention and Varicella Complications [Unvaccinated Children and Adults]
- Urinary Schistosomiasis (Bilharzia) [All Ages (Endemic areas: Sub-Saharan Africa, Middle East)]
- Appendicitis [All Ages (Peak: 10-30 years)]
- Herpes Simplex (Cold Sores and Genital Herpes) [All Ages]

Digestive Disorders (10 diseases)

- GERD (Gastroesophageal Reflux Disease) [Adults]
- Irritable Bowel Syndrome (IBS) [Adults]
- Peptic Ulcer Disease [Adults]
- Liver Cirrhosis [Adults]
- Inflammatory Bowel Disease (Crohn's and Colitis) [Adults (often diagnosed 15-35 years)]
- Celiac Disease [All Ages]
- Hemorrhoids (Piles) [Adults (common in Pregnancy)]
- Pancreatitis (Acute) [Adults]
- Gallstones (Cholelithiasis) [Adults (Women 4x more than Men, Obese, over 40 — 4 Fs: Fat, Forty, Female, Fertile)]
- Diverticulitis [Adults (usually over 40)]

Kidney & Urological Conditions (4 diseases)

- Kidney Stones (Nephrolithiasis) [Adults]
- Chronic Kidney Disease [Adults (especially diabetic and hypertensive)]
- Nephrotic Syndrome [Children and Adults]
- Prostatitis [Men (All Ages)]

Thyroid & Endocrine Disorders (6 diseases)

- Hypothyroidism [Adults (women 5-10x more common)]
- Hyperthyroidism / Graves Disease [Adults (women more common)]
- Addison's Disease (Adrenal Insufficiency) [Adults]
- Goiter (Enlarged Thyroid) [All Ages (Women more than Men)]
- Cushing's Syndrome [Adults (Women more than Men)]
- Pituitary Disorders (Prolactinoma) [Adults (Women more common)]

Common Illnesses

24 Conditions Covered — 8 Guidance Fields Each

Fever (Pyrexia)

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Body temperature above 100.4°F (38°C), chills, sweating, headache, muscle aches, dehydration, weakness
■ MEDICATIONS	Acetaminophen (Tylenol) 325-650mg every 4-6 hrs for adults; Ibuprofen 200-400mg every 6-8 hrs; Children: Acetaminophen or Ibuprofen weight-based dosing; Avoid Aspirin in children
■ HOME REMEDIES	Lukewarm sponge bath, stay hydrated (8-10 glasses water/day), rest, cool compress on forehead, coconut water, ginger tea with honey, tulsi leaves tea
■ DIET & NUTRITION	Light easily digestible foods, rice porridge, soups, broths, fresh fruits (citrus), avoid spicy/oily food, increase fluid intake
■■ PRECAUTIONS	Monitor temperature every 2-4 hours, keep environment cool, wear light clothing, avoid dehydration, don't bundle in heavy blankets
■ WHEN TO GO TO HOSPITAL	Temperature above 103°F (39.4°C), fever lasting more than 3 days, difficulty breathing, severe headache with stiff neck, seizures, rash, confusion, fever in infants under 3 months
■■ RECOVERY TIME	3-5 days for viral fever; bacterial fever may take longer with antibiotics

Common Cold (Rhinitis)

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Runny nose, sneezing, congestion, sore throat, mild cough, watery eyes, mild fatigue, low-grade fever
■ MEDICATIONS	Decongestants (Pseudoephedrine), Antihistamines (Cetirizine, Loratadine), Cough suppressants (Dextromethorphan), Nasal saline spray, Throat lozenges
■ HOME REMEDIES	Steam inhalation, honey-lemon-ginger tea, gargling with warm saltwater, chicken soup, turmeric milk, eucalyptus oil steam, Vitamin C foods
■ DIET & NUTRITION	Warm soups, broths, herbal teas, citrus fruits, garlic, ginger, honey, avoid dairy products and cold beverages
■■ PRECAUTIONS	Wash hands frequently, avoid touching face, cover mouth when coughing/sneezing, disinfect surfaces, avoid close contact with others

■ WHEN TO GO TO HOSPITAL	High fever above 103°F, symptoms lasting more than 10 days, difficulty breathing, chest pain, severe sinus pain, ear pain
■■ RECOVERY TIME	7-10 days typically

Influenza (Flu)

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Sudden high fever (100-104°F), severe body aches, extreme fatigue, dry cough, headache, chills, sore throat, sometimes vomiting and diarrhea
■ MEDICATIONS	Antiviral: Oseltamivir (Tamiflu) within 48 hours of symptoms; Acetaminophen or Ibuprofen for fever/pain; Rest and fluids; Annual flu vaccination recommended
■ HOME REMEDIES	Complete bed rest, plenty of fluids, steam inhalation, honey with warm water, ginger-tulsi tea, garlic soup, warm compress for body aches
■ DIET & NUTRITION	Light warm foods, vegetable soups, broths, juices, avoid solid heavy meals initially, increase Vitamin C intake
■■ PRECAUTIONS	Annual flu vaccine, frequent handwashing, avoid crowds during flu season, wear mask if infected, disinfect common surfaces
■ WHEN TO GO TO HOSPITAL	Difficulty breathing, persistent chest pain, confusion, severe vomiting, bluish lips, symptoms improving then worsening, children with extreme irritability
■■ RECOVERY TIME	1-2 weeks; fatigue may persist longer

Sore Throat (Pharyngitis)

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Pain and scratchiness in throat, pain when swallowing, swollen glands in neck, red swollen tonsils, white patches on tonsils, hoarse voice
■ MEDICATIONS	Strep throat: Amoxicillin or Penicillin antibiotics; Pain relief: Ibuprofen, Acetaminophen; Throat lozenges with benzocaine; Chlorhexidine gargles
■ HOME REMEDIES	Warm salt water gargles (1/4 tsp salt in 8oz water), honey-lemon-ginger tea, turmeric milk, steam inhalation, cold ice pops to soothe throat
■ DIET & NUTRITION	Soft foods, warm soups, ice cream, yogurt, smoothies, avoid spicy and acidic foods, warm herbal teas with honey
■■ PRECAUTIONS	Complete full antibiotic course if prescribed, avoid sharing utensils/drinks, wash hands, avoid allergens, stay hydrated
■ WHEN TO GO TO HOSPITAL	Difficulty breathing or swallowing, drooling, severe pain, high fever, stiff neck, rash, symptoms lasting more than a week

■■ RECOVERY TIME	Viral: 5-7 days; Bacterial (strep): 7-10 days with antibiotics

Cough (Acute Bronchitis)

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Persistent cough (dry or productive), mucus/phlegm production, wheezing, shortness of breath, chest tightness, fatigue, slight fever
■ MEDICATIONS	Dextromethorphan (dry cough suppressant), Guaifenesin (expectorant for wet cough), Codeine-based syrups (prescription), Bronchodilators if wheezing, Antibiotics only if bacterial
■ HOME REMEDIES	Honey (1 tsp before bed), steam inhalation with eucalyptus, ginger-honey tea, turmeric milk, licorice root tea, thyme tea, pineapple juice (bromelain)
■ DIET & NUTRITION	Warm fluids, broths, herbal teas, avoid cold drinks, dairy, and smoking, increase Vitamin C and zinc foods
■■ PRECAUTIONS	Avoid smoking and secondhand smoke, use humidifier, elevate head while sleeping, avoid allergens, cover mouth when coughing
■ WHEN TO GO TO HOSPITAL	Coughing up blood, high fever, difficulty breathing, cough lasting more than 3 weeks, wheezing, chest pain
■■ RECOVERY TIME	Acute bronchitis: 1-3 weeks

Headache (Tension & Migraine)

Category: Common Illness

■ APPLICABLE TO	Adults, Teens
■ SYMPTOMS	Tension: dull aching pain, tightness/pressure in forehead or sides of head. Migraine: throbbing pain (usually one side), nausea, vomiting, sensitivity to light and sound, aura
■ MEDICATIONS	Tension: Acetaminophen, Ibuprofen, Aspirin; Migraine: Sumatriptan (Imitrex), Rizatriptan, Ergotamines; Preventive: Propranolol, Topiramate, Amitriptyline
■ HOME REMEDIES	Cold or warm compress on forehead/neck, ginger tea, peppermint oil on temples, lavender oil aromatherapy, magnesium supplements, staying hydrated, rest in dark quiet room
■ DIET & NUTRITION	Avoid caffeine (or reduce slowly), skip trigger foods (aged cheese, red wine, processed meats, MSG), eat regular meals, stay hydrated
■■ PRECAUTIONS	Manage stress, maintain sleep schedule, avoid bright lights and loud sounds during migraine, limit screen time, take breaks from screens

■ WHEN TO GO TO HOSPITAL	Sudden severe worst headache of life, headache with fever and stiff neck, headache after head injury, headache with vision changes, weakness, or speech problems
■■ RECOVERY TIME	Tension: few hours to a day; Migraine: 4-72 hours

Stomach Ache / Gastritis

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Upper abdominal pain or burning, nausea, vomiting, bloating, belching, feeling of fullness, loss of appetite, indigestion
■ MEDICATIONS	Antacids (Tums, Maalox), H2 blockers (Famotidine/Pepcid), PPIs (Omeprazole, Pantoprazole), Anti-nausea (Ondansetron), H. pylori: Triple therapy antibiotics
■ HOME REMEDIES	Ginger tea, chamomile tea, aloe vera juice, apple cider vinegar diluted, licorice root, peppermint tea, yogurt with probiotics, small frequent meals
■ DIET & NUTRITION	Bland BRAT diet (Banana, Rice, Applesauce, Toast), avoid spicy/fatty/acidic foods, alcohol, caffeine, eat smaller portions
■■ PRECAUTIONS	Eat on time, chew food slowly, avoid NSAIDs on empty stomach, limit alcohol, avoid stress eating, don't lie down immediately after meals
■ WHEN TO GO TO HOSPITAL	Severe or worsening pain, vomiting blood, black tarry stools, significant weight loss, pain radiating to back or chest
■■ RECOVERY TIME	Acute gastritis: 2-10 days; Chronic needs ongoing management

Diarrhea

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Loose watery stools 3+ times per day, abdominal cramps, bloating, nausea, urgent need to defecate, fever, blood in stool (severe cases)
■ MEDICATIONS	ORS (Oral Rehydration Solution) - most important; Loperamide (Imodium) for adults; Zinc supplements for children; Antibiotics only if bacterial confirmed
■ HOME REMEDIES	ORS (1 liter water + 6 tsp sugar + 1/2 tsp salt), coconut water, banana, rice water, yogurt with live cultures, ginger tea, pomegranate juice
■ DIET & NUTRITION	BRAT diet (Bananas, Rice, Applesauce, Toast), clear broths, boiled potatoes, avoid dairy, fatty foods, raw vegetables, caffeine, alcohol
■■ PRECAUTIONS	Strict hand hygiene, drink only safe water, food safety practices, avoid raw/undercooked foods, prevent dehydration in children and elderly
■ WHEN TO GO TO HOSPITAL	Signs of dehydration (dry mouth, no urination, sunken eyes), blood in stool, high fever, diarrhea lasting more than 2 days in adults or 24 hours in infants

■■ RECOVERY TIME	Acute: 1-5 days with proper hydration

Vomiting / Nausea

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Nausea, forceful expulsion of stomach contents, retching, dizziness, sweating, pale skin, abdominal pain, loss of appetite
■ MEDICATIONS	Ondansetron (Zofran), Metoclopramide, Promethazine, ORS to prevent dehydration, Domperidone, Antacids if GERD-related
■ HOME REMEDIES	Ginger (tea, candy, or capsules), peppermint tea, acupressure wristbands, small sips of clear liquids, chamomile tea, lemon aromatherapy, crackers
■ DIET & NUTRITION	Clear liquids first (water, broth, diluted juice), progress to bland foods, BRAT diet, avoid strong smells, spicy/fatty foods, dairy
■■ PRECAUTIONS	Sit upright after eating, eat small frequent meals, avoid triggers, stay hydrated, avoid lying flat
■ WHEN TO GO TO HOSPITAL	Unable to keep any fluids down for 24 hours, signs of dehydration, blood in vomit, severe abdominal pain, head injury, confusion
■■ RECOVERY TIME	1-3 days for viral causes

Food Poisoning

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Nausea, vomiting, diarrhea (possibly bloody), abdominal cramps, fever, weakness, onset within hours to days of eating contaminated food
■ MEDICATIONS	ORS for hydration (primary treatment), Loperamide (avoid if bloody diarrhea), Antibiotics only for severe bacterial cases (Ciprofloxacin, Azithromycin)
■ HOME REMEDIES	ORS, coconut water, ginger-lemon tea, apple cider vinegar diluted in water, probiotics (yogurt), banana, complete rest
■ DIET & NUTRITION	Clear fluids first, then BRAT diet, gradually reintroduce normal foods, avoid dairy, fatty, spicy foods until recovered
■■ PRECAUTIONS	Cook food thoroughly, refrigerate promptly, wash hands before handling food, avoid cross-contamination, check expiry dates
■ WHEN TO GO TO HOSPITAL	High fever above 102°F, blood in stool or vomit, severe dehydration, neurological symptoms (double vision, muscle weakness), symptoms lasting more than 3 days
■■ RECOVERY TIME	1-7 days depending on cause

Constipation

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Fewer than 3 bowel movements per week, hard dry stools, straining during defecation, feeling of incomplete emptying, abdominal bloating and discomfort
■ MEDICATIONS	Bulk-forming laxatives (Psyllium/Metamucil), Stool softeners (Docusate), Osmotic laxatives (Polyethylene glycol, Lactulose), Stimulant laxatives (Bisacodyl) short-term only
■ HOME REMEDIES	Warm water with lemon in morning, prune juice, aloe vera juice, castor oil (small dose), isabgol (psyllium husk), sesame seeds, flaxseeds, warm milk with ghee
■ DIET & NUTRITION	High fiber foods (fruits, vegetables, whole grains, legumes), 8-10 glasses water daily, prunes, kiwi, papaya, avoid processed/refined foods
■■ PRECAUTIONS	Regular physical activity (30 min/day), don't ignore urge to defecate, establish regular bathroom routine, reduce stress, limit red meat and dairy
■ WHEN TO GO TO HOSPITAL	Constipation lasting more than 3 weeks, blood in stool, severe abdominal pain, significant weight loss, alternating constipation and diarrhea
■■ RECOVERY TIME	Days to weeks with lifestyle changes

Urinary Tract Infection (UTI)

Category: Common Illness

■ APPLICABLE TO	All Ages (More common in women)
■ SYMPTOMS	Burning/pain during urination, frequent urge to urinate, cloudy/dark/foul-smelling urine, pelvic pain, lower back pain, blood in urine, mild fever
■ MEDICATIONS	Antibiotics: Trimethoprim-Sulfamethoxazole, Nitrofurantoin, Ciprofloxacin (3-7 day course); Phenazopyridine for urinary pain relief; Drink plenty of water
■ HOME REMEDIES	Drink 8-10 glasses water daily, unsweetened cranberry juice, probiotics, vitamin C foods, avoid holding urine, warm compress on lower abdomen
■ DIET & NUTRITION	Lots of water, cranberry juice, vitamin C rich foods, probiotic foods, avoid caffeine, alcohol, spicy foods, citrus juice during active infection
■■ PRECAUTIONS	Wipe front to back (women), urinate after sexual intercourse, wear cotton underwear, avoid harsh soaps in genital area, stay hydrated
■ WHEN TO GO TO HOSPITAL	Fever with chills, back/flank pain (kidney involvement), nausea/vomiting, symptoms not improving after 2 days of antibiotics, UTI in men/children/pregnant women
■■ RECOVERY TIME	3-7 days with antibiotics

Dengue Fever

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Sudden high fever (102-104°F), severe headache, pain behind eyes, intense joint and muscle pain (breakbone fever), skin rash appearing after 2-5 days, mild bleeding (nose, gums), fatigue and weakness
■ MEDICATIONS	No specific antiviral; Paracetamol/Acetaminophen for fever and pain (NEVER Aspirin or Ibuprofen — increases bleeding risk); IV fluids for severe dengue; Platelet transfusion if critically low
■ HOME REMEDIES	Papaya leaf juice (widely used to raise platelets), coconut water for electrolyte replenishment, neem leaves juice, giloy (Tinospora) juice, plenty of rest, cool compress for fever
■ DIET & NUTRITION	High fluid intake (ORS, coconut water, fruit juices, soups), papaya leaf juice, kiwi, pomegranate juice; avoid spicy, oily, or fried foods; soft easily digestible meals; no alcohol
■■ PRECAUTIONS	Use DEET-based mosquito repellent, wear full-sleeve clothing, eliminate standing water around home, use mosquito nets, Aedes mosquito bites during DAYTIME — be cautious all day
■ WHEN TO GO TO HOSPITAL	Warning signs: severe abdominal pain, persistent vomiting, bleeding from any site, blood in vomit or stool, extreme fatigue, rapid breathing, platelet count below 50,000 — go immediately
■■ RECOVERY TIME	Uncomplicated dengue: 7-10 days; severe dengue may need 2-3 weeks with hospitalization

Chickenpox (Varicella)

Category: Common Illness

■ APPLICABLE TO	Children and Unvaccinated Adults
■ SYMPTOMS	Itchy blistery rash starting on face, chest, and back spreading to whole body; progression from red spots to fluid-filled blisters to scabs; fever, fatigue, headache, and loss of appetite before rash appears
■ MEDICATIONS	Calamine lotion for itching; antihistamines (Cetirizine, Diphenhydramine) for itch relief; Paracetamol for fever — NEVER Aspirin in children (Reye syndrome risk); Acyclovir antiviral for high-risk patients or severe cases
■ HOME REMEDIES	Oatmeal bath (colloidal oatmeal soothes itching), baking soda paste on rash, neem leaves water bath, keep nails trimmed short to prevent scratching and scarring, loose cotton clothing, cool environment
■ DIET & NUTRITION	Soft bland foods, ice cream or cold yogurt (soothing for mouth sores), plenty of fluids, avoid salty or acidic foods if mouth blisters present, citrus fruits for vitamin C

■■ PRECAUTIONS	Vaccinate with Varicella vaccine (2 doses); isolate infected person until ALL blisters crust over (5-7 days); keep away from pregnant women, newborns, and immunocompromised people; do not scratch blisters
■ WHEN TO GO TO HOSPITAL	High fever above 104°F, blisters near or in eyes, difficulty breathing, extreme drowsiness, signs of skin bacterial infection (increased redness, warmth, pus), stiff neck
■■ RECOVERY TIME	7-10 days; virus remains dormant and may reactivate as Shingles later in life

Typhoid Fever

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Stepwise rising fever (reaching 103-104°F by end of first week), persistent headache, relative bradycardia (slower heartbeat than expected for fever), abdominal pain, constipation or diarrhea, rose-colored spots on trunk, enlarged spleen and liver, general weakness
■ MEDICATIONS	Azithromycin (oral, mild-moderate cases); Ceftriaxone IV (severe/hospitalized cases); Ciprofloxacin (check local resistance); complete full antibiotic course of 10-14 days; Paracetamol for fever management
■ HOME REMEDIES	Complete bed rest (prevents intestinal perforation), ORS for hydration, cool compresses for fever, lemon water, giloy juice (supportive), pomegranate juice
■ DIET & NUTRITION	High-calorie easily digestible soft diet: soft-cooked rice, mashed potatoes, khichdi, banana, curd, soups, boiled vegetables; avoid raw vegetables, high-fiber foods, and spicy or fatty foods; small frequent meals; plenty of fluids
■■ PRECAUTIONS	Drink only boiled or bottled water; eat only thoroughly cooked hot food; thorough handwashing before meals; Typhoid vaccination (oral or injectable) for travelers to endemic areas; strict personal hygiene
■ WHEN TO GO TO HOSPITAL	Intestinal perforation (sudden severe abdominal pain — EMERGENCY), internal bleeding (blood in stool), uncontrolled high fever, confusion or delirium, not improving after 48 hours of antibiotics
■■ RECOVERY TIME	2-3 weeks with appropriate antibiotics; untreated typhoid carries significant complication risk

Jaundice (Viral Hepatitis)

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Yellowing of skin, eyes (sclera), and urine; dark tea-colored urine; pale/clay-colored stools; fatigue and weakness; loss of appetite; nausea and vomiting; right upper abdominal discomfort; low-grade fever; itching of skin

■ MEDICATIONS	Hepatitis A/E: Supportive care (no antiviral); Hepatitis B acute: Supportive, antiviral if severe (Tenofovir, Entecavir); Hepatitis C: Direct-acting antivirals (Sofosbuvir-based); avoid all alcohol; avoid paracetamol in high doses; avoid herbal supplements without doctor approval
■ HOME REMEDIES	Complete rest, sugar cane juice (widely used in India, easily metabolized by liver), coconut water, lemon water, amla (Indian gooseberry) juice, papaya leaves (supportive), milk thistle (silymarin) — all supportive only
■ DIET & NUTRITION	High carbohydrate low fat diet; easily digestible foods (rice, dal, khichdi, banana, apple, boiled vegetables); fresh fruit juices; strict no alcohol; avoid oily, fried, spicy foods; avoid red meat; small frequent light meals
■■ PRECAUTIONS	Safe drinking water and food hygiene (Hepatitis A, E are fecal-oral); Hepatitis A and B vaccination; do not share needles, razors, or toothbrushes; safe sexual practices; regular liver function tests
■ WHEN TO GO TO HOSPITAL	Severe vomiting unable to keep food down, confusion or drowsiness (liver failure sign), ascites (abdominal swelling), jaundice not improving after 2 weeks, very high bilirubin or liver enzymes
■■ RECOVERY TIME	Hepatitis A: 4-8 weeks; Hepatitis B acute: up to 6 months; Hepatitis C: curable with 8-12 week antiviral treatment

Malaria

Category: Common Illness

■ APPLICABLE TO	All Ages (High risk: Children, Pregnant Women, Elderly)
■ SYMPTOMS	Classic cyclical pattern: cold/shivering phase → high fever phase → sweating and defervescence; headache, muscle aches, fatigue, nausea, vomiting, enlarged spleen; in severe malaria (P. falciparum): confusion, seizures, organ failure, dark urine (blackwater fever), severe anemia
■ MEDICATIONS	Artemisinin-based Combination Therapy (ACT) — gold standard: Artemether-Lumefantrine (Coartem) or Artesunate-Amodiaquine; IV Artesunate for severe malaria; P. vivax: add Primaquine (G6PD test first); Paracetamol for fever; Prevention: Hydroxychloroquine, Doxycycline
■ HOME REMEDIES	Supportive: Paracetamol for fever (NOT Aspirin), ORS for hydration, rest, mosquito net use, neem and papaya leaf juice (traditional supportive); NOTE: must use proper antimalarial medications — home remedies alone are not sufficient
■ DIET & NUTRITION	High calorie protein-rich diet to fight infection; plenty of fluids and ORS; iron-rich foods for anemia recovery; easily digestible foods; fresh fruits; avoid alcohol; regular small meals
■■ PRECAUTIONS	DEET mosquito repellent (apply on exposed skin), permethrin-treated bed nets, long-sleeve clothing especially at dusk, eliminate standing water (mosquito breeding sites), chemoprophylaxis for travelers, window screens

■ WHEN TO GO TO HOSPITAL	All suspected malaria — confirm with rapid diagnostic test (RDT) or blood smear; URGENT if: confusion, seizures, inability to take oral medications, very high fever, breathing difficulty, jaundice, dark urine
■■ RECOVERY TIME	Uncomplicated malaria: 3-7 days with ACT; P. vivax/ovale may relapse without Primaquine; severe malaria: weeks in hospital

Scabies

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Intense itching (worse at night), small red bumps or blisters, thin wavy burrow lines on skin (often between fingers, wrists, elbows, beltline, genitals), crusty Norwegian scabies (immunocompromised), secondary bacterial infection from scratching
■ MEDICATIONS	Permethrin 5% cream (first-line, apply from neck down, leave 8-14 hours, repeat in 1 week); Ivermectin oral (for crusted scabies or failure of topical); Antihistamines for itch relief; Antibiotics if secondary bacterial infection; treat ALL household members simultaneously
■ HOME REMEDIES	Tea tree oil diluted in carrier oil (anti-parasitic properties), neem oil and neem soap, turmeric paste with neem leaves, clove oil (diluted), aloe vera for soothing; NOTE: medical treatment is essential alongside remedies
■ DIET & NUTRITION	No specific dietary restrictions; focus on immune-boosting foods (vitamin C, zinc, probiotics) to prevent secondary infections; good hydration
■■ PRECAUTIONS	Treat ALL household members and sexual contacts at the same time; wash all clothing, bedding, towels in hot water (60°C) and dry on high heat; vacuum mattresses; items that can't be washed: seal in plastic bag for 72 hours; avoid close physical contact until treated
■ WHEN TO GO TO HOSPITAL	Crusted (Norwegian) scabies (widespread thick crusts — highly contagious), secondary skin infection (cellulitis), failure of home treatment, immunocompromised patients
■■ RECOVERY TIME	Itching may persist for 2-4 weeks after successful treatment (immune reaction); re-treat if new burrows appear

Heat Stroke / Heat Exhaustion

Category: Common Illness

■ APPLICABLE TO	All Ages (High risk: Elderly, Athletes, Outdoor Workers)
■ SYMPTOMS	Heat Exhaustion: heavy sweating, cold pale clammy skin, weak rapid pulse, nausea, dizziness, fainting, muscle cramps, headache. Heat Stroke: high body temp above 104°F, hot dry skin (no sweating), rapid strong pulse, confusion, slurred speech, loss of consciousness — medical emergency

■ MEDICATIONS	Heat Exhaustion: move to cool place, cool fluids, rest, ORS; Heat Stroke: EMERGENCY — rapid cooling (ice packs on neck/armpits/groin, cold water immersion), IV fluids in hospital, Benzodiazepines if seizures, treat organ failure in ICU
■ HOME REMEDIES	Move immediately to cool shaded area, cool wet cloths on skin, fan while misting with water, coconut water, ORS, cold compress on neck and armpits, elevate legs (if no head injury), remove excess clothing
■ DIET & NUTRITION	ORS to replace fluids and electrolytes, coconut water, cool water, sports drinks (diluted), avoid alcohol and caffeine, light easily digestible foods, watermelon and cucumber (high water content)
■■ PRECAUTIONS	Avoid outdoor activity in peak heat (11am-4pm), wear light loose breathable clothing, stay hydrated before/during/after outdoor activity, acclimatize gradually to heat, never leave children or pets in parked cars, check on elderly neighbors in heatwaves
■ WHEN TO GO TO HOSPITAL	Heat Stroke symptoms (confusion, hot dry skin, loss of consciousness) — CALL EMERGENCY IMMEDIATELY; core temperature above 104°F; not improving with cooling; seizures; organ failure signs
■■ RECOVERY TIME	Heat exhaustion: 24-48 hours with rest and fluids; Heat stroke: days to weeks in hospital; some lasting neurological damage possible

Dehydration

Category: Common Illness

■ APPLICABLE TO	All Ages (High risk: Infants, Elderly)
■ SYMPTOMS	Mild: thirst, dry mouth, darker urine, decreased urination. Moderate: headache, fatigue, dizziness, dry skin, muscle cramps. Severe: very dry mouth, sunken eyes, no urination for 8+ hours, rapid heartbeat, low blood pressure, confusion, lethargy — especially dangerous in infants (sunken fontanelle)
■ MEDICATIONS	Mild-Moderate: ORS (Oral Rehydration Solution) — most effective; Sports drinks (diluted); Plain water for adults. Severe: IV fluid replacement (Normal Saline or Ringer's Lactate in hospital); Treat underlying cause (diarrhea, vomiting, fever)
■ HOME REMEDIES	WHO-ORS recipe: 1 liter safe water + 6 teaspoons sugar + 1/2 teaspoon salt; coconut water (natural electrolytes); rice water; barley water; banana (potassium); watermelon; cucumber; small frequent sips if vomiting present
■ DIET & NUTRITION	Water-rich foods (cucumber, watermelon, oranges, strawberries, soup), ORS as primary treatment, coconut water, avoid caffeine and alcohol (diuretics), avoid high-sugar drinks (worsen diarrheal dehydration)
■■ PRECAUTIONS	Drink at least 8 glasses water daily, increase intake in hot weather/exercise/illness, monitor urine color (pale yellow = adequate), increase fluid intake at first signs of illness, elderly and infants need proactive hydration
■ WHEN TO GO TO HOSPITAL	Infant with no wet diaper for 6 hours, sunken eyes, no tears when crying, extreme lethargy; Adult with no urination for 12+ hours, rapid heart rate, confusion, inability to keep fluids down

■■ RECOVERY TIME	Mild-moderate: 24-48 hours with ORS; Severe: hours to days with IV fluids

Allergic Reaction (Mild to Moderate)

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Skin: hives (raised itchy welts), redness, itching, swelling. Eyes: itchy watery red eyes. Nose: runny nose, sneezing. GI: nausea, vomiting, abdominal cramps. Respiratory: mild wheezing. Triggered by foods, medications, insect stings, latex, environmental allergens
■ MEDICATIONS	Antihistamines: Cetirizine, Loratadine (non-drowsy), Diphenhydramine (Benadryl — quick but drowsy); Topical hydrocortisone cream; Calamine lotion for skin; Corticosteroids (Prednisolone) for moderate reactions; Epinephrine auto-injector (EpiPen) for severe anaphylaxis
■ HOME REMEDIES	Cool compress on hive-affected skin, oatmeal bath, aloe vera gel, avoid trigger substance, antihistamine immediately, baking soda paste on skin rash, chamomile (topical or tea)
■ DIET & NUTRITION	Identify and strictly avoid food allergens (common: peanuts, tree nuts, shellfish, fish, milk, eggs, wheat, soy), read food labels carefully, communicate allergies at restaurants, keep epinephrine at hand
■■ PRECAUTIONS	Carry EpiPen if history of anaphylaxis, medical alert bracelet, allergy testing to identify triggers, inform school/workplace, know the difference between allergy and anaphylaxis, always have antihistamines available
■ WHEN TO GO TO HOSPITAL	Anaphylaxis signs: throat tightening, difficulty breathing, rapid pulse, drop in blood pressure, loss of consciousness — USE EPIPEN AND CALL EMERGENCY IMMEDIATELY; swelling of face/throat
■■ RECOVERY TIME	Mild allergic reaction: hours with antihistamines; anaphylaxis: requires hospital monitoring for 4-6 hours after epinephrine

Nosebleed (Epistaxis)

Category: Common Illness

■ APPLICABLE TO	All Ages (Common in Children 3-10 years and Elderly)
■ SYMPTOMS	Bleeding from one or both nostrils; anterior nosebleeds (most common — from front of nose) are easily managed at home; posterior nosebleeds (from deeper in nose) are harder to control and occur more in elderly; may also bleed into throat; can be triggered by dry air, nose picking, trauma, blowing nose hard, high BP, blood thinners
■ MEDICATIONS	First aid measures (primary treatment); Oxymetazoline nasal spray (decongestant to constrict vessels); Nasal packing (medical procedure for persistent bleeding); Cauterization (silver nitrate or electrocautery for recurrent anterior bleeds); Treat underlying cause (hypertension, blood thinners review)

■ HOME REMEDIES	Lean FORWARD (not back — prevents swallowing blood), pinch soft part of nose for 10-15 continuous minutes, breathe through mouth, ice pack on nose bridge, cold compress on back of neck, remain calm (anxiety worsens bleeding), petroleum jelly inside nostril after bleeding stops (prevents recurrence in dry weather)
■ DIET & NUTRITION	Vitamin K rich foods for clotting (leafy greens), vitamin C for blood vessel integrity, bioflavonoids (berries, citrus), stay well hydrated, humidify dry indoor air, avoid aspirin if recurrent bleeds
■■ PRECAUTIONS	Humidify bedroom air in dry seasons, saline nasal spray in dry climates, avoid nose picking and forceful blowing, control blood pressure, review blood thinning medications with doctor
■ WHEN TO GO TO HOSPITAL	Bleeding not stopping after 20-30 minutes of direct pressure, very heavy bleeding, blood thinners or clotting disorder, frequent recurrent nosebleeds (3+ per week), posterior nosebleed (blood down throat), nosebleed after head injury
■■ RECOVERY TIME	Most anterior nosebleeds stop within 15-20 minutes; avoid strenuous activity, hot liquids, and nose blowing for 24 hours after

Eye Strain (Digital Eye Strain / Computer Vision Syndrome)

Category: Common Illness

■ APPLICABLE TO	Adults and Children (Screen Users)
■ SYMPTOMS	Eyestrain and tired eyes, headache (frontal/temporal), blurred or double vision, dry or watery eyes, eye redness, burning or stinging sensation, neck and shoulder pain, difficulty refocusing between near and far, increased sensitivity to light — all worsened by prolonged screen use
■ MEDICATIONS	Artificial tears (lubricating eye drops — preservative-free preferred for frequent use); Antihistamine drops if allergic component; Prescription glasses for computer use if needed; Blue light filtering glasses (evidence mixed but many find helpful); Treat underlying dry eye disease
■ HOME REMEDIES	20-20-20 rule: every 20 minutes look at something 20 feet away for 20 seconds; blink consciously and frequently; palming (cover eyes with warm palms); cold cucumber slices on eyes; warm compress for dry eyes; position screen at arm's length and slightly below eye level
■ DIET & NUTRITION	Vitamin A and beta-carotene (carrots, sweet potato, leafy greens), omega-3 fatty acids (reduces dry eyes), lutein and zeaxanthin (kale, eggs, corn), stay well hydrated (reduces dry eye symptoms), vitamin C and E antioxidants
■■ PRECAUTIONS	20-20-20 rule strictly, adjust screen brightness to match environment, reduce screen glare (anti-glare screen protectors), increase text size to reduce squinting, take regular breaks, blink reminder apps, optimize room lighting to reduce contrast with screen
■ WHEN TO GO TO HOSPITAL	Persistent blurred vision after rest, sudden vision changes, eye pain (not strain), double vision, red eye with discharge, severe headaches

■■ RECOVERY TIME	Symptoms resolve with rest from screens; chronic if screen use habits not changed; proper ergonomics prevent recurrence

Mouth Ulcers (Aphthous Stomatitis)

Category: Common Illness

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Painful round or oval sores inside mouth (inner lips, cheeks, tongue base, soft palate); white or yellow center with red border; intense pain especially while eating, drinking, or talking; single or multiple ulcers; usually heal within 1-2 weeks; recurrent aphthous stomatitis (RAS) is chronic recurring form
■ MEDICATIONS	Topical anesthetics: Benzocaine gel, Lidocaine mouthwash; Topical corticosteroids: Triamcinolone paste (Kenalog in Orabase), Beclomethasone inhaler sprayed on ulcer; Chlorhexidine mouthwash (antiseptic); Cauterizing agents (Silver nitrate — done by dentist); Vitamin B12 supplementation for recurrent ulcers
■ HOME REMEDIES	Salt water rinse (1/2 tsp salt in 1 cup warm water — gargle 3x daily), honey directly applied (antibacterial and soothing), coconut oil pulling, aloe vera gel applied topically, turmeric paste with honey, ice chips to numb pain, avoid acidic or spicy foods
■ DIET & NUTRITION	Avoid acidic foods (citrus, tomato, vinegar), spicy foods, hard crunchy foods (potato chips — trauma); soft foods during acute phase; vitamin B12 foods (eggs, dairy, meat — deficiency causes recurrent ulcers); iron and folate rich foods; zinc foods (pumpkin seeds, oysters)
■■ PRECAUTIONS	Avoid known triggers (stress, citrus fruits, sodium lauryl sulfate toothpaste — switch to SLS-free), soft-bristled toothbrush, manage stress (major trigger), rule out nutritional deficiencies and systemic disease (IBD, celiac disease) if recurrent
■ WHEN TO GO TO HOSPITAL	Ulcer not healing after 3 weeks (rule out oral cancer), large ulcers above 1cm, fever with ulcers, difficulty swallowing or breathing, ulcers in patient with immunocompromising conditions
■■ RECOVERY TIME	Minor aphthous ulcers: 7-14 days spontaneously; with treatment: 5-10 days; major aphthous ulcers: up to 6 weeks

Respiratory Diseases

14 Conditions Covered — 8 Guidance Fields Each

Asthma

Category: Respiratory

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Wheezing, shortness of breath, chest tightness, cough (especially at night or early morning), breathing difficulty triggered by allergens, exercise, cold air, or stress
■ MEDICATIONS	Rescue inhalers: Salbutamol (Albuterol) for acute attacks; Controller: Inhaled corticosteroids (Fluticasone, Budesonide), Long-acting beta-agonists (Salmeterol), Leukotriene modifiers (Montelukast), Theophylline
■ HOME REMEDIES	Breathing exercises (Buteyko method), yoga and pranayama, steam inhalation, ginger tea, honey with black pepper, avoid known triggers, clean air filters
■ DIET & NUTRITION	Anti-inflammatory foods (omega-3 rich fish, leafy greens), avoid sulfite-containing foods (wine, dried fruits), vitamin D and magnesium rich foods, avoid cold foods
■■ PRECAUTIONS	Identify and avoid triggers, use HEPA air purifiers, keep inhaler always accessible, no smoking, regular medical checkups, asthma action plan
■ WHEN TO GO TO HOSPITAL	Severe breathlessness at rest, rescue inhaler not helping, bluish lips/fingernails, peak flow below 50% of best, rapid breathing with chest retractions
■■ RECOVERY TIME	Chronic condition - manageable with medication; acute attacks resolve in minutes to hours with treatment

Pneumonia

Category: Respiratory

■ APPLICABLE TO	All Ages (High risk: elderly, children, immunocompromised)
■ SYMPTOMS	High fever with chills, productive cough (yellow/green/rusty mucus), chest pain (worse with breathing), shortness of breath, rapid breathing, fatigue, loss of appetite, confusion (elderly)
■ MEDICATIONS	Bacterial: Amoxicillin, Azithromycin, Doxycycline, Fluoroquinolones (Levofloxacin); Viral: Supportive care; Hospitalized: IV antibiotics; Fever: Acetaminophen/Ibuprofen; Oxygen therapy if needed
■ HOME REMEDIES	Complete bed rest, stay well hydrated, steam inhalation, warm soup, honey-ginger tea, deep breathing exercises (if tolerated), elevate head while sleeping
■ DIET & NUTRITION	Protein-rich foods for recovery, warm soups, broths, fresh fruits and vegetables, plenty of fluids, avoid solid heavy foods during acute phase

■■ PRECAUTIONS	Pneumonia vaccine (PCV13, PPSV23), annual flu vaccine, no smoking, good hygiene, avoid people with respiratory infections
■ WHEN TO GO TO HOSPITAL	All pneumonia cases should be evaluated by doctor; Emergency: oxygen saturation below 92%, confusion, very high fever, inability to eat/drink, rapid progression
■■ RECOVERY TIME	Mild: 1-3 weeks; Severe/hospitalized: 4-6 weeks or longer

Tuberculosis (TB)

Category: Respiratory

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Persistent cough lasting 3+ weeks, coughing blood, chest pain, weight loss, night sweats, fatigue, low-grade fever, loss of appetite
■ MEDICATIONS	DOTS Therapy (Directly Observed Treatment): Isoniazid + Rifampicin + Pyrazinamide + Ethambutol for 2 months; then Isoniazid + Rifampicin for 4 months; Total 6-month course (never stop early)
■ HOME REMEDIES	Supportive: Adequate nutrition, rest, sunlight exposure (Vitamin D), garlic (adjunct), avoid spreading to others
■ DIET & NUTRITION	High protein diet, calorie-dense foods, Vitamin D and calcium rich foods, fresh vegetables, avoid alcohol
■■ PRECAUTIONS	Complete full 6-month treatment, cover mouth when coughing, wear N95 mask, ventilate living space, family members should get tested, BCG vaccination for children
■ WHEN TO GO TO HOSPITAL	All suspected TB cases need medical evaluation; blood in sputum, breathing difficulty, drug side effects (yellow eyes, skin rash)
■■ RECOVERY TIME	6 months minimum for drug-sensitive TB; MDR-TB: 18-24 months

COPD (Chronic Obstructive Pulmonary Disease)

Category: Respiratory

■ APPLICABLE TO	Adults (usually 40+)
■ SYMPTOMS	Chronic cough with mucus, progressive shortness of breath, wheezing, chest tightness, frequent respiratory infections, fatigue, weight loss in severe cases, barrel chest
■ MEDICATIONS	Bronchodilators: Tiotropium (LAMA), Salmeterol (LABA); ICS: Fluticasone; Combination inhalers; Roflumilast (severe); Oxygen therapy (severe); Pulmonary rehabilitation
■ HOME REMEDIES	Pursed-lip breathing exercises, diaphragmatic breathing, stay hydrated, steam inhalation, avoid smoke and dust, light physical activity as tolerated

■ DIET & NUTRITION	Small frequent meals (large meals worsen breathlessness), high protein and calorie foods, antioxidant-rich fruits and vegetables, avoid gassy foods (bloating worsens breathing)
■■ PRECAUTIONS	Stop smoking (most important!), avoid air pollution, annual flu vaccine, pneumonia vaccine, pulmonary rehabilitation program, regular follow-ups
■ WHEN TO GO TO HOSPITAL	Severe breathlessness, oxygen levels below 88%, confusion, bluish lips, fever with increased cough and sputum (exacerbation)
■■ RECOVERY TIME	Chronic, progressive - no cure but symptoms managed with treatment

Sinusitis

Category: Respiratory

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Facial pain/pressure (forehead, cheeks, around eyes), nasal congestion, thick yellow-green nasal discharge, reduced sense of smell, headache, cough, fatigue, tooth pain
■ MEDICATIONS	Nasal saline irrigation, Nasal corticosteroids (Fluticasone, Budesonide), Decongestants (Pseudoephedrine), Antihistamines if allergic, Antibiotics (Amoxicillin-Clavulanate) only for bacterial cases
■ HOME REMEDIES	Steam inhalation with eucalyptus oil, Neti pot saline rinse, warm compress on face, ginger-turmeric tea, apple cider vinegar steam, hydration
■ DIET & NUTRITION	Anti-inflammatory foods, broths, warm fluids, spicy foods (capsaicin helps drainage), vitamin C rich foods, avoid dairy and cold foods
■■ PRECAUTIONS	Treat allergies promptly, use humidifier, manage allergies, avoid smoke, treat nasal polyps, nasal irrigation regularly during allergy season
■ WHEN TO GO TO HOSPITAL	Severe headache or facial pain, high fever, swelling around eyes, stiff neck, vision changes, symptoms not improving after 10 days
■■ RECOVERY TIME	Acute: 4 weeks; Chronic sinusitis: 12+ weeks management needed

Bronchitis (Chronic)

Category: Respiratory

■ APPLICABLE TO	Adults
■ SYMPTOMS	Persistent productive cough with mucus for at least 3 months, recurring 2+ years, wheezing, shortness of breath, chest discomfort, fatigue
■ MEDICATIONS	Bronchodilators (Salbutamol, Ipratropium), Inhaled corticosteroids, Mucolytics (N-Acetylcysteine, Ambroxol), Antibiotics during bacterial flares, Annual flu vaccine

■ HOME REMEDIES	Steam inhalation, honey-ginger-lemon tea, eucalyptus oil inhalation, stay hydrated, postural drainage, deep breathing exercises
■ DIET & NUTRITION	Antioxidant-rich foods, omega-3 fatty acids, avoid dairy (increases mucus), warm fluids, avoid smoking and alcohol
■■ PRECAUTIONS	Quit smoking immediately, avoid air pollutants, annual vaccinations, avoid respiratory infections, regular pulmonary function tests
■ WHEN TO GO TO HOSPITAL	Severe breathlessness, blood in sputum, high fever, rapid deterioration, oxygen saturation dropping
■■ RECOVERY TIME	Chronic condition requiring ongoing management

Sleep Apnea

Category: Respiratory

■ APPLICABLE TO	Adults
■ SYMPTOMS	Loud snoring, episodes of stopped breathing during sleep (reported by partner), gasping for air during sleep, morning headache, excessive daytime sleepiness, difficulty concentrating, irritability
■ MEDICATIONS	CPAP (primary treatment), BiPAP, Oral appliances, Weight loss, Positional therapy, Surgery (uvulopalatopharyngoplasty) in select cases, Avoid sedatives/alcohol
■ HOME REMEDIES	Sleep on side (not back), elevate head of bed, weight loss if overweight, avoid alcohol before bed, nasal strips, throat exercises (myofunctional therapy)
■ DIET & NUTRITION	Reduce body weight, anti-inflammatory diet, avoid alcohol especially at night, avoid heavy meals before bed
■■ PRECAUTIONS	Regular use of CPAP, annual follow-up with sleep specialist, treat nasal congestion, maintain healthy weight, avoid sedative medications
■ WHEN TO GO TO HOSPITAL	Observed episodes of stopped breathing, extreme daytime sleepiness affecting daily function, morning headaches, suspected cardiac complications
■■ RECOVERY TIME	Managed lifelong with CPAP; symptoms improve significantly with treatment

Pleuritis (Pleurisy)

Category: Respiratory

■ APPLICABLE TO	Adults
■ SYMPTOMS	Sharp stabbing chest pain that worsens with breathing, coughing, or sneezing; pain may radiate to shoulder; shallow breathing to avoid pain; sometimes fever; pleural friction rub heard with stethoscope

■ MEDICATIONS	NSAIDs (Ibuprofen, Naproxen) — first line for pain and inflammation; Colchicine (recurrent pleurisy); treat underlying cause (antibiotics for pneumonia/TB, anticoagulants if pulmonary embolism); Paracetamol for fever
■ HOME REMEDIES	Rest on the affected side (reduces pain by restricting movement), warm compress on chest, pillows pressed against chest while coughing, splinting the chest, slow deep breathing exercises
■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids, adequate hydration, vitamin C rich foods for immune support, light easily digestible meals during acute phase
■■ PRECAUTIONS	Identify and treat underlying cause, complete antibiotic course if prescribed, monitor for pleural effusion development, avoid strenuous activity during acute phase, follow up with chest X-ray
■ WHEN TO GO TO HOSPITAL	Severe chest pain, difficulty breathing at rest, coughing up blood, high fever with pleurisy (empyema), symptoms not improving in 48 hours, large pleural effusion
■■ RECOVERY TIME	Viral pleurisy: 1-2 weeks; bacterial: weeks with antibiotics; depends heavily on underlying cause

Pulmonary Embolism (PE)

Category: Respiratory

■ APPLICABLE TO	Adults (especially post-surgery, immobile, or with clotting disorders)
■ SYMPTOMS	Sudden shortness of breath, sharp chest pain (worsens with deep breath), rapid heart rate, coughing (may produce blood), lightheadedness or fainting, leg pain and swelling (DVT), blue-tinged lips or fingernails, sense of impending doom
■ MEDICATIONS	EMERGENCY ANTICOAGULATION: Heparin IV (immediate), Low Molecular Weight Heparin (Enoxaparin), Oral anticoagulants (Rivaroxaban, Apixaban, Warfarin for long-term); Thrombolytics (tPA) for massive PE with shock; Embolectomy surgery for massive PE
■ HOME REMEDIES	NO home treatment — this is a life-threatening emergency. CALL EMERGENCY IMMEDIATELY. Prevention: calf exercises during long travel, compression stockings, stay hydrated on long flights, walk regularly
■ DIET & NUTRITION	Stable Vitamin K intake for Warfarin users (consistent leafy greens), anti-inflammatory diet, omega-3 fatty acids (anti-clotting), adequate hydration to reduce blood viscosity
■■ PRECAUTIONS	Compression stockings for high-risk individuals, early mobilization after surgery, anticoagulation prophylaxis post-surgery, stay hydrated during long travel, move legs during long flights
■ WHEN TO GO TO HOSPITAL	IMMEDIATELY — this is a life-threatening emergency. Do not wait. Call 108/911 at first suspicion of pulmonary embolism
■■ RECOVERY TIME	Hospitalization 5-10 days; anticoagulation 3-6 months to lifelong; recovery of lung tissue weeks to months

Whooping Cough (Pertussis)

Category: Respiratory

■ APPLICABLE TO	All Ages (Most Severe in Infants Under 1 Year)
■ SYMPTOMS	Begins like a cold (runny nose, mild fever), then progresses to severe coughing fits with characteristic 'whoop' sound when inhaling; cough fits may cause vomiting; infants may turn blue or stop breathing (apnea); exhaustion after coughing fits; cough lasts weeks to months
■ MEDICATIONS	Azithromycin or Clarithromycin (antibiotics — most effective in early catarrhal stage); same antibiotics for all household contacts regardless of vaccination status; hospitalization and oxygen for infants; supportive: plenty of fluids, small meals
■ HOME REMEDIES	Small frequent meals (large meals trigger cough fits); humidifier in room; honey for older children (over 1 year); ginger tea; fresh air without cold wind; keep head elevated during sleep; complete rest
■ DIET & NUTRITION	Small frequent meals (6-8 small meals per day to avoid cough-triggering fullness); soft easily digestible foods; plenty of warm fluids; high calorie and protein to maintain nutrition during prolonged illness; avoid dairy if it increases mucus
■■ PRECAUTIONS	DTaP vaccination for children (5 doses); Tdap booster for adults, especially caregivers and pregnant women (between 27-36 weeks each pregnancy); isolate infected person; treat all household contacts
■ WHEN TO GO TO HOSPITAL	All infants under 1 year must be hospitalized; turning blue during cough; apnea (breathing stops); severe dehydration; cough lasting more than 3 weeks without improvement
■■ RECOVERY TIME	Known as the 100-day cough; complete resolution takes 2-3 months; acute phase 1-2 weeks

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Allergic Rhinitis (Hay Fever)

Category: Respiratory

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Sneezing (often repeated), runny nose with clear watery discharge, nasal congestion, itchy nose/eyes/throat/ears, watery red eyes, post-nasal drip, fatigue, headache; triggered by pollen, dust mites, pet dander, mold
■ MEDICATIONS	Antihistamines: Cetirizine, Loratadine, Fexofenadine (non-drowsy), Diphenhydramine (drowsy); Nasal corticosteroid sprays: Fluticasone, Budesonide (most effective long-term); Decongestants; Montelukast (Singulair); Allergy immunotherapy (desensitization)
■ HOME REMEDIES	Saline nasal irrigation (Neti pot), HEPA air purifiers, steam inhalation, local honey (builds tolerance to local pollen), butterbur extract, quercetin-rich foods, keep windows closed during high pollen season

■ DIET & NUTRITION	Anti-inflammatory diet, quercetin-rich foods (apples, onions, berries), omega-3 fatty acids, vitamin C, local honey, avoid alcohol and spicy foods (can worsen symptoms), cold foods may worsen nasal congestion
■■ PRECAUTIONS	Monitor pollen counts, keep windows closed on high pollen days, shower after outdoor exposure, change clothes after being outside, use air conditioning, avoid known allergens, HEPA filters in bedroom
■ WHEN TO GO TO HOSPITAL	Asthma complications, severe sinusitis, polyps, very poor quality of life despite medication, considering allergy testing and immunotherapy, suspected anaphylaxis
■■ RECOVERY TIME	Chronic seasonal condition; allergy immunotherapy over 3-5 years can significantly reduce or eliminate symptoms

Laryngitis

Category: Respiratory

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Hoarse or raspy voice (hoarseness), weak or loss of voice (aphonia), raw or sore throat, dry scratchy throat, dry cough, tickling sensation in throat, mild fever; acute laryngitis most often follows URI or voice overuse; chronic laryngitis from acid reflux, smoking, or chronic voice abuse
■ MEDICATIONS	Viral: Voice rest and steam inhalation (main treatment); Anti-inflammatory: Ibuprofen for pain; Throat lozenges; Humidifier; GERD-related: PPIs (Omeprazole); Antibiotics only if bacterial (rare); Corticosteroid injection (performers with urgent need to use voice)
■ HOME REMEDIES	Complete voice rest (most important — do not whisper, which strains more than normal speech), steam inhalation with eucalyptus, honey-lemon-ginger tea, warm water with turmeric, stay hydrated, slippery elm tea, marshmallow root tea, gargle with warm salt water
■ DIET & NUTRITION	Warm soothing fluids, honey and lemon in warm water, avoid caffeine and alcohol (dehydrate throat), avoid spicy and acidic foods (acid reflux worsens laryngitis), avoid very hot drinks, warm soups and broths
■■ PRECAUTIONS	Complete voice rest (not even whispering), avoid throat clearing (traumatic), use amplification if must speak, steam inhalation 2-3 times daily, manage GERD if present, avoid smoking and secondhand smoke, stay hydrated
■ WHEN TO GO TO HOSPITAL	Difficulty breathing or swallowing, high fever, complete voice loss lasting more than 3 weeks, blood in cough, neck mass, hoarseness in smoker (rule out laryngeal cancer), stridor (breathing noise in children — possible epiglottitis)
■■ RECOVERY TIME	Acute viral laryngitis: 1-3 weeks with voice rest; GERD-related: weeks to months of treatment

Lung Cancer

Category: Respiratory

■ APPLICABLE TO	Adults (especially Smokers over 50)
■ SYMPTOMS	Persistent new cough or change in chronic cough, coughing up blood (hemoptysis), chest pain (dull aching or sharp), shortness of breath, hoarseness, weight loss, fatigue, loss of appetite, recurrent pneumonia or bronchitis, bone pain (metastatic), headache (brain metastasis), swelling of face/neck (SVC syndrome)
■ MEDICATIONS	Non-Small Cell LC: Surgery (lobectomy/pneumonectomy for early stage), Chemotherapy (Cisplatin/Carboplatin-based), Radiation, Targeted therapy (EGFR inhibitors: Erlotinib, Osimertinib; ALK inhibitors: Crizotinib), Immunotherapy (Pembrolizumab, Nivolumab); Small Cell LC: Chemotherapy + Radiation; Palliative care for advanced
■ HOME REMEDIES	Supportive care: ginger for chemotherapy nausea, exercise as tolerated, breathing exercises, manage anxiety, adequate nutrition, social support; PREVENTION: stop smoking immediately (most important — reduces risk even after years of smoking)
■ DIET & NUTRITION	High protein and calorie diet (prevent cancer cachexia), colorful antioxidant-rich vegetables, omega-3 fatty acids, cruciferous vegetables (broccoli, cauliflower — anti-cancer compounds), no alcohol, adequate vitamin D
■■ PRECAUTIONS	Smoking cessation is most important preventive measure; annual low-dose CT (LDCT) screening for high-risk individuals (55+ with 30 pack-year smoking history); radon testing in home; avoid occupational carcinogens (asbestos, arsenic)
■ WHEN TO GO TO HOSPITAL	Persistent cough 3+ weeks especially in smoker, coughing blood, chest pain, unexplained weight loss, hoarseness, breathlessness — all require urgent evaluation including chest X-ray and CT
■■ RECOVERY TIME	Stage I NSCLC (surgery): 60-80% 5-year survival; Stage IV: significantly lower; targeted therapy and immunotherapy have dramatically improved outcomes for eligible patients

Respiratory Syncytial Virus (RSV)

Category: Respiratory

■ APPLICABLE TO	Infants and Toddlers under 2 years (also Elderly and Immunocompromised)
■ SYMPTOMS	Starts like common cold (runny nose, decreased appetite), progresses to: wheezing, rapid breathing, flaring nostrils, retractions (skin between ribs pulls in with breathing), cough, low-grade fever, feeding difficulties in infants; most dangerous in premature infants and infants under 3 months
■ MEDICATIONS	Mainly supportive: ensure adequate hydration, nasal suctioning (bulb syringe), humidified oxygen if needed; Paracetamol for fever; Hospitalized severe cases: oxygen therapy, IV fluids, high-flow nasal cannula; Nirsevimab (Beyfortus) — new monoclonal antibody prophylaxis; Palivizumab (Synagis) for high-risk infants

■ HOME REMEDIES	Nasal saline drops and suctioning (clears breathing), humidifier in room, keep head slightly elevated, small frequent feeds (breathing difficulty makes large feeds hard), cool mist humidifier, avoid secondhand smoke
■ DIET & NUTRITION	Continue breastfeeding or formula — breast milk provides antibodies; small frequent feeds; maintain adequate hydration; ORS if refusing feeds; no solid foods during acute illness in infants
■■ PRECAUTIONS	Frequent handwashing (RSV survives on surfaces 4-7 hours), avoid exposing young infants to sick people during RSV season (autumn-winter), keep premature infants away from crowds, do not smoke around infants, Nirsevimab vaccine for newborns
■ WHEN TO GO TO HOSPITAL	Breathing rate above 60/min in infant, nasal flaring, chest retractions, blue-tinged lips or fingertips (cyanosis), dehydration (no wet diapers), extreme lethargy, feeding less than half normal intake for 2 consecutive feeds
■■ RECOVERY TIME	Mild RSV: 1-2 weeks; severe bronchiolitis: 2-3 weeks; hospitalized infants: average 3-5 days; increased asthma risk in children with severe RSV

Cardiovascular & Metabolic Diseases

16 Conditions Covered — 8 Guidance Fields Each

Hypertension (High Blood Pressure)

Category: Cardiovascular

■ APPLICABLE TO	Adults
■ SYMPTOMS	Usually asymptomatic (silent killer). Severe: headache, shortness of breath, nosebleeds, chest pain, vision changes, fatigue. BP consistently above 130/80 mmHg
■ MEDICATIONS	ACE inhibitors (Enalapril, Lisinopril), ARBs (Losartan, Valsartan), Calcium channel blockers (Amlodipine), Thiazide diuretics (Hydrochlorothiazide), Beta-blockers (Atenolol, Metoprolol)
■ HOME REMEDIES	Regular aerobic exercise 30 min/day, meditation and yoga, deep breathing (reduces stress), garlic, hibiscus tea, beetroot juice, banana (potassium), olive oil
■ DIET & NUTRITION	DASH diet (Dietary Approaches to Stop Hypertension): low sodium (under 2300mg/day), high potassium (bananas, spinach), high fiber, low saturated fat, limit alcohol
■■ PRECAUTIONS	Take medications as prescribed (never stop suddenly), monitor BP regularly, quit smoking, limit alcohol, reduce salt intake, manage stress, regular check-ups
■ WHEN TO GO TO HOSPITAL	BP above 180/120 (hypertensive crisis), chest pain, severe headache, vision changes, shortness of breath, stroke symptoms (FAST: Face drooping, Arm weakness, Speech difficulty, Time to call emergency)
■■ RECOVERY TIME	Lifelong management; BP typically controlled within weeks of treatment

Heart Attack (Myocardial Infarction)

Category: Cardiovascular

■ APPLICABLE TO	Adults (risk increases with age)
■ SYMPTOMS	Chest pain/pressure/tightness, pain radiating to left arm, jaw, neck, back; shortness of breath, nausea/vomiting, cold sweats, lightheadedness, sense of doom. Women may have atypical symptoms (fatigue, jaw pain)
■ MEDICATIONS	EMERGENCY - Call ambulance immediately! Aspirin 325mg (chew, don't swallow) while waiting. Hospital: Thrombolytics, PCI (stent), CABG, Heparin, Beta-blockers, ACE inhibitors, Statins, Clopidogrel
■ HOME REMEDIES	Prevention only: omega-3 fatty acids, garlic, turmeric, regular exercise, stress management, heart-healthy diet. DURING ATTACK: Call emergency immediately - no home treatment

■ DIET & NUTRITION	Mediterranean diet for prevention: olive oil, fish, nuts, fruits, vegetables, whole grains, limited red meat, minimal processed foods
■■ PRECAUTIONS	Know heart attack warning signs, take preventive medications as prescribed, control BP/diabetes/cholesterol, quit smoking, regular cardiac checkups if high risk
■ WHEN TO GO TO HOSPITAL	IMMEDIATELY - Call 911/102/108. Every minute matters. Do not drive yourself. Chew aspirin while waiting for ambulance
■■ RECOVERY TIME	Hospitalization 3-7 days; cardiac rehabilitation 3-6 months; full recovery months to a year

Diabetes Type 1

Category: Metabolic/Endocrine

■ APPLICABLE TO	Children, Young Adults
■ SYMPTOMS	Frequent urination, excessive thirst, unexplained weight loss, extreme hunger, blurred vision, fatigue, irritability, bedwetting in children, ketoacidosis (fruity breath, nausea)
■ MEDICATIONS	Insulin therapy (life-essential): Rapid-acting (Lispro, Aspart), Long-acting (Glargine, Detemir), Insulin pump (CSII), Regular blood glucose monitoring, CGM (Continuous Glucose Monitor)
■ HOME REMEDIES	Consistent meal timing, regular physical activity (monitor glucose during exercise), stress management, adequate sleep. No substitute for insulin
■ DIET & NUTRITION	Carbohydrate counting approach, consistent carb intake, avoid sugary beverages, high fiber foods, low glycemic index foods, regular meal schedule
■■ PRECAUTIONS	Never skip insulin, carry fast-acting sugar for hypoglycemia, medical alert ID bracelet, regular HbA1c testing (every 3 months), annual eye/kidney/foot exams
■ WHEN TO GO TO HOSPITAL	DKA symptoms (vomiting, abdominal pain, fruity breath, confusion), severe hypoglycemia (unconscious/seizure), blood sugar below 54 mg/dL or above 300 mg/dL
■■ RECOVERY TIME	Lifelong condition requiring continuous insulin management

Diabetes Type 2

Category: Metabolic/Endocrine

■ APPLICABLE TO	Adults (increasingly in children too)
■ SYMPTOMS	Often asymptomatic initially. Increased thirst, frequent urination, fatigue, blurred vision, slow healing wounds, frequent infections, tingling in hands/feet, dark patches in skin folds

■ MEDICATIONS	Metformin (first-line), Sulfonylureas (Glipizide, Glimepiride), SGLT2 inhibitors (Empagliflozin, Dapagliflozin), GLP-1 agonists (Semaglutide, Liraglutide), DPP-4 inhibitors (Sitagliptin), Insulin (if advanced)
■ HOME REMEDIES	Fenugreek seeds soaked overnight, bitter melon (karela) juice, cinnamon, berberine, regular walking, weight management, stress reduction
■ DIET & NUTRITION	Low glycemic index diet, whole grains, legumes, non-starchy vegetables, lean protein, healthy fats, avoid refined carbs, sugary drinks, processed foods. Portion control critical
■■ PRECAUTIONS	Daily blood sugar monitoring, foot care (daily inspection), annual dilated eye exam, kidney function tests, dental check-ups, flu/pneumonia vaccines, no smoking
■ WHEN TO GO TO HOSPITAL	Hypoglycemia (shaking, sweating, confusion), blood sugar above 250 with ketones, symptoms of DKA, severe foot wounds, chest pain
■■ RECOVERY TIME	Lifelong management; with lifestyle changes, remission possible in some cases

Heart Failure (Congestive)

Category: Cardiovascular

■ APPLICABLE TO	Adults (especially elderly)
■ SYMPTOMS	Shortness of breath (especially lying flat or exertion), leg/ankle swelling, fatigue, rapid weight gain (fluid), persistent cough/wheezing, reduced ability to exercise, rapid heartbeat
■ MEDICATIONS	ACE inhibitors/ARBs, Beta-blockers (Carvedilol, Metoprolol), Diuretics (Furosemide/Lasix - 'water pill'), Aldosterone antagonists (Spironolactone), SGLT2 inhibitors, Digoxin, Ivabradine
■ HOME REMEDIES	Daily weight monitoring (report 2+ lb gain in a day), fluid restriction as advised, low sodium diet, elevation of legs, light activity as tolerated
■ DIET & NUTRITION	Strict sodium restriction (under 2000mg/day), possible fluid restriction, heart-healthy diet, monitor potassium levels if on diuretics, avoid alcohol
■■ PRECAUTIONS	Weigh daily at same time, take all medications as prescribed, monitor for edema, limit alcohol, no smoking, have a heart failure action plan
■ WHEN TO GO TO HOSPITAL	Sudden weight gain of 3+ lbs in a day, severe breathing difficulty at rest, coughing up pink frothy mucus, severe swelling, confusion
■■ RECOVERY TIME	Chronic condition with acute decompensation requiring hospitalization; managed lifelong

Stroke (Cerebrovascular Accident)

Category: Cardiovascular/Neurological

■ APPLICABLE TO	Adults (risk increases with age)

■ SYMPTOMS	FAST: Face drooping (uneven smile), Arm weakness (one arm drifts down), Speech difficulty (slurred/strange words), Time to call emergency; Also: sudden severe headache, vision changes, balance problems, confusion
■ MEDICATIONS	EMERGENCY ONLY. Ischemic stroke: tPA (clot-buster) within 4.5 hours, mechanical thrombectomy within 24 hours. Prevention: Antiplatelets (Aspirin, Clopidogrel), Anticoagulants, Statins, BP medications
■ HOME REMEDIES	Prevention: Mediterranean diet, regular exercise, quit smoking, control BP and diabetes. DURING STROKE: No home treatment - call emergency immediately
■ DIET & NUTRITION	Mediterranean diet for prevention, low sodium, low saturated fat, high fiber, omega-3 rich foods, limit alcohol
■■ PRECAUTIONS	Control hypertension (biggest risk factor), no smoking, manage diabetes, healthy weight, regular exercise, limit alcohol, treat AFib if present
■ WHEN TO GO TO HOSPITAL	IMMEDIATELY upon any FAST symptoms. Time = Brain: every minute 1.9 million neurons die. Call 108/911 immediately
■■ RECOVERY TIME	Hospitalization: days to weeks; Recovery: months to years; Some disability may be permanent

Atherosclerosis / High Cholesterol

Category: Cardiovascular

■ APPLICABLE TO	Adults
■ SYMPTOMS	Usually silent until complications. Angina (chest pain with exertion), leg pain when walking (PAD), TIA symptoms. Diagnosed by blood test: LDL above 130 mg/dL
■ MEDICATIONS	Statins (Atorvastatin, Rosuvastatin) - first line; Ezetimibe; PCSK9 inhibitors (Evolocumab); Bile acid sequestrants; Fibrates for high triglycerides; Niacin
■ HOME REMEDIES	Oats (beta-glucan), garlic, fish oil (omega-3), psyllium husk, plant sterols, red yeast rice, flaxseeds, walnuts, olive oil, regular aerobic exercise
■ DIET & NUTRITION	Mediterranean diet, high fiber (oats, legumes, fruits), omega-3 fatty acids (fatty fish), avoid trans fats, limit saturated fats, increase plant sterols
■■ PRECAUTIONS	Annual cholesterol screening after 20, take statins as prescribed, quit smoking, regular exercise, manage blood pressure and diabetes, healthy weight
■ WHEN TO GO TO HOSPITAL	Chest pain, shortness of breath, leg pain when walking (PAD), TIA symptoms, coronary artery disease evaluation
■■ RECOVERY TIME	Chronic condition - atherosclerosis can regress with aggressive treatment

Atrial Fibrillation (AFib)

Category: Cardiovascular

■ APPLICABLE TO	Adults (Especially Elderly)
■ SYMPTOMS	Irregular rapid heartbeat (palpitations), fluttering sensation in chest, shortness of breath, fatigue, dizziness, chest pain, reduced ability to exercise; may be asymptomatic (discovered on ECG); major risk factor for stroke
■ MEDICATIONS	Rate control: Beta-blockers (Metoprolol), Calcium channel blockers (Diltiazem), Digoxin; Rhythm control: Amiodarone, Flecainide; Anticoagulation (stroke prevention): Warfarin or NOACs (Rivaroxaban, Apixaban, Dabigatran); Cardioversion (electrical or chemical)
■ HOME REMEDIES	Vagal maneuvers for paroxysmal AFib (Valsalva — bear down, cold water on face, controlled breathing); regular aerobic exercise; stress management; limit alcohol and caffeine; maintain healthy weight; omega-3 fatty acids
■ DIET & NUTRITION	Heart-healthy diet (Mediterranean), limit alcohol (major AFib trigger), limit caffeine, consistent Vitamin K intake if on Warfarin (consistent leafy greens — do not suddenly increase or decrease), omega-3 rich foods
■■ PRECAUTIONS	Never miss anticoagulant doses (stroke risk), regular INR monitoring if on Warfarin, avoid alcohol binge drinking (holiday heart), CHADS-VASc score assessment, regular cardiology follow-up, know stroke warning signs
■ WHEN TO GO TO HOSPITAL	New onset palpitations, rapid heart rate above 150 bpm, chest pain, breathlessness, fainting, any stroke symptoms (FAST — Face/Arm/Speech/Time)
■■ RECOVERY TIME	Chronic condition managed lifelong; paroxysmal AFib episodes can be brief; persistent/permanent AFib managed with rate control and anticoagulation

Angina Pectoris

Category: Cardiovascular

■ APPLICABLE TO	Adults (especially 50+)
■ SYMPTOMS	Chest pain, pressure, tightness, or squeezing triggered by physical exertion or emotional stress; pain may radiate to left arm, jaw, or shoulder; usually lasts 1-5 minutes and is relieved by rest or Nitroglycerin; shortness of breath, sweating
■ MEDICATIONS	Sublingual Nitroglycerin (GTN spray or tablets) for acute attacks — carry always; Long-acting nitrates (Isosorbide mononitrate), Beta-blockers (Atenolol, Metoprolol), Calcium channel blockers; Aspirin (daily), Statins; Revascularization (stent or bypass) for unstable or refractory angina
■ HOME REMEDIES	Rest immediately when chest pain occurs; Nitroglycerin as prescribed; stress management and relaxation; light cardiac exercise program (cardiac rehab); omega-3 supplementation; garlic and turmeric (anti-inflammatory)
■ DIET & NUTRITION	Mediterranean heart-healthy diet; avoid large heavy meals (increases oxygen demand); limit saturated and trans fats; high fiber; omega-3 fatty acids; limit alcohol; maintain healthy weight

■■ PRECAUTIONS	Always carry sublingual Nitroglycerin; avoid exertion in extreme heat or cold; sexual activity guidance from cardiologist; avoid heavy lifting; no smoking; recognize when stable angina becomes unstable (rest pain = emergency)
■ WHEN TO GO TO HOSPITAL	Chest pain at rest or lasting more than 15 minutes after Nitroglycerin (unstable angina or heart attack — EMERGENCY); new onset chest pain; increasing frequency of angina attacks; pain not relieved by 2-3 Nitroglycerin doses
■■ RECOVERY TIME	Stable angina managed lifelong; revascularization can significantly reduce or eliminate symptoms

Deep Vein Thrombosis (DVT)

Category: Cardiovascular

■ APPLICABLE TO	Adults
■ SYMPTOMS	Leg pain (often like a cramp), swelling in one leg, warmth and redness over affected area, skin discoloration (red or bluish), heaviness in leg; may be asymptomatic; can be life-threatening if clot travels to lungs (PE)
■ MEDICATIONS	Anticoagulants: Low Molecular Weight Heparin (Enoxaparin), Direct oral anticoagulants (Rivaroxaban, Apixaban — preferred), Warfarin; duration 3-6 months minimum; compression stockings; thrombolytics for massive DVT; IVC filter in select cases
■ HOME REMEDIES	Leg elevation above heart level, compression stockings, calf exercises, walking as soon as permitted on anticoagulants; warm (NOT hot) compress for pain; stay well hydrated; ginger and turmeric anti-inflammatory support
■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids (fish, walnuts, flaxseed), adequate hydration, consistent Vitamin K intake if on Warfarin, avoid alcohol, maintain healthy weight
■■ PRECAUTIONS	Compression stockings during long travel, regular leg exercises on long flights, stay hydrated, early mobilization after surgery, anticoagulation prophylaxis for high-risk patients, know signs and symptoms
■ WHEN TO GO TO HOSPITAL	Suspected DVT — confirm with ultrasound Doppler; EMERGENCY if sudden shortness of breath, chest pain, coughing blood — may indicate pulmonary embolism; anticoagulation should start as soon as DVT confirmed
■■ RECOVERY TIME	Acute DVT: symptoms improve in days to weeks on anticoagulation; anticoagulation continued 3 months to lifelong; post-thrombotic syndrome possible

Varicose Veins

Category: Cardiovascular

■ APPLICABLE TO	Adults (especially Women, Pregnant Women, People who Stand Long Hours)

■ SYMPTOMS	Bulging twisted dark blue or purple veins visible under skin (mostly legs); aching, heaviness, or tired feeling in legs; swelling in lower legs and ankles; itching around veins; skin discoloration; occasional bleeding from superficial varicose vein
■ MEDICATIONS	Sclerotherapy (injection to close vein), Laser therapy (EVLA/RFA — endovenous), Compression stockings (conservative management), Phlebectomy (surgical removal), Diosmin + Hesperidin (Daflon — vascular tone), topical Heparinoid cream
■ HOME REMEDIES	Elevation of legs above heart level for 15-20 minutes several times daily, regular walking and leg exercise, cold water rinse on legs, horse chestnut extract (aescin), grape seed extract, compression stockings
■ DIET & NUTRITION	High-fiber diet (prevent constipation — straining worsens veins), flavonoid-rich foods (berries, citrus), vitamin C for vessel wall strength, omega-3 fatty acids, maintain healthy weight, limit salt (reduces swelling), avoid prolonged standing or sitting
■■ PRECAUTIONS	Wear compression stockings during prolonged standing, elevate feet at end of day, avoid crossing legs, take regular walking breaks, maintain healthy weight, exercise regularly (swimming and cycling are excellent)
■ WHEN TO GO TO HOSPITAL	Severe pain, swelling, or sudden redness over a varicose vein (thrombophlebitis), open sores or skin ulcers, bleeding from varicose vein (apply pressure and go immediately), signs of DVT
■■ RECOVERY TIME	Conservative management ongoing; sclerotherapy and laser: cosmetic improvement in 4-6 weeks; minimally invasive procedures: return to normal activity in 1-2 weeks

Obesity and Metabolic Syndrome

Category: Cardiovascular / Metabolic

■ APPLICABLE TO	Adults and Children
■ SYMPTOMS	BMI above 30 (obesity); cluster of metabolic syndrome: central obesity (waist above 35 inches women, 40 inches men), high blood pressure, high blood sugar, high triglycerides, low HDL cholesterol; fatigue, sleep apnea, joint pain, breathlessness on exertion, low self-esteem
■ MEDICATIONS	Lifestyle modification (cornerstone); Semaglutide (Ozempic/Wegovy — weight loss injection), Tirzepatide (Mounjaro), Orlistat (Xenical), Phentermine; Bariatric surgery for severe obesity (BMI above 40 or 35 with comorbidities): Sleeve Gastrectomy, Roux-en-Y Gastric Bypass
■ HOME REMEDIES	Regular aerobic exercise (150 min/week minimum), strength training (2x/week), mindful eating, food journaling, apple cider vinegar diluted (appetite suppression — limited evidence), green tea, intermittent fasting under supervision

■ DIET & NUTRITION	Caloric deficit (500-1000 cal/day below maintenance), high protein, high fiber, low refined carbohydrate; Mediterranean or low-glycemic diet; avoid ultra-processed foods, sugary drinks, fast food; portion control; eat slowly and mindfully
■■ PRECAUTIONS	Set realistic weight loss goals (0.5-1 kg per week), regular monitoring of BP, blood sugar and lipids, sleep apnea treatment, psychological support for emotional eating, structured exercise program, regular medical follow-up
■ WHEN TO GO TO HOSPITAL	BMI above 40, obesity-related complications (severe sleep apnea, uncontrolled diabetes, joint destruction), bariatric surgery evaluation, rapid unintended weight loss (rule out cancer)
■■ RECOVERY TIME	Lifestyle changes show metabolic improvement in weeks; significant weight loss: months to years; bariatric surgery: most weight lost in first 12-18 months

Raynaud's Disease / Phenomenon

Category: Cardiovascular

■ APPLICABLE TO	Adults (Women more than Men)
■ SYMPTOMS	Color changes in fingers and toes triggered by cold or stress: white (pallor — blood vessel spasm), blue (cyanosis — lack of oxygen), red (reactive hyperemia — blood returning); accompanied by numbness, tingling, throbbing pain; episodes last minutes to hours; Primary Raynaud's (no underlying disease) vs Secondary (associated with scleroderma, lupus, rheumatoid arthritis)
■ MEDICATIONS	Calcium channel blockers (Amlodipine, Nifedipine — first-line, relax blood vessels), Phosphodiesterase inhibitors (Sildenafil for severe), Alpha-blockers (Prazosin), Prostacyclin infusions (severe digital ischemia), Topical Nitroglycerin; avoid beta-blockers (worsen Raynaud's)
■ HOME REMEDIES	Keep hands and feet warm (gloves, heated gloves, wool socks), hand warmers, warm water immersion of hands during attack, layered clothing, avoid cold triggers, grip warm objects during attacks, keep whole body warm (cold body triggers extremity vasospasm)
■ DIET & NUTRITION	Omega-3 fatty acids (fish oil — improves vascular reactivity), reduce caffeine and alcohol, stay well hydrated, warm foods and drinks, ginkgo biloba (some evidence), L-arginine foods (meat, dairy, nuts — nitric oxide precursor)
■■ PRECAUTIONS	Avoid cold triggers (cold air, cold water, frozen food handling), gloves in supermarket freezer aisles, avoid smoking (vasoconstrictor), manage stress, keep home warm, avoid vibrating tools, identify and treat secondary causes
■ WHEN TO GO TO HOSPITAL	Digital ulcers developing (skin breakdown on finger tips), gangrene, suspected secondary Raynaud's (systemic connective tissue disease evaluation), severe disabling symptoms not responding to calcium channel blockers
■■ RECOVERY TIME	Primary Raynaud's: chronic manageable condition, does not cause tissue damage; Secondary: depends on underlying disease treatment

Pericarditis

Category: Cardiovascular

■ APPLICABLE TO	Adults
■ SYMPTOMS	Sharp stabbing chest pain (worse lying flat, better leaning forward — characteristic), radiating to left shoulder and neck, low-grade fever, fatigue, pericardial friction rub (scratching sound with stethoscope), shortness of breath; often follows viral infection; ECG changes (diffuse ST elevation saddle-shaped)
■ MEDICATIONS	NSAIDs (Aspirin 750-1000mg every 8 hours for 1-2 weeks OR Ibuprofen) — first-line; Colchicine (added to NSAIDs — halves recurrence rate, continue 3 months); Corticosteroids (only if NSAIDs contraindicated or refractory, low-dose taper); Restrict activity until symptom-free; Antibiotics if bacterial cause
■ HOME REMEDIES	Rest and activity restriction until pain-free and CRP normalized, sit upright and lean forward position for comfort, warm compress, avoid strenuous exercise (weeks to months), stress management
■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids, avoid alcohol during recovery, maintain adequate hydration, no specific restrictions but heart-healthy diet recommended
■■ PRECAUTIONS	Activity restriction until symptom-free AND CRP normal AND echo normal (prevents recurrence), complete full Colchicine course (3 months), return to exercise very gradually, monitor for complications (pericardial effusion, cardiac tamponade)
■ WHEN TO GO TO HOSPITAL	Cardiac tamponade signs (muffled heart sounds, low BP, distended neck veins — Beck's triad — EMERGENCY), high fever (bacterial pericarditis), not responding to NSAIDs, large effusion on echocardiogram
■■ RECOVERY TIME	Acute pericarditis: 4-6 weeks; 15-30% recurrence rate (reduced by Colchicine); rare progression to constrictive pericarditis

Anemia (Iron Deficiency)

Category: Cardiovascular / Hematological

■ APPLICABLE TO	All Ages (Common in Women, Pregnant Women, Children)
■ SYMPTOMS	Fatigue and weakness (most common), pale skin, pale inner eyelids and gums, shortness of breath on exertion, dizziness and lightheadedness, cold hands and feet, brittle nails, spoon-shaped nails (koilonychia), hair loss, rapid heartbeat, headache, difficulty concentrating, pica (craving for non-food items like ice, clay, dirt)
■ MEDICATIONS	Ferrous sulfate 325mg (65mg elemental iron) taken with Vitamin C for absorption, 3 times daily — continue 3 months after Hb normalizes; Ferrous gluconate (better tolerated GI); IV iron (Ferric carboxymaltose, Iron sucrose) for severe anemia or oral intolerance; Blood transfusion for life-threatening anemia; Find and treat underlying cause

■ HOME REMEDIES	Cook in cast iron cookware (increases dietary iron), take iron with lemon juice (vitamin C enhances absorption), avoid iron with tea/coffee/milk (tannins and calcium reduce absorption), drink water with iron tablet NOT milk
■ DIET & NUTRITION	Heme iron (best absorbed): red meat, chicken liver, fish. Non-heme iron: spinach, lentils, beans, tofu, fortified cereals, pumpkin seeds, dark chocolate; always pair non-heme iron with vitamin C; avoid tea and coffee with meals; Cook in cast iron pans
■■ PRECAUTIONS	Find underlying cause (heavy periods, GI bleeding — occult blood test, GI endoscopy in men and postmenopausal women), take iron with vitamin C not tea, address constipation from iron supplements (fiber and water), annual CBC for high-risk groups (women with heavy periods)
■ WHEN TO GO TO HOSPITAL	Severe symptoms (fainting, chest pain, extreme breathlessness), Hb below 7g/dL, iron deficiency anemia in men or postmenopausal women (GI bleeding must be excluded), failure to respond to oral iron after 4 weeks
■■ RECOVERY TIME	Hb rises 1g/dL per week with adequate iron; continue treatment 3 months after normalization; address underlying cause

Peripheral Artery Disease (PAD)

Category: Cardiovascular

■ APPLICABLE TO	Adults (especially Smokers, Diabetics over 50)
■ SYMPTOMS	Intermittent claudication (cramping pain in leg muscles — thighs, calves — triggered by walking, relieved by rest); shiny hairless cold skin on legs; weak or absent pulses in legs; wounds on feet that heal slowly or not at all; rest pain in toes at night (severe PAD); reduced ABI (Ankle-Brachial Index) below 0.9
■ MEDICATIONS	Antiplatelet: Aspirin 75-100mg daily, Clopidogrel; Statins (Atorvastatin — essential in PAD regardless of cholesterol); Cilostazol (improves walking distance); Revascularization: Angioplasty and stenting, Bypass surgery for severe critical limb ischemia; Wound care for foot ulcers
■ HOME REMEDIES	Supervised walking exercise program (most effective at improving claudication distance — builds collateral circulation), quit smoking immediately (most important intervention), keep legs lower than heart (opposite of venous disease), foot care and wound prevention
■ DIET & NUTRITION	Mediterranean diet, quit smoking (strongest risk reduction), strict diabetes control, blood pressure control, statins for lipid management, omega-3 fatty acids, avoid trans fats
■■ PRECAUTIONS	Strict diabetes and BP control, quit smoking (single most important step), inspect feet daily (especially diabetics — reduced sensation), proper footwear, avoid tight socks and hot water bottles (burn risk), ankle-brachial index (ABI) test screening for at-risk

■ WHEN TO GO TO HOSPITAL	Critical limb ischemia (rest pain at night, non-healing ulcers, gangrene — urgent revascularization needed), sudden onset severe leg pain with coolness and pallor (acute limb ischemia — EMERGENCY), new foot ulcer in diabetic with PAD
■■ RECOVERY TIME	PAD is chronic and progressive; exercise program improves walking distance 50-150%; revascularization significantly improves quality of life; limb loss risk 5-10% in critical limb ischemia without intervention

Children's Diseases

22 Conditions Covered — 8 Guidance Fields Each

Chickenpox (Varicella)

Category: Children's Disease

■ APPLICABLE TO	Children (also adults who are unvaccinated)
■ SYMPTOMS	Itchy blister-like rash starting on face/chest/back spreading to whole body, fever, fatigue, headache, loss of appetite. Rash progresses: red spots → fluid-filled blisters → scabs
■ MEDICATIONS	Usually self-limiting; Calamine lotion for itching, Antihistamines (Diphenhydramine, Cetirizine), Acetaminophen for fever (NEVER Aspirin in children - Reye syndrome risk), Acyclovir for severe cases or immunocompromised
■ HOME REMEDIES	Oatmeal bath to soothe itching, baking soda paste on rash, neem leaves bath water, keep nails trimmed, loose cotton clothing, cool compress, coconut oil on rash
■ DIET & NUTRITION	Soft bland foods, ice cream, cold yogurt (soothing), plenty of fluids, avoid salty/acidic foods if mouth sores present
■■ PRECAUTIONS	Vaccinate (Varicella vaccine 2 doses), keep child home until all blisters crust over (5-7 days), prevent scratching to avoid scarring/infection, separate from pregnant women and immunocompromised
■ WHEN TO GO TO HOSPITAL	High fever above 104°F, rash spreading to eyes, difficulty breathing, extreme drowsiness, severe bacterial skin infection, stiff neck
■■ RECOVERY TIME	7-10 days; virus remains dormant and can cause shingles later in life

Measles (Rubeola)

Category: Children's Disease

■ APPLICABLE TO	Children (unvaccinated)
■ SYMPTOMS	High fever, cough, runny nose, red watery eyes (conjunctivitis), Koplik spots (white spots inside mouth - pathognomonic), red blotchy rash starting on face spreading downward
■ MEDICATIONS	No antiviral; Vitamin A supplementation (reduces severity and mortality), Acetaminophen for fever, Antibiotics for bacterial complications (pneumonia, ear infection), supportive care
■ HOME REMEDIES	Rest, plenty of fluids, dim lighting (eyes sensitive to light), vitamin A rich foods, isolation from unvaccinated people

■ DIET & NUTRITION	Soft nutritious foods, vitamin A rich foods (carrots, sweet potato, dairy), broths and soups, plenty of fluids
■■ PRECAUTIONS	MMR vaccination (2 doses - most effective prevention), isolation for 4 days after rash onset, vitamin A supplementation in deficient children, report to health authorities
■ WHEN TO GO TO HOSPITAL	Difficulty breathing (pneumonia), ear pain, seizures, confusion, severe dehydration, eye problems, high fever not responding to medication
■■ RECOVERY TIME	7-10 days; complications may prolong recovery

Mumps

Category: Children's Disease

■ APPLICABLE TO	Children (unvaccinated)
■ SYMPTOMS	Swollen tender salivary glands (parotitis) - puffy cheeks and jaw, fever, headache, fatigue, muscle aches, loss of appetite, difficulty chewing/swallowing
■ MEDICATIONS	No specific antiviral; Acetaminophen or Ibuprofen for pain and fever, warm/cold compresses for jaw pain, supportive care, rest
■ HOME REMEDIES	Cold or warm compress on swollen glands, rest, ice chips to soothe mouth, soft foods, marshmallow root tea (soothing)
■ DIET & NUTRITION	Soft foods that don't require much chewing (soups, yogurt, mashed potatoes, smoothies), avoid acidic or sour foods that trigger saliva production and pain
■■ PRECAUTIONS	MMR vaccination prevents mumps, isolate until 5 days after swelling begins, complications: orchitis in post-pubertal males (may cause infertility), meningitis, deafness
■ WHEN TO GO TO HOSPITAL	Severe headache/stiff neck (meningitis), testicular swelling and pain in males, hearing loss, abdominal pain, high fever
■■ RECOVERY TIME	1-2 weeks

Hand, Foot and Mouth Disease

Category: Children's Disease

■ APPLICABLE TO	Infants and Children under 5
■ SYMPTOMS	Fever, sore throat, painful sores/blisters on tongue and inside mouth, rash on palms, soles of feet, and buttocks, loss of appetite, irritability in infants
■ MEDICATIONS	No specific antiviral; Acetaminophen/Ibuprofen for fever and pain, topical anesthetic mouthwash for mouth pain, ensure adequate hydration
■ HOME REMEDIES	Cold beverages (popsicles, cold milk), saltwater gargles for older children, cold soft foods, rinsing mouth after meals

■ DIET & NUTRITION	Cold soft foods (ice cream, cold yogurt, cold applesauce), avoid acidic, salty, or spicy foods, cold liquids, popsicles, smoothies
■■ PRECAUTIONS	Frequent handwashing, disinfect toys and surfaces, keep child home from school/daycare, avoid sharing utensils, most contagious first week
■ WHEN TO GO TO HOSPITAL	Severe dehydration (no urination, dry mouth), high fever, confusion, rapid breathing, unusually drowsy (rare viral encephalitis complication)
■■ RECOVERY TIME	7-10 days

Whooping Cough (Pertussis)

Category: Children's Disease

■ APPLICABLE TO	All Ages (most severe in infants under 1 year)
■ SYMPTOMS	Starts like cold, progresses to severe coughing fits with 'whoop' sound when inhaling, vomiting after coughing, exhaustion, infants may turn blue/stop breathing, cough lasts weeks to months
■ MEDICATIONS	Azithromycin or Clarithromycin (antibiotics - most effective early), Erythromycin, same antibiotics for household contacts; hospitalization for infants
■ HOME REMEDIES	Supportive care, plenty of fluids, small frequent meals (vomiting), humidifier, rest, avoid smoke and dust triggers
■ DIET & NUTRITION	Small frequent meals to avoid vomiting triggers, soft foods, plenty of fluids, high calorie foods to maintain nutrition
■■ PRECAUTIONS	DTaP vaccine (children) and Tdap vaccine (adults, especially pregnant women), isolate infected person, treat household contacts, booster vaccine every 10 years
■ WHEN TO GO TO HOSPITAL	Infants under 1 year should be hospitalized, turning blue during cough, difficulty breathing, severe dehydration, cough lasting more than 2 weeks
■■ RECOVERY TIME	100-day cough; weeks to months for complete resolution

Croup

Category: Children's Disease

■ APPLICABLE TO	Infants and Children 6 months - 3 years
■ SYMPTOMS	Distinctive barking cough (like a seal), hoarse voice, stridor (harsh breathing sound), low-grade fever, symptoms worse at night, mild cold symptoms preceding
■ MEDICATIONS	Dexamethasone (single dose oral steroid), Epinephrine (racemic) nebulization for severe cases, Acetaminophen for fever/discomfort, cool mist humidifier
■ HOME REMEDIES	Cool night air (take child outside in cool air), steam from shower in closed bathroom, remain calm (anxiety worsens stridor), hydration, comfort and reassurance

■ DIET & NUTRITION	Cool popsicles, cold beverages soothe the airway, plenty of fluids, soft easy-to-swallow foods
■■ PRECAUTIONS	Keep child calm, avoid agitating child, cool air helps immediately, humidifier in bedroom, keep vaccinations current (some viruses causing croup are vaccine-preventable)
■ WHEN TO GO TO HOSPITAL	Stridor at rest, severe respiratory distress, bluish/pale skin, drooling or difficulty swallowing, extremely agitated or exhausted, symptoms not improving with home measures
■■ RECOVERY TIME	3-7 days; peaks around day 2

Febrile Seizures

Category: Children's Disease

■ APPLICABLE TO	Children 6 months - 5 years
■ SYMPTOMS	Convulsions triggered by fever, whole body stiffening and jerking, loss of consciousness, eye rolling, brief cessation of breathing, usually lasts less than 5 minutes, child drowsy afterward
■ MEDICATIONS	Simple febrile seizures don't require ongoing medication; Rectal Diazepam (Diastat) for prolonged seizures; Treat underlying fever cause; Antipyretics (Acetaminophen, Ibuprofen) to reduce fever
■ HOME REMEDIES	Prevent injury during seizure (clear area, side position), tepid sponging to reduce fever, stay calm, time the seizure
■ DIET & NUTRITION	No specific diet; maintain good nutrition and hydration; breastfeeding continuation if applicable
■■ PRECAUTIONS	During seizure: protect from injury, don't restrain, don't put anything in mouth, place on side; call emergency if more than 5 minutes; control fevers promptly
■ WHEN TO GO TO HOSPITAL	Seizure lasting more than 5 minutes, not recovering within 1 hour, second seizure same day, difficulty breathing, seizure in child under 6 months, first-time seizure
■■ RECOVERY TIME	Single febrile seizure resolves in minutes; recurrence risk 30-50%; most children outgrow by age 5-6

Otitis Media (Ear Infection)

Category: Children's Disease

■ APPLICABLE TO	Infants and Children
■ SYMPTOMS	Ear pain (babies tug at ears), fever, irritability, difficulty sleeping, fluid drainage from ear, difficulty hearing, balance problems, fussiness in infants

■ MEDICATIONS	Watchful waiting for 48-72 hours in mild cases; Amoxicillin (first-line antibiotic), Amoxicillin-Clavulanate if resistant, Acetaminophen/Ibuprofen for pain, Ear drops for pain relief
■ HOME REMEDIES	Warm compress over ear, elevate head during sleep, olive oil ear drops (warm), breastfeeding continues, ginger-garlic in warm oil ear drops (folk remedy)
■ DIET & NUTRITION	Continue normal diet, extra fluids, breastfeeding is protective, avoid lying flat with bottle (increases ear infection risk in infants)
■■ PRECAUTIONS	Breastfeed if possible (protective), avoid bottle feeding lying down, avoid secondhand smoke, ensure vaccinations (PCV, flu vaccine), address allergies, consider ear tubes if recurrent
■ WHEN TO GO TO HOSPITAL	High fever, severe pain, swelling behind ear (mastoiditis), facial weakness, dizziness, speech/hearing delay, drainage from ear
■■ RECOVERY TIME	Mild: 3-5 days with or without antibiotics; with antibiotics improvement in 2-3 days

Roseola Infantum

Category: Children's Disease

■ APPLICABLE TO	Infants 6 months - 2 years
■ SYMPTOMS	High sudden fever (103-105°F) for 3-5 days, then fever breaks and distinctive rose-pink rash appears on trunk spreading to neck/arms (non-itchy), febrile seizure possible during high fever phase
■ MEDICATIONS	Acetaminophen or Ibuprofen for fever (age-appropriate dosing), no antiviral needed, supportive care only
■ HOME REMEDIES	Tepid sponge bath for high fever, plenty of fluids (breastmilk/formula for infants), cool light clothing, rest, comfort
■ DIET & NUTRITION	Continue breastfeeding or formula, age-appropriate soft foods, extra fluids
■■ PRECAUTIONS	Control fever to prevent febrile seizures, keep child comfortable, reassure parents that rash after fever is expected and benign
■ WHEN TO GO TO HOSPITAL	Febrile seizure occurs, severe breathing difficulty, extreme lethargy, rash spreading or changing character, fever lasting more than 8 days
■■ RECOVERY TIME	7-10 days total (fever 3-5 days, rash 1-2 days)

ADHD (Attention Deficit Hyperactivity Disorder)

Category: Children's Neurological

■ APPLICABLE TO	Children (diagnosed typically 3-12 years), can persist into adulthood
■ SYMPTOMS	Inattention: difficulty focusing, easily distracted, forgetful, loses things. Hyperactivity/Impulsivity: excessive talking, can't sit still, interrupts, acts without thinking, difficulty waiting turn

■ MEDICATIONS	Stimulants (first-line): Methylphenidate (Ritalin, Concerta), Amphetamines (Adderall, Vyvanse); Non-stimulants: Atomoxetine (Strattera), Guanfacine, Clonidine; Behavioral therapy combined
■ HOME REMEDIES	Structured routine, organized environment, homework breaks every 20-30 minutes, omega-3 supplementation, reduce sugar and artificial additives, physical exercise daily
■ DIET & NUTRITION	Balanced protein-rich breakfast, omega-3 rich foods (fish, walnuts), avoid artificial food dyes and preservatives (may worsen symptoms), limit sugar
■■ PRECAUTIONS	Behavioral therapy essential alongside medication, communicate with school for accommodations (IEP/504 plan), consistent boundaries, positive reinforcement, monitor medication effects
■ WHEN TO GO TO HOSPITAL	Psychiatric evaluation for proper diagnosis, medication side effects (loss of appetite, insomnia, growth concerns), mood changes on medication, self-harm thoughts
■■ RECOVERY TIME	Lifelong condition; symptoms often improve with maturation; management improves quality of life significantly

Autism Spectrum Disorder (ASD)

Category: Children's Neurological

■ APPLICABLE TO	Children (typically diagnosed before age 3)
■ SYMPTOMS	Difficulty with social interaction and communication; limited eye contact; delay in speech or language development; repetitive behaviors (hand-flapping, rocking, lining up objects); restricted interests; sensory sensitivities (over- or under-reactive to sounds, textures, lights); resistance to routine changes
■ MEDICATIONS	No medication treats core autism; medications for associated conditions: Risperidone or Aripiprazole (irritability, aggression), SSRIs (anxiety, OCD behaviors), Methylphenidate (ADHD symptoms), Melatonin (sleep disturbances); Behavioral therapy is primary treatment
■ HOME REMEDIES	Structured consistent daily routine, visual schedules, sensory integration activities, communication aids (picture cards — PECS), social stories, family-based interventions, minimize sensory triggers, occupational therapy at home
■ DIET & NUTRITION	Gluten-free casein-free diet (some families report improvement — limited scientific evidence), omega-3 fatty acids (brain development), magnesium and B6 supplementation (some evidence), avoid artificial additives; address nutritional deficiencies (many ASD children are picky eaters)
■■ PRECAUTIONS	Early intervention is critical (before age 5 for best outcomes), IEP (Individual Education Plan) at school, multidisciplinary team approach (speech therapy, OT, ABA therapy), carer respite support, safe home environment, wandering prevention

■ WHEN TO GO TO HOSPITAL	Developmental regression (loss of previously acquired skills), seizure evaluation (30% of ASD children develop epilepsy), psychiatric evaluation for severe behaviors, nutritional assessment if very selective eating
■■ RECOVERY TIME	Lifelong condition; early intensive behavioral intervention (ABA) significantly improves outcomes; many individuals lead fulfilling independent lives

Childhood Asthma

Category: Children's Respiratory

■ APPLICABLE TO	Children (can start in infancy)
■ SYMPTOMS	Recurrent wheezing, coughing (especially at night or with exercise), shortness of breath, chest tightness; symptoms triggered by allergens (dust, pets, pollen), exercise, cold air, viral infections, smoke; may have recurrent 'chest infections' that are actually asthma
■ MEDICATIONS	Rescue: Salbutamol (Ventolin) inhaler via spacer for acute attacks; Controller: Inhaled corticosteroids (Budesonide — Pulmicort, Fluticasone — Flixotide) with spacer daily; Montelukast (Singulair) for mild persistent; step-up treatment for poor control
■ HOME REMEDIES	Identify and avoid triggers, HEPA air purifiers in bedroom, remove carpets and stuffed animals from bedroom, mattress and pillow dust-mite covers, honey for cough (over 1 year old), steam with eucalyptus (caution in young children), breathing exercises
■ DIET & NUTRITION	Omega-3 rich foods (fish, walnuts), Mediterranean diet (reduces asthma risk), vitamin D rich foods, avoid sulfite-containing foods (dried fruits, wine), magnesium-rich foods, avoid food triggers if identified
■■ PRECAUTIONS	Written asthma action plan (green/yellow/red zones), always keep rescue inhaler accessible, teach correct inhaler technique (spacer for children), annual flu vaccine, enrol in school asthma program, avoid secondhand smoke
■ WHEN TO GO TO HOSPITAL	Not responding to 4-6 puffs of Salbutamol with spacer in 20 minutes, peak flow below 50% of best, difficulty speaking in full sentences, blue lips or fingertips, extreme distress or exhaustion
■■ RECOVERY TIME	Chronic condition; 50-80% of children improve significantly or outgrow asthma by adolescence; requires ongoing management

Neonatal Jaundice (Newborn Jaundice)

Category: Children's Disease

■ APPLICABLE TO	Newborns (first 2-3 weeks of life)
■ SYMPTOMS	Yellowing of skin starting from face progressing to chest, abdomen, then limbs; yellow tint to whites of eyes; baby may appear sleepy, feed poorly, have high-pitched cry in severe cases; physiological jaundice peaks days 3-5 then resolves; pathological jaundice appears within first 24 hours

■ MEDICATIONS	Phototherapy (bili lights — light therapy for moderate jaundice); Exchange transfusion (severe jaundice with very high bilirubin — specialized NICU procedure); Intravenous immunoglobulin (IVIG) for hemolytic disease; encourage frequent breastfeeding (8-12 times/day)
■ HOME REMEDIES	Frequent breastfeeding (prevents severe jaundice by increasing bowel movements); indirect morning sunlight exposure on skin (not direct harsh sunlight); ensure baby is well-hydrated; no home treatment substitutes medical phototherapy for significant jaundice
■ DIET & NUTRITION	Continue breastfeeding frequently (minimum 8-12 times per day), supplement with formula if breast milk supply is insufficient and jaundice is worsening, adequate fluid intake for mother if breastfeeding
■■ PRECAUTIONS	Bilirubin level check before hospital discharge and at day 3-5 of life, know warning signs of severe jaundice (extreme sleepiness, poor feeding, high-pitched cry, arched back), follow up with pediatrician as scheduled
■ WHEN TO GO TO HOSPITAL	Jaundice appearing in first 24 hours of life (always pathological), jaundice reaching legs and feet, bilirubin above treatment threshold on nomogram, poor feeding, extreme sleepiness, high-pitched cry, pale stools, dark urine
■■ RECOVERY TIME	Physiological jaundice: resolves by 2 weeks in term babies, 3 weeks in preterm; phototherapy works within 1-2 days

Childhood Obesity

Category: Children's Disease

■ APPLICABLE TO	Children and Adolescents
■ SYMPTOMS	BMI-for-age above 95th percentile (obese) or 85th-95th percentile (overweight); excessive fat especially around abdomen; breathlessness with normal activity; fatigue; low self-esteem; social difficulties; possible associated conditions: high BP, high cholesterol, early Type 2 diabetes, sleep apnea, orthopedic issues
■ MEDICATIONS	Lifestyle modification is primary treatment; Metformin (if Type 2 diabetes or pre-diabetes develops); Semaglutide (Wegovy) approved for adolescents 12+ years; Orlistat (for adolescents above 12); treat associated conditions; bariatric surgery only in extreme cases with severe comorbidities
■ HOME REMEDIES	Family-based lifestyle changes (most effective), limit screen time to 2 hours/day for children, increase physical play and outdoor activity to 60 min/day, family meals together, role modeling healthy habits, cook at home, involve child in cooking
■ DIET & NUTRITION	Reduce ultra-processed foods and sugary drinks; increase fruits, vegetables, whole grains, lean protein; appropriate portion sizes; eliminate sweet beverages (use water); family healthy eating approach — do not single out obese child; structured meal times

■■ PRECAUTIONS	Whole-family approach (not just the child), avoid stigmatizing language, screen for depression and bullying, regular pediatrician monitoring of BP/blood sugar/lipids, school-based programs, limit marketing exposure to junk food, adequate sleep
■ WHEN TO GO TO HOSPITAL	Metabolic complications (diabetes, hypertension, sleep apnea), BMI extremely elevated, psychological impact severe, specialty obesity clinic referral
■■ RECOVERY TIME	Gradual sustainable lifestyle changes over months to years; aim for weight maintenance (not loss) in growing children — allow growth to reduce BMI naturally

Juvenile Idiopathic Arthritis (JIA)

Category: Children's Disease

■ APPLICABLE TO	Children Under 16 Years
■ SYMPTOMS	Joint pain, swelling, warmth, and stiffness (especially morning stiffness lasting more than 30 minutes); limping; joint deformity with long-standing disease; fatigue; in systemic JIA: high spiking fever, salmon-pink rash, enlarged lymph nodes; uveitis (eye inflammation — may be silent but cause blindness if untreated)
■ MEDICATIONS	NSAIDs (Naproxen, Ibuprofen); Methotrexate (cornerstone DMARD for polyarticular JIA); Biologics: Etanercept, Adalimumab, Abatacept, Tocilizumab (for systemic JIA); Corticosteroids (short-term flares or eye drops for uveitis); physical and occupational therapy
■ HOME REMEDIES	Warm baths for morning stiffness, gentle stretching exercises, hot/cold packs for joints, hydrotherapy (pool exercises — gentle on joints), adequate rest balanced with activity, massage therapy, omega-3 supplementation
■ DIET & NUTRITION	Anti-inflammatory diet (omega-3 rich foods, colorful fruits and vegetables), calcium and Vitamin D for bone health (medications can cause bone loss), adequate calories for growth, avoid inflammatory foods (refined carbs, processed meats)
■■ PRECAUTIONS	Regular ophthalmology check-ups (every 3-6 months for uveitis — may be asymptomatic!), dental monitoring (TMJ involvement), physiotherapy to prevent contractures, school accommodations, vaccines before starting biologics (live vaccines contraindicated), monitor growth
■ WHEN TO GO TO HOSPITAL	Eye redness or vision changes, sudden severe joint swelling, high fever with rash (systemic JIA flare), suspected Macrophage Activation Syndrome (MAS — life-threatening), medication side effects
■■ RECOVERY TIME	Variable; many children achieve remission by adulthood; some have persistent disease into adult life; early aggressive treatment improves outcomes

Bedwetting (Nocturnal Enuresis)

Category: Children's Disease

■ APPLICABLE TO	Children (typically 5-12 years)
■ SYMPTOMS	Involuntary urination during sleep in children who are otherwise toilet trained; primary enuresis (child never achieved dry nights); secondary enuresis (previously dry child starts wetting again — may indicate stress, infection, or diabetes); not a behavioral problem
■ MEDICATIONS	Desmopressin (DDAVP) nasal spray or tablet (reduces urine production at night); Imipramine (TCAs — second line, less preferred); treat underlying UTI or constipation if present; bedwetting alarm (first-line behavioral — most effective long-term)
■ HOME REMEDIES	Bedwetting alarm (moisture sensor — very effective, takes 4-6 weeks), restrict fluids 1-2 hours before bedtime, ensure child urinates just before bed, positive reinforcement and praise (NEVER punish), waterproof mattress protector, lifting (wake child to urinate at parents bedtime)
■ DIET & NUTRITION	Reduce fluid intake in evening hours (NOT overall restriction — important for health), avoid caffeine (cola, chocolate) in evening, ensure adequate daytime fluid intake, treat constipation (contributes to bedwetting) with high fiber diet
■■ PRECAUTIONS	Be patient and supportive — children cannot control this; avoid shame or punishment; rule out UTI or constipation; screen for stress or bullying (secondary enuresis); most children outgrow bedwetting naturally by adolescence
■ WHEN TO GO TO HOSPITAL	Secondary enuresis (new onset after 6+ months dry), associated daytime accidents, painful urination, excessive thirst and weight loss (diabetes), urinary tract infection, suspected neurological cause
■■ RECOVERY TIME	Spontaneous resolution in 15% per year; with bedwetting alarm: 60-70% achieve dryness; Desmopressin works immediately but relapse common

Kawasaki Disease

Category: Children's Disease

■ APPLICABLE TO	Children under 5 years
■ SYMPTOMS	Fever lasting 5+ days (hallmark), rash on trunk, red cracked lips and strawberry tongue, red eyes (conjunctivitis without discharge), redness and swelling of palms/soles followed by peeling of skin from fingers and toes, swollen lymph node in neck (usually one side); complication: coronary artery aneurysms if untreated
■ MEDICATIONS	URGENT treatment: High-dose Aspirin (anti-inflammatory) during fever phase; IV Immunoglobulin (IVIG) — single high dose (2g/kg) — reduces coronary aneurysm risk from 25% to 3-5% if given within 10 days; Low-dose Aspirin continued for 6-8 weeks (antiplatelet); Infliximab for IVIG-resistant cases
■ HOME REMEDIES	Supportive: keep child comfortable, cool compresses for high fever, adequate hydration, gentle moisturizer for peeling skin; NOTE: this condition requires urgent medical treatment — home remedies are supplementary only
■ DIET & NUTRITION	Adequate fluids and nutrition during fever, soft easily digestible foods, high-calorie foods if reduced appetite during illness, lip balm for cracked lips

■■ PRECAUTIONS	Early diagnosis is critical (coronary complications preventable with IVIG within 10 days); echocardiogram to check coronary arteries; follow-up cardiology monitoring; long-term aspirin if coronary aneurysms present; avoid live vaccines within 11 months of IVIG
■ WHEN TO GO TO HOSPITAL	Any child with fever 5+ days with 3 of the 5 classic features — immediate pediatric evaluation and likely hospitalization for IVIG treatment; this is a pediatric emergency if untreated
■■ RECOVERY TIME	Acute phase: 10-14 days; skin peeling continues 2-3 weeks; coronary monitoring for months to years if aneurysms present; majority recover fully with prompt treatment

Intussusception

Category: Children's Disease

■ APPLICABLE TO	Infants and Children 3 months to 6 years (peak 5-10 months)
■ SYMPTOMS	Sudden onset severe colicky abdominal pain (infant draws up legs and screams for 15-20 minutes then appears normal between episodes — episodic pattern), vomiting, pale and lethargic appearance, red currant jelly stool (blood and mucus — late sign), sausage-shaped abdominal mass, progressive lethargy as condition worsens
■ MEDICATIONS	MEDICAL EMERGENCY — requires immediate hospital treatment: Air enema reduction (first-line — success rate 80-95%); Hydrostatic enema; Surgical reduction if enema fails or perforation suspected; IV fluids and pain management; Antibiotics if bowel compromise suspected
■ HOME REMEDIES	NO home treatment — this is a surgical emergency in children. If suspected, go to emergency department immediately. Do not give anything by mouth
■ DIET & NUTRITION	Post-reduction: IV fluids initially, gradual reintroduction of feeds starting with clear fluids then breast milk/formula, advance diet as tolerated
■■ PRECAUTIONS	Seek emergency care immediately for any infant with episodic severe abdominal pain and drawing up legs; 10% recurrence rate after air enema reduction (watch for recurrence in first 24-48 hours)
■ WHEN TO GO TO HOSPITAL	IMMEDIATELY — this is a pediatric emergency. Any infant with episodic abdominal pain, vomiting, and lethargy needs emergency ultrasound to rule out intussusception
■■ RECOVERY TIME	Post-enema reduction: 24-48 hours observation; surgical cases: 3-5 days hospital stay; full recovery expected with prompt treatment

Pyloric Stenosis

Category: Children's Disease

■ APPLICABLE TO	Infants 2-8 weeks of age (First-born Males most common)

■ SYMPTOMS	Projectile vomiting after every feed (forceful, shoots across room), hunger immediately after vomiting (baby feeds eagerly again), progressive weight loss, dehydration, olive-sized mass palpable in upper abdomen, visible peristaltic waves across abdomen, metabolic alkalosis (from losing stomach acid in vomit)
■ MEDICATIONS	Pyloromyotomy (Ramstedt procedure) — surgical treatment: longitudinal incision through pyloric muscle; IV fluid correction of electrolyte imbalance before surgery; PPI medications are NOT effective and operation is required; early surgery has excellent outcomes
■ HOME REMEDIES	No effective home treatment — this condition requires surgery. While waiting for evaluation: feed smaller volumes more frequently, burp after every small feed, keep upright after feeds
■ DIET & NUTRITION	Post-operative: small feeds of breast milk or formula start 6-12 hours after surgery, gradually increasing volumes over 24-48 hours; full feeds by 24-48 hours post-surgery in most cases
■■ PRECAUTIONS	Recognize early signs in infants 2-8 weeks (progressive non-bilious projectile vomiting); early surgical repair before severe malnutrition; pyloric stenosis runs in families (50% risk if parent had it)
■ WHEN TO GO TO HOSPITAL	Any infant with persistent projectile vomiting after feeds — urgent pediatric evaluation; ultrasound of pylorus is diagnostic (pyloric muscle thickness above 4mm, channel length above 16mm)
■■ RECOVERY TIME	Pyloromyotomy: full recovery in 24-48 hours; excellent long-term prognosis; vomiting resolves within days after surgery

Meningitis (Bacterial — Childhood)

Category: Children's Disease

■ APPLICABLE TO	Infants and Children (also Adults)
■ SYMPTOMS	Classic triad: stiff neck (meningism), high fever, severe headache. Also: photophobia, phonophobia, nausea/vomiting, altered consciousness, seizures; Non-blanching petechial or purpuric rash (dark red/purple spots that do NOT fade with glass pressure — bacterial meningococcal rash is EMERGENCY sign); Infants: bulging fontanelle, high-pitched cry, poor feeding, limpness
■ MEDICATIONS	LIFE-SAVING EMERGENCY: IV Ceftriaxone or Cefotaxime immediately (first dose before LP if suspected); IV Dexamethasone (reduces hearing loss complication); IV fluids; ICU care; Treatment of complications (seizures, raised ICP, DIC); Prevention: Meningococcal ACWY vaccine, MenB vaccine, Hib vaccine, PCV vaccine
■ HOME REMEDIES	NO home treatment. This is a life-threatening emergency. Call 108/911 immediately if suspected. Do not wait for rash to develop — start antibiotics before full diagnosis confirmed
■ DIET & NUTRITION	IV nutritional support during acute illness; nasogastric feeds if unable to take oral; normal feeds as recovery progresses

■■ PRECAUTIONS	Vaccination is the ONLY prevention: complete Hib, PCV, MenACWY, MenB vaccine schedules; know the glass test for non-blanching rash; seek emergency care immediately — bacterial meningitis kills within hours; close contacts need antibiotic prophylaxis (Rifampicin or Ciprofloxacin)
■ WHEN TO GO TO HOSPITAL	ANY suspected meningitis — CALL EMERGENCY IMMEDIATELY. Non-blanching rash alone is an emergency even before other symptoms develop. Time is brain and life
■■ RECOVERY TIME	Intensive care for 1-2 weeks; full recovery 4-8 weeks; potential permanent sequelae: hearing loss (15%), brain damage, limb amputation (meningococcal septicemia); mortality 5-15% even with treatment

Childhood Constipation

Category: Children's Disease

■ APPLICABLE TO	Children (extremely common)
■ SYMPTOMS	Less than 3 bowel movements per week, large or painful stools, blood on stool or toilet paper (hard stool causing anal fissure), soiling (overflow encopresis — liquid stool leaking around impacted hard stool, mistaken for diarrhea), stomach pain, poor appetite, palpable stool mass in abdomen, withholding behavior (squeezing buttocks, getting on tip-toes)
■ MEDICATIONS	Disimpaction first: Polyethylene glycol (PEG/Movicol — gold standard) at disimpaction dose, or enema; Maintenance: Low-dose PEG daily for months (this is not laxative dependence — bowel needs time to recover normal tone and sensation); Lactulose (alternative); Toilet training and behavioral therapy
■ HOME REMEDIES	High fluid intake (most important), warm bath to relax sphincter, abdominal massage (clockwise direction), bicycle legs exercise for infants, avoid cow's milk if intolerant, prune or pear juice for infants, dietary fiber
■ DIET & NUTRITION	High fiber foods (whole grains, fruits — especially prunes, kiwi, pear, berries; vegetables, legumes), at least 5 portions fruit and veg daily, adequate water, prune juice for infants, avoid excess cow's milk (can be constipating), limit processed foods
■■ PRECAUTIONS	Do not punish for soiling (child cannot control overflow encopresis), positive toilet routine (scheduled sits after meals for 5-10 minutes), footstool to support feet on toilet (proper squatting position), continue laxatives for months (premature stopping causes relapse), regular follow-up
■ WHEN TO GO TO HOSPITAL	No meconium in first 48 hours of life (Hirschsprung's disease), abdominal distension, constipation from birth, failure to thrive with constipation, severe refractory constipation, suspected Hirschsprung's disease
■■ RECOVERY TIME	6-24 months of treatment often needed; majority resolve with sustained treatment; 50% relapse if treatment stopped too soon

Tonsillitis

Category: Children's Disease

■ APPLICABLE TO	Children and Young Adults
■ SYMPTOMS	Sore throat, difficulty and pain swallowing, red swollen tonsils (often with white or yellow patches/pus), high fever (38-40°C), swollen tender lymph nodes in neck, bad breath, muffled or 'hot potato' voice, loss of appetite; recurrent tonsillitis: 5+ episodes per year for 2 years or 7+ in one year
■ MEDICATIONS	Viral (most common in children): Supportive — Paracetamol/Ibuprofen, throat lozenges; Bacterial (Group A Strep): Amoxicillin or Penicillin for 10 days (complete full course — incomplete treatment leads to rheumatic fever); Dexamethasone (single dose) for severe swelling; Tonsillectomy for recurrent tonsillitis
■ HOME REMEDIES	Warm salt water gargling (1/4 tsp salt in glass of warm water, several times daily), honey-lemon-ginger tea, ice chips or popsicles (soothing), cold ice cream or yogurt, steam inhalation, adequate rest, avoid cold drinks
■ DIET & NUTRITION	Cold soft foods (ice cream, cold yogurt, smoothies, pudding, cold jelly), warm soups and broths, ice pops, avoid hard crunchy foods, adequate fluids, honey and warm tea, avoid spicy and acidic foods
■■ PRECAUTIONS	Complete full antibiotic course if prescribed (prevents rheumatic fever and nephritis), avoid sharing utensils, hand hygiene, adequate rest from school, throat swab culture before antibiotics if uncertain (RADT test for strep)
■ WHEN TO GO TO HOSPITAL	Peritonsillar abscess (asymmetric tonsil swelling, uvula deviation, drooling, muffled voice — quinsy — requires drainage), airway compromise, inability to swallow any fluids, high fever not responding to antipyretics, recurrent tonsillitis (tonsillectomy evaluation)
■■ RECOVERY TIME	Viral tonsillitis: 7-10 days; Bacterial with antibiotics: 3-5 days; post-tonsillectomy: full recovery 1-2 weeks

Pregnancy & Women's Specific Conditions

16 Conditions Covered — 8 Guidance Fields Each

Morning Sickness (Nausea and Vomiting of Pregnancy)

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women (especially 1st trimester)
■ SYMPTOMS	Nausea, vomiting (not only morning), food aversions, heightened sense of smell, typically 6-12 weeks gestation, usually improves by 16-20 weeks
■ MEDICATIONS	Vitamin B6 (Pyridoxine), Doxylamine (Unisom + B6 = Diclegis/Bonjesta), Ginger supplements, Ondansetron (severe cases, after 1st trimester discussion with OB), Promethazine
■ HOME REMEDIES	Ginger (tea, ginger ale, ginger candies, ginger capsules), lemon (smell or tea), peppermint tea, sea-bands/acupressure wristbands, crackers before getting up, cold foods, chamomile
■ DIET & NUTRITION	Small frequent meals every 1-2 hours, bland dry foods (crackers, toast, plain rice), cold foods less nauseating, protein-rich snacks, avoid spicy/fatty/strong-smelling foods, stay hydrated with small sips
■■ PRECAUTIONS	Rest well, avoid triggers (strong smells, greasy foods), eat before getting out of bed, take prenatal vitamins at night, stay hydrated, differentiate from Hyperemesis Gravidarum
■ WHEN TO GO TO HOSPITAL	Unable to keep any food/fluid down for 24 hours (Hyperemesis Gravidarum), weight loss, dark urine (dehydration), blood in vomit, signs of dehydration
■■ RECOVERY TIME	Usually resolves by week 16-20; some women have symptoms throughout pregnancy

Gestational Diabetes

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women (usually 24-28 weeks)
■ SYMPTOMS	Usually asymptomatic (detected on routine screening); Sometimes: increased thirst, frequent urination, fatigue, blurred vision. Diagnosed by OGTT (Oral Glucose Tolerance Test)
■ MEDICATIONS	Diet and exercise (first-line), Metformin (considered safe in pregnancy), Insulin therapy (safest - most commonly needed if diet fails), regular glucose monitoring
■ HOME REMEDIES	30-minute walk after meals, light prenatal exercise, portion-controlled meals, blood sugar monitoring as directed by OB

■ DIET & NUTRITION	Controlled carbohydrate diet, complex carbs over simple, lean protein with each meal, non-starchy vegetables, limit fruit portions, spread carbs throughout day, avoid sugary drinks and refined carbs
■■ PRECAUTIONS	Regular prenatal visits, fetal monitoring (kick counts, ultrasound, NST), monitor blood sugar 4 times daily, control weight gain, plan for delivery at hospital with NICU capability if poorly controlled
■ WHEN TO GO TO HOSPITAL	Blood sugar consistently above 95 mg/dL fasting or 120 mg/dL 2 hours after meals, fetal movement changes, signs of preeclampsia, polyhydramnios
■■ RECOVERY TIME	Usually resolves after delivery; 50% risk of developing T2DM within 10 years; annual glucose screening recommended

Preeclampsia

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women (after 20 weeks)
■ SYMPTOMS	High blood pressure (above 140/90) + protein in urine; severe headache, upper right abdominal pain, swelling of face/hands, sudden weight gain, visual disturbances (flashing lights, blurred vision), nausea
■ MEDICATIONS	Antihypertensives (Labetalol, Nifedipine, Methyldopa - safe in pregnancy), Magnesium sulfate (prevents eclamptic seizures), Low-dose Aspirin (preventive, started early pregnancy in high-risk women), delivery is definitive treatment
■ HOME REMEDIES	Rest (left lateral position improves blood flow), monitor BP at home, kick counts, NO home treatment for established preeclampsia - requires medical management
■ DIET & NUTRITION	Low sodium diet, adequate protein intake, good hydration, calcium-rich foods (may be preventive), no alcohol or caffeine, avoid processed foods
■■ PRECAUTIONS	Regular prenatal BP checks, urine protein testing, low-dose aspirin from 12-16 weeks if high risk, immediate medical attention for warning signs, know the signs of eclampsia
■ WHEN TO GO TO HOSPITAL	URGENTLY if: severe headache, visual changes, epigastric pain, seizures (eclampsia), sudden severe swelling, decreased fetal movement, systolic BP above 160 or diastolic above 110
■■ RECOVERY TIME	Delivery is the only cure; most cases resolve within 6 weeks postpartum; may need BP medication for weeks after

Iron Deficiency Anemia in Pregnancy

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women

■ SYMPTOMS	Fatigue, weakness, shortness of breath, pale skin and gums, dizziness, rapid heartbeat, cold hands/feet, headache, brittle nails, cravings for non-food items (pica)
■ MEDICATIONS	Oral iron supplements: Ferrous sulfate 325mg (elemental iron 65mg) once to thrice daily, Ferrous gluconate (better tolerated), Intravenous iron (severe cases), Continue throughout pregnancy and postpartum
■ HOME REMEDIES	Take iron with Vitamin C to enhance absorption, avoid taking with calcium, coffee, or tea; cast iron cookware increases dietary iron, avoid inhibitors with meals
■ DIET & NUTRITION	Iron-rich foods: spinach, lentils, beans, tofu, fortified cereals, red meat, chicken, fish; enhance absorption with Vitamin C (orange juice, tomatoes) at same meal; avoid tea/coffee/calcium with iron-rich meals
■■ PRECAUTIONS	Take iron supplements as prescribed, take with vitamin C not with tea/coffee, address constipation from iron (increase fiber, fluids), regular Hb monitoring, prenatal vitamins
■ WHEN TO GO TO HOSPITAL	Hb below 7 g/dL, symptoms of cardiac strain, inability to tolerate oral iron, failure to respond to oral supplements, IV iron therapy needed
■■ RECOVERY TIME	Hb improves in 2-4 weeks of treatment; continue 3 months after Hb normalizes

Gestational Hypertension

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women (after 20 weeks)
■ SYMPTOMS	BP above 140/90 on two occasions 4 hours apart after 20 weeks, no proteinuria, no other features of preeclampsia; usually asymptomatic
■ MEDICATIONS	Antihypertensives if BP above 160/110 (Labetalol, Nifedipine, Methyldopa), close monitoring, may progress to preeclampsia
■ HOME REMEDIES	Rest in left lateral position, monitor BP regularly at home, moderate activity, salt reduction, stress reduction
■ DIET & NUTRITION	Moderate sodium restriction, balanced diet, adequate hydration, calcium-rich foods, avoid caffeine and alcohol
■■ PRECAUTIONS	Twice-weekly BP monitoring, weekly prenatal visits, fetal surveillance (NST, ultrasound), watch for signs of preeclampsia progression
■ WHEN TO GO TO HOSPITAL	BP above 160/110, development of protein in urine, severe headache, visual disturbances, epigastric pain, decreased fetal movement
■■ RECOVERY TIME	Resolves after delivery; BP monitoring for 3-6 weeks postpartum

Hyperemesis Gravidarum

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women (1st trimester, sometimes persists)
■ SYMPTOMS	Severe persistent nausea and vomiting (more than 3 times/day), dehydration, weight loss more than 5% pre-pregnancy weight, ketosis, electrolyte imbalance, inability to work/function
■ MEDICATIONS	IV fluid replacement, Ondansetron IV, Promethazine, Metoclopramide, Corticosteroids (severe refractory cases), Vitamin B1 (Thiamine) supplementation, Total parenteral nutrition (severe)
■ HOME REMEDIES	Ginger (all forms), acupressure P6 point, small frequent cold foods, highly palatable individual foods, rest, avoid triggers
■ DIET & NUTRITION	Whatever can be tolerated (highly individualized), cold foods often better tolerated, small sips of liquid, high protein foods, avoid all triggers, nutritional supplementation
■■ PRECAUTIONS	Monitor for Wernicke's encephalopathy (Vitamin B1 deficiency), maintain hydration, emotional support (affects mental health significantly), medical leave from work often needed
■ WHEN TO GO TO HOSPITAL	Severe dehydration, weight loss above 5%, bloody vomit, inability to keep any fluids down, neurological symptoms, ketones in urine
■■ RECOVERY TIME	Typically improves by 20 weeks; 20% persist throughout pregnancy

Postpartum Depression

Category: Pregnancy/Postpartum

■ APPLICABLE TO	New Mothers (within first year postpartum)
■ SYMPTOMS	Persistent sadness, anxiety, feeling overwhelmed, crying spells, difficulty bonding with baby, changes in sleep/appetite, withdrawal from family, difficulty concentrating, feelings of worthlessness or guilt, thoughts of harming self or baby
■ MEDICATIONS	SSRIs (Sertraline - safest for breastfeeding, Paroxetine, Fluoxetine), SNRIs, Brexanolone (IV - for severe PPD, new drug), Psychotherapy (CBT, IPT) often combined with medication
■ HOME REMEDIES	Accept help from family/friends, mother-baby groups, adequate sleep (critical), regular light exercise, sunlight exposure, omega-3 supplementation, mindfulness
■ DIET & NUTRITION	Omega-3 rich foods (fish, walnuts), vitamin D rich foods, iron-rich foods (anemia common postpartum), protein-rich foods, avoid excessive caffeine and alcohol
■■ PRECAUTIONS	Screen all mothers for PPD at 2-week and 6-week visits (Edinburgh scale), partner support is critical, don't suffer alone - seek help promptly, most treatable with proper care
■ WHEN TO GO TO HOSPITAL	Thoughts of harming self or baby, hearing voices/hallucinations (postpartum psychosis - emergency), complete inability to function, suicidal thoughts
■■ RECOVERY TIME	With treatment: weeks to months; untreated can last a year or more

Urinary Tract Infections in Pregnancy

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women
■ SYMPTOMS	Burning urination, frequent urge to urinate, cloudy/foul-smelling urine, pelvic discomfort, may be asymptomatic (asymptomatic bacteriuria); if kidney infection: fever, chills, back pain
■ MEDICATIONS	Safe antibiotics in pregnancy: Nitrofurantoin (avoid in 3rd trimester), Cephalexin (Keflex), Amoxicillin-Clavulanate, Trimethoprim-Sulfamethoxazole (avoid 1st trimester); treat asymptomatic bacteriuria in pregnancy
■ HOME REMEDIES	Plenty of water (8-10 glasses), cranberry juice (prevention), urinate after intercourse, front-to-back wiping, avoid bubble baths
■ DIET & NUTRITION	Lots of water, unsweetened cranberry juice, probiotic foods (yogurt), vitamin C rich foods, avoid caffeine
■■ PRECAUTIONS	Routine urine culture at first prenatal visit, complete antibiotic course, treat even asymptomatic cases in pregnancy (risk of preterm labor), follow-up urine culture after treatment
■ WHEN TO GO TO HOSPITAL	Any fever during pregnancy, back/flank pain, no improvement after 48 hours on antibiotics, recurrent UTIs (need low-dose prophylaxis)
■■ RECOVERY TIME	3-7 days with appropriate antibiotics; retest after completion

Placenta Previa

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women (usually 2nd-3rd trimester)
■ SYMPTOMS	Painless bright red vaginal bleeding (hallmark symptom) occurring suddenly in second or third trimester; bleeding may stop and restart; no associated abdominal pain (distinguishes from placental abruption); may be discovered incidentally on routine ultrasound
■ MEDICATIONS	Hospitalization for significant bleeding; Betamethasone (corticosteroids for fetal lung maturity if preterm delivery anticipated), Magnesium sulfate (neuroprotection if delivery before 32 weeks), blood transfusion if significant blood loss, planned Caesarean section (vaginal delivery contraindicated in confirmed previa)
■ HOME REMEDIES	Complete pelvic rest (no intercourse, no internal exams, no vigorous exercise), bed rest if bleeding occurs, immediate hospital transfer for any bleeding, monitor kick counts daily
■ DIET & NUTRITION	Iron-rich foods and iron supplements (anemia prevention due to bleeding risk), folate-rich foods, adequate hydration, small frequent nutritious meals
■■ PRECAUTIONS	All women with placenta previa must plan hospital delivery with capacity for emergency C-section and blood transfusion; no internal examinations; pelvic rest; plan for early delivery if needed

■ WHEN TO GO TO HOSPITAL	ANY vaginal bleeding in pregnancy — go immediately; heavy bleeding — call emergency ambulance; contractions with placenta previa require urgent assessment
■■ RECOVERY TIME	Low-lying placenta at 20 weeks often resolves by 34-36 weeks; confirmed previa at term requires planned Caesarean section

Ectopic Pregnancy

Category: Pregnancy

■ APPLICABLE TO	Women of Reproductive Age
■ SYMPTOMS	Sharp stabbing pain on one side of lower abdomen (may radiate to shoulder — diaphragm irritation from internal bleeding), light vaginal bleeding or spotting, dizziness or fainting, nausea, feeling of rectal pressure; symptoms of internal bleeding: severe pain, pale clammy skin, rapid heart rate, shoulder tip pain
■ MEDICATIONS	Methotrexate (medical management — only for stable unruptured ectopic with low HCG); Surgical treatment: Laparoscopic salpingostomy or salpingectomy (ruptured ectopic = EMERGENCY surgery); Rh(D) immunoglobulin if Rh negative blood type
■ HOME REMEDIES	NO home treatment — ectopic pregnancy is a medical emergency when ruptured. Suspected ectopic pregnancy requires immediate medical evaluation
■ DIET & NUTRITION	Post-treatment: iron-rich foods for anemia recovery, folate for subsequent pregnancy planning, adequate nutrition; if on Methotrexate — avoid folate supplements, alcohol; avoid NSAIDs
■■ PRECAUTIONS	Early ultrasound in women with risk factors (previous ectopic, tubal surgery, PID, IUD, infertility treatment), know symptoms, any woman with positive pregnancy test and abdominal pain needs urgent evaluation to exclude ectopic
■ WHEN TO GO TO HOSPITAL	IMMEDIATELY if suspected ectopic pregnancy — especially shoulder tip pain, collapse, or severe one-sided pain; ruptured ectopic is life-threatening within hours
■■ RECOVERY TIME	Surgical recovery: 1-2 weeks; Methotrexate: HCG monitored weekly until undetectable (weeks to months); fertility implications depend on extent of tubal damage

Breast Engorgement (Postpartum)

Category: Pregnancy/Postpartum

■ APPLICABLE TO	Breastfeeding and Non-Breastfeeding New Mothers
■ SYMPTOMS	Breasts feel hard, swollen, heavy, and very tender; skin appears tight and shiny; areola may become hard making it difficult for baby to latch; low-grade fever (milk fever); breast warmth; occurs 2-5 days after delivery when milk 'comes in'

■ MEDICATIONS	Ibuprofen or Paracetamol for pain and fever; for non-breastfeeding mothers: firm supportive bra, cold compresses, avoid stimulation (no warm showers directly on breasts); for breastfeeding: frequent feeding is primary treatment; Dopamine agonists to suppress milk production in women who choose not to breastfeed
■ HOME REMEDIES	Frequent breastfeeding (8-12 times per day — every 2-3 hours); hand expression or pump before feeding to soften areola (reverse pressure softening); warm compress before feeding to help let-down; cold cabbage leaves in bra between feeds (anti-inflammatory, evidence moderate); cold compress after feeding to reduce swelling
■ DIET & NUTRITION	Stay well hydrated (important for milk production), balanced nutrition, continue prenatal vitamins, no restriction of fluids (restricting fluids does NOT reduce engorgement), lactation-supportive foods (oats, fenugreek seeds, methi)
■■ PRECAUTIONS	Ensure correct breastfeeding latch (prevents mastitis from inadequate drainage), frequent feeding rather than rigid schedule, do not restrict feeds, lactation consultant support, watch for mastitis (unilateral redness, fever, flu-like symptoms — requires antibiotics)
■ WHEN TO GO TO HOSPITAL	High fever above 38.5°C, one breast red hot and extremely painful (mastitis), flu-like symptoms (mastitis needs antibiotics), breast lump not resolving (abscess formation), baby unable to latch and breasts becoming very hard
■■ RECOVERY TIME	Physiological engorgement resolves in 24-48 hours with frequent feeding; if inadequately managed, mastitis risk increases

Pelvic Girdle Pain (PGP) in Pregnancy

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women (any trimester)
■ SYMPTOMS	Pain over the pubic symphysis (front of pelvis), pain in the sacroiliac joints (back of pelvis), pain radiating to inner thighs; worsened by walking, climbing stairs, turning in bed, standing on one leg, prolonged standing or sitting; clicking or grinding in pelvis
■ MEDICATIONS	Paracetamol (safe in pregnancy for pain); Pelvic support belt/girdle; physiotherapy; water exercises (aquanatal classes); AVOID: NSAIDs in 3rd trimester, bed rest (worsens outcome), activities that worsen pain
■ HOME REMEDIES	Pregnancy support belt, keep legs together when turning in bed or getting in and out of car, sit down when dressing (do not balance on one leg), ice pack on painful joint, rest periods, sleeping with pillow between knees
■ DIET & NUTRITION	Calcium and Vitamin D for joint and bone health, omega-3 fatty acids (anti-inflammatory), adequate magnesium, maintain healthy pregnancy weight gain
■■ PRECAUTIONS	Early physiotherapy referral (most effective treatment), use crutches or walker if needed, adapt activities to within pain limits, avoid carrying heavy bags, plan delivery position to minimize pelvic stress, PGP can worsen postpartum — continue physio

■ WHEN TO GO TO HOSPITAL	Inability to weight-bear, severe pain not improving with conservative measures, neurological symptoms (numbness, tingling in legs), suspected symphysis pubis diastasis (separation)
■■ RECOVERY TIME	Symptoms typically resolve within 3 months postpartum; early physiotherapy leads to faster recovery; small minority have persistent symptoms

Miscarriage (Spontaneous Abortion)

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women (affects 10-20% of known pregnancies)
■ SYMPTOMS	Vaginal bleeding (ranging from spotting to heavy), cramping and lower abdominal pain, passage of tissue or clots, decrease or disappearance of pregnancy symptoms; may be discovered on ultrasound without symptoms (missed miscarriage — fetus has died but no bleeding yet); most occur in first 12 weeks
■ MEDICATIONS	Expectant management (wait for natural completion — 80% effective within 2 weeks); Medical: Misoprostol (prostaglandin — causes uterine contractions to complete miscarriage); Surgical: ERPC (Evacuation of Retained Products of Conception) — most complete; Rh(D) immunoglobulin if Rh-negative blood group; Pain management: Ibuprofen or Codeine
■ HOME REMEDIES	Physical: rest and adequate pain management, use sanitary pads not tampons, avoid intercourse until bleeding stops. Emotional: acknowledge grief (miscarriage is a real bereavement), allow time to grieve, peer support groups (one in five pregnancies end in miscarriage — not alone)
■ DIET & NUTRITION	Iron-rich foods and supplements (blood loss causes anemia), folate-rich foods (for next pregnancy planning), adequate nutrition, omega-3 fatty acids, no alcohol
■■ PRECAUTIONS	Most miscarriages are due to chromosomal abnormalities — not caused by normal activities, sex, or mild exercise; investigations after 3 recurrent miscarriages; pre-pregnancy screening for antiphospholipid syndrome; folic acid 400mcg daily before conception and first trimester
■ WHEN TO GO TO HOSPITAL	Heavy bleeding (soaking more than one pad per hour for 2+ consecutive hours), severe abdominal pain, fever with miscarriage (infection/septic miscarriage — EMERGENCY), signs of ectopic pregnancy, incomplete miscarriage with retained tissue
■■ RECOVERY TIME	Physical recovery: 1-2 weeks; periods return in 4-6 weeks; emotional recovery: months to years; next pregnancy: most doctors advise one normal period before trying again; 85% of women who miscarry go on to have successful pregnancies

Premature Labor (Preterm Labor)

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women

■ SYMPTOMS	Regular uterine contractions (every 10 minutes or more frequently before 37 completed weeks), low backache (persistent dull ache), pelvic pressure (feeling baby is pressing down), change in vaginal discharge (increased, watery, mucousy, or bloody — bloody show), abdominal cramping, fluid leaking from vagina (membranes rupturing)
■ MEDICATIONS	Tocolytics (short-term — to delay delivery 48 hours for steroids): Nifedipine, Indomethacin, Atosiban; Betamethasone (steroids — 2 doses IM 24 hours apart — critically accelerates fetal lung maturity, given between 24-34 weeks); Magnesium sulfate (neuroprotection for baby if delivery before 32 weeks); Cervical cerclage if incompetent cervix; Progesterone (17-OHPC for prevention in high-risk women)
■ HOME REMEDIES	Bed rest with pelvic rest (activity restriction), hydration (dehydration can trigger contractions), left lateral lying position, avoid heavy lifting and sexual intercourse; these measures support medical treatment but do not replace it
■ DIET & NUTRITION	Adequate hydration (dehydration can cause Braxton-Hicks and preterm contractions), balanced nutrition for baby's development, omega-3 fatty acids (associated with longer gestation), adequate protein, folate
■■ PRECAUTIONS	Regular antenatal visits — cervical length measurement at 18-24 weeks in high-risk women, progesterone supplementation for short cervix, treat infections (UTI, bacterial vaginosis), quit smoking (major risk factor), manage stress, recognize symptoms early
■ WHEN TO GO TO HOSPITAL	Any signs of preterm labor before 37 weeks — go immediately; even if unsure, go to hospital for assessment; early intervention can delay delivery and allow steroid administration to mature baby's lungs
■■ RECOVERY TIME	If delivery delayed: outcome depends on gestational age at birth; babies born after 34 weeks generally do well; steroids significantly improve outcomes; NICU support for premature babies

Antenatal Depression

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women
■ SYMPTOMS	Persistent low mood or sadness lasting more than 2 weeks during pregnancy, crying for no reason, feeling hopeless or worthless, loss of interest in things previously enjoyed, changes in appetite or sleep, anxiety about baby's health or ability to parent, difficulty bonding with baby, irritability, social withdrawal, thoughts of self-harm
■ MEDICATIONS	Psychotherapy (CBT — first-line for mild-moderate), SSRIs (Sertraline, Fluoxetine — considered relatively safe in pregnancy after risk-benefit discussion with OB and psychiatrist), Mirtazapine for sleep and appetite; avoid Paroxetine (cardiac risks) and Lithium if possible; risk of untreated depression to baby must be weighed against medication risks

■ HOME REMEDIES	Regular gentle exercise (proven to improve mood in pregnancy), adequate sleep and rest, social support network, mindfulness and prenatal meditation, prenatal yoga, journaling, omega-3 supplementation (safe in pregnancy), bright light therapy for mild depression
■ DIET & NUTRITION	Omega-3 fatty acids (fish oil — safe in pregnancy, benefits fetal brain development and maternal mood), folate and B-vitamin rich foods, vitamin D, iron (anemia worsens mood), balanced nutritious diet, avoid alcohol
■■ PRECAUTIONS	All pregnant women should be screened for depression at first antenatal visit and again in third trimester (Edinburgh Postnatal Depression Scale), open communication with midwife and OB, do not suffer in silence, involve partner in support, plan for postpartum support
■ WHEN TO GO TO HOSPITAL	Suicidal thoughts, thoughts of harming self or baby, inability to care for self, severe anxiety attacks, not eating or sleeping for days, psychotic symptoms
■■ RECOVERY TIME	With CBT: improvement in 8-20 sessions; SSRIs: 4-8 weeks for effect; most women recover fully with appropriate treatment; postpartum continuation of treatment reduces PPD risk

Round Ligament Pain in Pregnancy

Category: Pregnancy

■ APPLICABLE TO	Pregnant Women (typically 2nd trimester)
■ SYMPTOMS	Sharp stabbing or dull aching pain in lower abdomen and groin, usually one-sided (can be both sides), triggered by sudden movement (rolling over in bed, standing quickly, coughing, sneezing, exercise), lasts seconds to minutes then resolves; caused by stretching of round ligaments supporting growing uterus — normal part of pregnancy
■ MEDICATIONS	Paracetamol (safest pain relief in pregnancy) for severe episodes; warm (not hot) bath; supportive pregnancy belt; no specific medical treatment required as this is a normal pregnancy symptom
■ HOME REMEDIES	Bend and flex hips toward pain before coughing/sneezing (reduces pull on ligament), pregnancy support belt worn during activity, warm (not hot) compress on painful area, rest in comfortable position, move slowly when changing position (avoid sudden movements), prenatal yoga for flexibility
■ DIET & NUTRITION	Adequate hydration, calcium and magnesium for muscle and ligament health, anti-inflammatory foods (omega-3s, turmeric, ginger), maintain healthy pregnancy weight gain
■■ PRECAUTIONS	Distinguish from more serious conditions (appendicitis, ectopic concerns earlier in pregnancy, placental abruption, preterm labor); ligament pain is benign and brief; if pain is severe, prolonged, or accompanied by bleeding, fever, or contractions — seek medical evaluation
■ WHEN TO GO TO HOSPITAL	Pain lasting more than a few minutes, pain with fever, pain with vaginal bleeding, pain with contractions, severe pain that does not settle with rest and Paracetamol, pain on both sides simultaneously

■■ RECOVERY TIME	Episodic — occurs throughout 2nd and 3rd trimester; resolves after delivery; no lasting effect

Men's Health Conditions

9 Conditions Covered — 8 Guidance Fields Each

Benign Prostatic Hyperplasia (BPH)

Category: Men's Health

■ APPLICABLE TO	Men (over 50)
■ SYMPTOMS	Frequent nighttime urination (nocturia), weak/slow urine stream, difficulty starting urination, feeling of incomplete bladder emptying, urgency, dribbling, intermittent urine stream
■ MEDICATIONS	Alpha-blockers (Tamsulosin/Flomax, Alfuzosin) - relax muscle; 5-alpha reductase inhibitors (Finasteride, Dutasteride) - shrink prostate; Combination therapy; TURP (surgery) for severe cases
■ HOME REMEDIES	Avoid caffeine and alcohol (especially evenings), bladder training, double void technique, saw palmetto (mixed evidence), pumpkin seed oil, beta-sitosterol, regular exercise
■ DIET & NUTRITION	Reduce caffeine and alcohol (diuretics), limit evening fluids, saw palmetto foods, tomatoes (lycopene), green tea, avoid spicy foods
■■ PRECAUTIONS	Annual PSA testing and digital rectal exam after 50, avoid decongestants and antihistamines (worsen symptoms), stay active, maintain healthy weight, urological follow-up
■ WHEN TO GO TO HOSPITAL	Complete inability to urinate (acute urinary retention - emergency), blood in urine, UTI symptoms, kidney problems, severe symptoms
■■ RECOVERY TIME	Chronic condition; managed with medications or surgery

Erectile Dysfunction

Category: Men's Health

■ APPLICABLE TO	Men (any age, more common 40+)
■ SYMPTOMS	Inability to get or maintain erection sufficient for sexual intercourse, reduced sexual desire. May indicate underlying cardiovascular disease, diabetes, or hormonal issues
■ MEDICATIONS	PDE5 inhibitors: Sildenafil (Viagra), Tadalafil (Cialis - daily or as-needed), Vardenafil (Levitra); Address underlying causes (BP, diabetes, depression); Testosterone if deficient
■ HOME REMEDIES	Regular aerobic exercise, weight loss, stress management, quit smoking, limit alcohol, L-citrulline/L-arginine supplements (food: watermelon), dark chocolate, pelvic floor exercises

■ DIET & NUTRITION	Mediterranean diet (improves ED in studies), antioxidant-rich foods, omega-3 fatty acids, limit alcohol, limit saturated fat
■■ PRECAUTIONS	Treat underlying conditions (hypertension, diabetes, high cholesterol), discuss all medications with doctor (many cause ED), psychological counseling if psychogenic component
■ WHEN TO GO TO HOSPITAL	Rule out cardiovascular disease (ED often first sign), testosterone evaluation, sudden onset ED, ED in young men without obvious cause
■■ RECOVERY TIME	Highly variable depending on cause; lifestyle changes and medication effective in most cases

Prostate Cancer

Category: Men's Health

■ APPLICABLE TO	Men (risk increases after 50)
■ SYMPTOMS	Early: usually no symptoms (importance of screening). Advanced: difficulty urinating, weak urine stream, blood in urine or semen, painful ejaculation, erectile dysfunction, bone pain (metastatic)
■ MEDICATIONS	Active surveillance (low risk), Radical prostatectomy, Radiation therapy (external beam, brachytherapy), Hormone therapy (Leuprolide, Enzalutamide), Chemotherapy (Docetaxel), Immunotherapy
■ HOME REMEDIES	Prevention: tomatoes (lycopene), green tea, cruciferous vegetables, exercise; post-treatment: pelvic floor exercises (Kegel) for incontinence, erectile rehabilitation
■ DIET & NUTRITION	Tomatoes and lycopene-rich foods, cruciferous vegetables (broccoli, Brussels sprouts), soy foods, green tea, omega-3s, limit red meat and dairy
■■ PRECAUTIONS	PSA screening discussion with doctor at 50 (or 45 if African American or family history), know family history, maintain healthy weight, regular prostate exams
■ WHEN TO GO TO HOSPITAL	PSA elevation on screening, urinary symptoms, bone pain, blood in urine or semen, any suspected prostate issue
■■ RECOVERY TIME	Highly variable by stage and treatment; early-stage prostate cancer has excellent prognosis (99%+ 5-year survival for localized)

Testicular Disorders (Torsion, Cancer, Epididymitis)

Category: Men's Health

■ APPLICABLE TO	Males (all ages)
■ SYMPTOMS	Torsion: sudden severe one-sided scrotal pain, swelling, nausea - EMERGENCY. Epididymitis: gradual pain and swelling, fever, discharge. Testicular cancer: painless lump, heaviness

■ MEDICATIONS	Torsion: SURGICAL EMERGENCY within 6 hours. Epididymitis: Doxycycline/Azithromycin (STI-related) or Ofloxacin, Scrotal support, Ice. Testicular cancer: Orchiectomy, chemotherapy (Cisplatin-based), radiation
■ HOME REMEDIES	Epididymitis: scrotal support, ice pack, rest. Cancer: no home treatment - early detection and surgical management
■ DIET & NUTRITION	Balanced diet for recovery, selenium and zinc-rich foods for testicular health, antioxidant-rich foods
■■ PRECAUTIONS	Monthly testicular self-examination (age 15+), cryptorchidism (undescended testicles) increases cancer risk, any new lump needs immediate evaluation
■ WHEN TO GO TO HOSPITAL	TORSION - IMMEDIATELY (within 6 hours or testis lost). Any painless testicular lump. Epididymitis not improving. Sudden severe testicular pain
■■ RECOVERY TIME	Torsion: immediate surgery; Testicular cancer: excellent prognosis (95%+ cure rate) if caught early

Male Infertility

Category: Men's Health

■ APPLICABLE TO	Men of Reproductive Age
■ SYMPTOMS	Inability to conceive after 12 months of regular unprotected intercourse; reduced sexual desire; erectile dysfunction; pain or swelling in testicular area; gynecomastia (enlarged breast tissue); reduced facial or body hair; abnormal semen analysis: low count (oligospermia), poor motility (asthenospermia), abnormal morphology (teratospermia), no sperm (azoospermia)
■ MEDICATIONS	Hormonal: Clomiphene citrate, HCG, FSH injections (hypogonadism); treat varicocele surgically (varicocelectomy); antibiotics for infection; antioxidants: Vitamin C, E, CoQ10, Zinc, Selenium, L-Carnitine, Folic acid; ART: IUI, IVF, ICSI for severe cases
■ HOME REMEDIES	Quit smoking (damages sperm DNA), reduce alcohol, maintain healthy weight, reduce scrotal heat (avoid hot tubs, tight underwear, laptops on lap), manage stress, regular but not excessive exercise, adequate sleep, vitamin D supplementation
■ DIET & NUTRITION	Antioxidant-rich diet (berries, tomatoes, nuts, dark chocolate), zinc-rich foods (oysters, pumpkin seeds, beef), folate (leafy greens, legumes), omega-3 fatty acids, vitamin E (avocado, nuts, seeds), limit processed meats and trans fats, limit alcohol
■■ PRECAUTIONS	Semen analysis as first step if trying to conceive more than 12 months, both partners should be evaluated simultaneously, avoid anabolic steroids (cause azoospermia), avoid unnecessary hot environments for testicles, STI treatment and prevention, genetic counseling if azoospermia found
■ WHEN TO GO TO HOSPITAL	Semen analysis evaluation after 12 months trying (or 6 months if partner over 35), testicular pain or lump, history of cancer treatment, undescended testes history, hormonal evaluation

■■ RECOVERY TIME	Sperm production cycle is 72 days — lifestyle changes take 3 months to affect sperm quality; ART success varies by cause and age

Kidney Stones in Men

Category: Men's Health

■ APPLICABLE TO	Men (2-3 times more common than women)
■ SYMPTOMS	Sudden severe colicky pain starting in loin radiating to groin (classic loin-to-groin pattern); nausea and vomiting; blood in urine (hematuria — urine may appear pink, red, or brown); urinary urgency and frequency; painful urination; if infected: fever with rigors and chills
■ MEDICATIONS	Pain: NSAIDs (Ketorolac, Diclofenac) or Opioids for severe pain; Alpha-blocker (Tamsulosin) to help stone passage; Procedures: ESWL (shock wave lithotripsy), Ureteroscopy with laser, Percutaneous nephrolithotomy (PCNL) for large stones; Prevention medication depends on stone type
■ HOME REMEDIES	High fluid intake (2.5-3 liters daily — pale urine target), lemon juice and water (citrate prevents calcium stones), hot water bottle for pain relief, strain urine to collect stone for analysis; basil juice and apple cider vinegar (folk remedies, limited evidence)
■ DIET & NUTRITION	Target 2.5-3 liters fluid per day; reduce sodium (reduces calcium in urine); adequate calcium from food (NOT calcium supplements — these increase risk); limit oxalate foods for oxalate stones (spinach, nuts, chocolate, tea); limit animal protein; limit sugar and fructose
■■ PRECAUTIONS	Always increase fluid intake permanently after first stone (highest risk factor for recurrence), 24-hour urine collection for stone risk assessment, stone analysis when passed, follow stone-type specific prevention diet, maintain healthy weight
■ WHEN TO GO TO HOSPITAL	Fever with stone (infected hydronephrosis — EMERGENCY), uncontrolled severe pain, single kidney with obstruction, unable to keep fluids down, stone larger than 6mm (unlikely to pass spontaneously), no urine output
■■ RECOVERY TIME	Small stones (under 4mm): 1-2 weeks to pass; 4-6mm: 2-4 weeks; larger stones need intervention; 50% recurrence within 5 years without prevention

Male Pattern Baldness (Androgenetic Alopecia)

Category: Men's Health

■ APPLICABLE TO	Men (can start from late teens onward)
■ SYMPTOMS	Progressive thinning starting at temples and crown (Norwood pattern); receding hairline forming M-shape; gradually widening bald patch at crown; fine thin hair replacing coarser hair; Hamilton-Norwood scale grades I-VII for severity
■ MEDICATIONS	Finasteride (Propecia) — 1mg daily oral, DHT blocker, 80-90% effective at preventing further loss, requires lifelong use; Minoxidil (Rogaine) 2-5% topical solution or foam daily; combination therapy most effective; PRP (Platelet-Rich Plasma) injections; Hair transplant (FUE, FUT) for permanent results

■ HOME REMEDIES	Rosemary oil massaged into scalp (comparable to 2% Minoxidil in one small study), saw palmetto (mild DHT blocking), pumpkin seed oil, scalp massage (increases blood flow), onion juice (limited evidence), avoid tight hairstyles and harsh chemical treatments
■ DIET & NUTRITION	Protein-rich foods (hair is made of keratin protein), biotin (eggs, nuts, seeds), zinc (pumpkin seeds, oysters), iron (spinach, red meat), omega-3 fatty acids, vitamin D, silica-rich foods; avoid crash dieting and nutrient deficiencies
■■ PRECAUTIONS	Start treatment early for best results (cannot regrow hair already lost), finasteride may cause sexual side effects in small percentage (discuss with doctor), use sunscreen on exposed scalp, hair loss can be traumatic — psychological support if needed
■ WHEN TO GO TO HOSPITAL	Sudden rapid hair loss (alopecia areata?), hair loss with scalp inflammation, hair loss associated with weight loss/fatigue (thyroid, iron deficiency evaluation), patchy (not pattern) hair loss
■■ RECOVERY TIME	Finasteride and Minoxidil: results visible in 3-6 months; must continue indefinitely or hair loss resumes; hair transplant results in 6-12 months

Epididymo-orchitis

Category: Men's Health

■ APPLICABLE TO	Males (All Ages)
■ SYMPTOMS	Pain and swelling in one testicle (usually gradual onset distinguishing from torsion), redness and warmth of scrotum, fever, urethral discharge (if STI-related), pain with urination, heaviness in scrotum; in young sexually active men: usually chlamydia or gonorrhea; in older men: usually E. coli urinary pathogens
■ MEDICATIONS	Young sexually active men: Doxycycline + Ceftriaxone (covers chlamydia and gonorrhea — treat empirically); Older men: Ciprofloxacin or Co-amoxiclav (enteric organisms); Scrotal support; NSAIDs for pain; Complete full antibiotic course; treat sexual partners if STI-related
■ HOME REMEDIES	Scrotal elevation and support (supportive underwear or scrotal support), ice pack wrapped in cloth (15-20 minutes several times daily), rest, avoid heavy lifting and strenuous activity during treatment, warm sitz bath for comfort
■ DIET & NUTRITION	Anti-inflammatory foods, adequate hydration, avoid alcohol during antibiotic treatment, healthy diet to support immune function
■■ PRECAUTIONS	Complete full antibiotic course, test and treat sexual partners (if STI-related), use condoms to prevent STI recurrence, follow-up ultrasound to confirm resolution, check for testicular abscess if not improving
■ WHEN TO GO TO HOSPITAL	Sudden severe testicular pain (rule out torsion — EMERGENCY ultrasound needed), not improving after 3 days of antibiotics, abscess formation, fever not settling, testicular mass
■■ RECOVERY TIME	7-14 days with appropriate antibiotics; pain may persist 4-6 weeks; scrotal swelling 2-4 weeks; rare complication: infertility if bilateral or severe

Hypogonadism (Low Testosterone)

Category: Men's Health

■ APPLICABLE TO	Adult Men (especially over 40)
■ SYMPTOMS	Decreased libido (sex drive), erectile dysfunction, fatigue and lack of energy, decreased muscle mass and strength, increased body fat (especially around waist), mood changes (depression, irritability), reduced body and facial hair, hot flushes (male menopause symptoms), decreased bone density, impaired concentration, infertility
■ MEDICATIONS	Testosterone Replacement Therapy (TRT): Testosterone gel (Testogel, Androgel — daily application), Testosterone injections (Sustanon, Nebido — every 10-12 weeks), Testosterone pellets (implanted every 3-6 months); Clomiphene (off-label — stimulates own testosterone production, preserves fertility); HCG injections (preserves fertility while on TRT)
■ HOME REMEDIES	Strength and resistance training (raises testosterone naturally), adequate sleep (testosterone peaks during deep sleep), stress reduction (cortisol suppresses testosterone), reduce alcohol (lowers testosterone), maintain healthy weight (obesity lowers testosterone), zinc supplementation
■ DIET & NUTRITION	Zinc-rich foods (oysters, beef, pumpkin seeds), vitamin D (salmon, fortified foods, sunlight — essential for testosterone), omega-3 fatty acids, healthy fats (avocado, olive oil, nuts — testosterone is made from cholesterol), avoid soy excess, limit alcohol and processed foods
■■ PRECAUTIONS	PSA and prostate exam before starting TRT (testosterone can stimulate prostate cancer), hematocrit monitoring (TRT increases red blood cells — thrombosis risk), testicular atrophy with exogenous testosterone, fertility considerations (HCG or Clomiphene if wanting children), regular blood tests
■ WHEN TO GO TO HOSPITAL	For diagnosis (morning blood testosterone level + LH/FSH), to identify underlying cause (pituitary tumor — prolactinoma), TRT monitoring, PSA monitoring, if considering fertility treatment
■■ RECOVERY TIME	Symptomatic improvement with TRT in 3-6 months; bone density improvement over 1-2 years; lifelong therapy typically required

Women's Health Conditions

11 Conditions Covered — 8 Guidance Fields Each

Polycystic Ovary Syndrome (PCOS)

Category: Women's Health

■ APPLICABLE TO	Women of reproductive age
■ SYMPTOMS	Irregular or absent periods, excess androgens (hirsutism, acne, hair loss), polycystic ovaries on ultrasound, weight gain (especially abdominal), infertility, skin darkening (acanthosis nigricans), mood changes
■ MEDICATIONS	Hormonal contraceptives (regulate periods, reduce androgens), Metformin (insulin resistance), Spironolactone (hirsutism/acne), Clomiphene (fertility), Letrozole (fertility), Inositol supplements
■ HOME REMEDIES	Regular exercise (30 min/day - improves insulin sensitivity), stress management, adequate sleep, spearmint tea (anti-androgen), cinnamon (insulin sensitivity), inositol supplements
■ DIET & NUTRITION	Low glycemic index diet, high fiber, lean protein, anti-inflammatory foods, avoid processed/refined carbs, limit sugar, Mediterranean or DASH diet principles
■■ PRECAUTIONS	Annual metabolic screening (glucose, lipids), manage weight, address emotional health (depression and anxiety common), long-term risk monitoring (T2DM, heart disease, endometrial cancer)
■ WHEN TO GO TO HOSPITAL	Trying to conceive without success after 6-12 months, suspected PCOS for diagnosis, severe acne/hirsutism, metabolic complications, abnormal uterine bleeding
■■ RECOVERY TIME	Chronic condition requiring lifelong management; symptoms significantly improvable with lifestyle changes

Endometriosis

Category: Women's Health

■ APPLICABLE TO	Women of reproductive age
■ SYMPTOMS	Severe painful periods (dysmenorrhea), chronic pelvic pain, pain during/after intercourse, heavy menstrual bleeding, pain with bowel movements or urination during periods, infertility, fatigue
■ MEDICATIONS	NSAIDs (Ibuprofen) for pain, Hormonal contraceptives (suppress growth), Progestins, GnRH agonists (Leuprolide) for severe cases, Laparoscopic surgery for diagnosis and treatment
■ HOME REMEDIES	Heat therapy (heating pad on abdomen/back), omega-3 supplementation, anti-inflammatory diet, acupuncture, pelvic floor physical therapy, stress management

■ DIET & NUTRITION	Anti-inflammatory diet (omega-3, fruits, vegetables, whole grains), avoid red meat, trans fats, avoid soy (phytoestrogens), limit caffeine and alcohol
■■ PRECAUTIONS	Early diagnosis is key (average 7-10 year diagnostic delay), consider impact on fertility (consult reproductive endocrinologist), join support groups, regular gynecological follow-up
■ WHEN TO GO TO HOSPITAL	Severe pelvic pain, trying to conceive without success, suspected bowel/bladder involvement, symptoms not controlled with medications, fertility consultation
■■ RECOVERY TIME	Chronic condition; no cure but manageable; menopause typically improves symptoms

Breast Cancer

Category: Women's Health

■ APPLICABLE TO	Women (and rarely men)
■ SYMPTOMS	Painless breast lump, change in breast size/shape, dimpling of skin (peau d'orange), nipple discharge (bloody or clear), nipple inversion, redness/thickening, swollen lymph nodes in armpit
■ MEDICATIONS	Surgery (lumpectomy or mastectomy), Chemotherapy (various regimens), Radiation therapy, Hormone therapy (Tamoxifen for ER+, Aromatase inhibitors), Targeted therapy (Trastuzumab/Herceptin for HER2+), Immunotherapy
■ HOME REMEDIES	Prevention: maintain healthy weight, limit alcohol, breastfeed if possible, exercise regularly. Support during treatment: ginger for nausea, gentle exercise, meditation, support groups
■ DIET & NUTRITION	Anti-cancer diet: fruits, vegetables, whole grains, limit alcohol (increases risk), limit processed meats, maintain healthy weight (obesity increases risk), cruciferous vegetables
■■ PRECAUTIONS	Monthly breast self-exam (know your normal), annual mammogram from 40 (earlier if high risk), know family history, BRCA testing if indicated, limit hormone therapy
■ WHEN TO GO TO HOSPITAL	Any new breast lump, nipple changes, skin changes, unexplained breast pain, abnormal mammogram result, swollen lymph nodes
■■ RECOVERY TIME	Highly variable by stage; early-stage (Stage I-II): excellent prognosis 90%+ 5-year survival

Menopause

Category: Women's Health

■ APPLICABLE TO	Women (typically 45-55 years)
■ SYMPTOMS	Hot flashes, night sweats, vaginal dryness, irregular then absent periods, sleep disturbances, mood changes, weight gain, decreased libido, cognitive changes (brain fog), joint aches, hair thinning

■ MEDICATIONS	Hormone Replacement Therapy (estrogen ± progestogen) - most effective; Non-hormonal: SSRIs/SNRIs (hot flashes), Gabapentin, Clonidine; Vaginal estrogen (local, for dryness); Ospemifene
■ HOME REMEDIES	Black cohosh (hot flashes), phytoestrogen foods (soy, flaxseed, tofu), regular exercise, pelvic floor exercises, cooling techniques, stress management, acupuncture
■ DIET & NUTRITION	Calcium and vitamin D rich foods (bone health), phytoestrogen foods (soy, flaxseed), omega-3 fatty acids (mood and joints), avoid triggers (spicy food, alcohol, caffeine for hot flashes)
■■ PRECAUTIONS	Bone density scan (osteoporosis risk increases), cardiovascular risk management, hormone therapy risk-benefit discussion with gynecologist, regular breast exams and mammograms
■ WHEN TO GO TO HOSPITAL	Abnormal uterine bleeding in postmenopausal women (must rule out endometrial cancer), severe symptoms affecting quality of life, bone fracture, cardiovascular concerns
■■ RECOVERY TIME	Perimenopause 4-8 years; hot flashes typically subside 2-5 years after last period

Cervical Cancer

Category: Women's Health

■ APPLICABLE TO	Women
■ SYMPTOMS	Early: often no symptoms (importance of Pap smear screening). Advanced: abnormal vaginal bleeding (after intercourse, between periods, postmenopausal), unusual vaginal discharge, pelvic pain, pain during intercourse
■ MEDICATIONS	Early: LEEP, cone biopsy, surgery. Local/Regional: radical hysterectomy, radiation + Cisplatin chemotherapy. Advanced: Bevacizumab, Pembrolizumab (immunotherapy), palliative care
■ HOME REMEDIES	Prevention: HPV vaccination, safe sex, quit smoking, healthy immune system, regular Pap smears
■ DIET & NUTRITION	Folate-rich foods (prevent DNA damage), antioxidants (vitamins C and E), cruciferous vegetables, beta-carotene rich foods, avoid smoking
■■ PRECAUTIONS	HPV vaccine (Gardasil 9) for 9-45 years - most important prevention; Pap smear every 3 years from age 21; HPV co-test every 5 years from 30; safe sex practices
■ WHEN TO GO TO HOSPITAL	Any abnormal Pap smear result, postcoital bleeding, postmenopausal bleeding, abnormal vaginal discharge, pelvic pain
■■ RECOVERY TIME	Early-stage: 90%+ 5-year survival; prevention and early detection are key

Ovarian Cysts

Category: Women's Health

■ APPLICABLE TO	Women of Reproductive Age
■ SYMPTOMS	Most are asymptomatic (found incidentally on ultrasound); when symptomatic: dull or sharp pelvic pain on affected side, abdominal bloating or fullness, irregular periods, pain during intercourse, urinary frequency if large cyst presses on bladder; emergency signs of rupture/torsion: sudden severe pain, nausea, fever
■ MEDICATIONS	Most resolve spontaneously (functional cysts); Combined oral contraceptive pills (prevent new cyst formation); surgical management (laparoscopy) for large, persistent, or suspicious cysts; hormone management for endometriomas; cancer workup (CA-125, ultrasound) for complex cysts
■ HOME REMEDIES	Heat pad for pelvic pain, rest during painful periods, NSAIDs (Ibuprofen) for pain, gentle yoga and stretching, castor oil pack on lower abdomen (traditional remedy), avoid vigorous exercise when cyst is large (risk of torsion)
■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids, high fiber diet, vitamin D, avoid refined carbohydrates and sugars (particularly important if PCOS-related cysts), maintain healthy weight
■■ PRECAUTIONS	Follow-up ultrasound as recommended (to confirm resolution), complex or large cysts need gynecological evaluation, recognize emergency symptoms of ovarian torsion or rupture (sudden severe pain needs emergency care), report symptoms promptly
■ WHEN TO GO TO HOSPITAL	Sudden severe pelvic pain, fever with pelvic pain, nausea and vomiting with pain (ovarian torsion or rupture — EMERGENCY), rapidly growing cyst, suspicious features on ultrasound
■■ RECOVERY TIME	Functional cysts: resolve spontaneously in 2-3 menstrual cycles; surgical management: recovery 1-2 weeks laparoscopically

Fibrocystic Breast Disease

Category: Women's Health

■ APPLICABLE TO	Women (20-50 years, rarely postmenopausal)
■ SYMPTOMS	Breast lumpiness and tenderness (especially before menstrual period), multiple movable lumps that change with menstrual cycle, breast pain (mastalgia), breast swelling, clear or slightly cloudy nipple discharge; symptoms worsen before period and improve after
■ MEDICATIONS	NSAIDs or Paracetamol for pain; Vitamin E 400-800 IU daily (reduces mastalgia in some women); Evening primrose oil (GLA supplements); Oral contraceptives (regulate hormones, may reduce symptoms); rarely Danazol or Bromocriptine for severe mastalgia; well-fitted supportive bra
■ HOME REMEDIES	Well-fitting supportive bra worn day and night during symptomatic periods; warm compress for pain; evening primrose oil; Vitamin E; caffeine reduction (coffee, tea, chocolate may worsen cyclical breast pain in some women); regular aerobic exercise

■ DIET & NUTRITION	Reduce caffeine (coffee, tea, cola, chocolate — evidence mixed but many women report benefit), low-fat diet, increase omega-3 fatty acids, adequate iodine (seafood, iodized salt), avoid salt premenstrually (reduces fluid retention), vitamin E rich foods
■■ PRECAUTIONS	Monthly breast self-examination (know your normal — fibrocystic changes are benign but new distinct lump needs evaluation); mammogram as age-appropriate; distinguish from malignancy (fibrocystic lumps move freely, change with cycle; cancer lumps tend to be hard, fixed, non-tender)
■ WHEN TO GO TO HOSPITAL	New dominant lump that does not change with menstrual cycle, bloody nipple discharge, skin changes over breast, lump not resolving after 2 menstrual cycles, new lump in postmenopausal woman
■■ RECOVERY TIME	Chronic benign condition; symptoms typically improve at menopause; no treatment needed unless symptomatic

Vulvodynia / Chronic Pelvic Pain

Category: Women's Health

■ APPLICABLE TO	Women (Any Age)
■ SYMPTOMS	Chronic vulvar pain (burning, stinging, rawness, soreness, aching) lasting more than 3 months without identifiable cause; pain may be localized (vestibulodynia — pain at vaginal opening) or generalized; pain triggered by touch, sex, tampon insertion, sitting; no visible abnormality on examination
■ MEDICATIONS	Topical lidocaine gel for pain relief, Tricyclic antidepressants (Amitriptyline low dose — first-line for neuropathic pain), Gabapentin or Pregabalin, topical estrogen (if postmenopausal atrophy contributes), Botox injections for pelvic floor hypertonicity; pelvic floor physiotherapy (cornerstone)
■ HOME REMEDIES	Cold gel packs on vulva for pain relief, cotton underwear (no synthetic fabrics), unscented soaps and avoid all vulvar irritants (scented products, bubble baths, harsh detergents), cool water rinse instead of wiping, sitz baths, Vaseline as barrier protection, sexual lubricants (silicone-based)
■ DIET & NUTRITION	Low oxalate diet (may reduce vulvar burning in oxalate-sensitive women — avoid spinach, nuts, chocolate, tea), reduce caffeine and alcohol, anti-inflammatory diet, calcium citrate supplements (binds oxalates)
■■ PRECAUTIONS	Avoid all vulvar irritants and triggers, pelvic floor physiotherapy critical, psychological support for chronic pain, sexual counseling for couple, treat associated anxiety/depression, trial of different treatments (often requires combination), patient support groups
■ WHEN TO GO TO HOSPITAL	Proper diagnosis to exclude treatable causes (infections, skin conditions, cancer), multidisciplinary pain management clinic, pelvic floor physiotherapy referral, sexual medicine specialist
■■ RECOVERY TIME	Chronic condition; multidisciplinary treatment improves symptoms in 60-80% over 1-2 years; complete resolution possible for some

Premenstrual Syndrome (PMS) and PMDD

Category: Women's Health

■ APPLICABLE TO	Women of Reproductive Age
■ SYMPTOMS	PMS (up to 7 days before period): bloating, breast tenderness, mood swings, fatigue, food cravings, acne, headache, sleep changes; PMDD (severe PMS): severe depression, anxiety, irritability, anger, hopelessness, tension, feeling out of control — symptoms significantly impair daily function; symptoms resolve within days of period starting
■ MEDICATIONS	PMS: calcium supplementation (1200mg/day), Vitamin B6, Evening primrose oil, NSAIDS for pain; PMDD: SSRIs (Fluoxetine/Sarafem, Sertraline, Citalopram) taken continuously or luteal phase only (day 14-28) — very effective for PMDD; Drospirenone-containing OCP; GnRH agonists (severe refractory — chemical menopause)
■ HOME REMEDIES	Regular aerobic exercise (significantly reduces PMS and PMDD symptoms), adequate sleep (consistent schedule), stress management, reduce caffeine and alcohol in luteal phase, mindfulness and CBT, heat pad for cramps, magnesium glycinate supplement
■ DIET & NUTRITION	Complex carbohydrates throughout luteal phase (reduces mood symptoms — tryptophan/serotonin connection), calcium-rich foods (dairy, fortified foods, sardines), magnesium (nuts, seeds, leafy greens), vitamin B6 (poultry, fish, bananas), reduce salt (less bloating), reduce caffeine and alcohol
■■ PRECAUTIONS	Track symptoms across 2 menstrual cycles to confirm diagnosis (symptom diary), differentiate PMDD from underlying depression or anxiety (symptoms must be cyclical and resolve after period), CBT is effective for psychological symptoms, regular exercise is evidence-based treatment
■ WHEN TO GO TO HOSPITAL	PMDD causing severe functional impairment, suicidal thoughts premenstrually, symptoms not following menstrual cycle (may be other condition), SSRI initiation and monitoring, hormonal management consideration
■■ RECOVERY TIME	PMS/PMDD managed monthly throughout reproductive years; SSRIs effective within 1-2 months; both conditions resolve at menopause

Vulvovaginal Candidiasis (Vaginal Thrush)

Category: Women's Health

■ APPLICABLE TO	Women (3 in 4 women will have at least one episode)
■ SYMPTOMS	Intense vulval itching (most prominent symptom), thick white cottage-cheese-like vaginal discharge (odourless), vulval redness, swelling and soreness, burning sensation especially with urination or intercourse, painful intercourse, white curdy patches on vaginal walls; triggered by antibiotics, diabetes, pregnancy, immunosuppression, tight synthetic clothing

■ MEDICATIONS	Topical antifungals: Clotrimazole vaginal pessary (500mg single dose or 100mg for 6 nights), Miconazole cream; Oral antifungal: Fluconazole 150mg single dose (not in pregnancy); Recurrent thrush (4+ per year): Fluconazole 150mg weekly for 6 months (suppressive therapy); address predisposing factors
■ HOME REMEDIES	Plain unsweetened yogurt applied topically (contains Lactobacillus, restores normal flora), coconut oil for external soothing (antifungal properties), cotton underwear, avoid tight clothing, avoid scented soaps and douching, cool water rinse, boric acid vaginal suppositories (recurrent thrush, not in pregnancy)
■ DIET & NUTRITION	Probiotics (Lactobacillus acidophilus — yogurt, kefir, supplements), reduce sugar intake (yeast thrives on glucose), avoid refined carbohydrates, garlic (natural antifungal), avoid alcohol during treatment, vitamin C for immune support
■■ PRECAUTIONS	Avoid douching (disrupts vaginal flora), wear cotton underwear, avoid scented products in vaginal area, complete antifungal course, treat predisposing conditions (diabetes, antibiotic use), confirm diagnosis before treating recurrent episodes (rule out BV and STIs which present similarly)
■ WHEN TO GO TO HOSPITAL	Recurrent thrush (4+ per year), first-time thrush in pregnancy, symptoms not responding to OTC treatment after 7 days, atypical symptoms, rule out STIs, diabetes screening (recurrent thrush can indicate undiagnosed diabetes)
■■ RECOVERY TIME	Acute episode: 1-7 days with antifungal treatment; recurrent thrush: 6-month suppressive therapy with 70-90% success

Dysmenorrhea (Painful Periods)

Category: Women's Health

■ APPLICABLE TO	Women (most common gynecological condition globally)
■ SYMPTOMS	Cramping pain in lower abdomen just before and during first days of menstrual period; may radiate to lower back and thighs; nausea and vomiting; headache; diarrhea; lightheadedness; fainting in severe cases; Primary dysmenorrhea (no underlying cause — common in teens and young women); Secondary dysmenorrhea (underlying cause: endometriosis, fibroids, adenomyosis)
■ MEDICATIONS	NSAIDs (most effective — start day before or at onset of period): Ibuprofen 400-600mg every 6-8 hours, Naproxen, Mefenamic acid; Combined oral contraceptive pill (reduces prostaglandins); Hormonal IUS (Mirena — significantly reduces period pain); Treat underlying cause in secondary dysmenorrhea
■ HOME REMEDIES	Heat therapy (hot water bottle or heating pad on lower abdomen — as effective as ibuprofen in studies), TENS (transcutaneous electrical nerve stimulation) device, massage of lower abdomen with clary sage or lavender oil, yoga (child's pose, supine twist), ginger tea, omega-3 fatty acids supplementation
■ DIET & NUTRITION	Omega-3 fatty acids (fish, walnuts, flaxseed — reduce prostaglandin production), anti-inflammatory diet, magnesium-rich foods (reduces cramps), vitamin D and calcium, ginger and turmeric (natural anti-inflammatory), reduce caffeine and alcohol (worsen cramps), reduce salty foods (reduce bloating)

■■ PRECAUTIONS	Start NSAIDs prophylactically (day before period if predictable) for maximum effect, regular aerobic exercise reduces dysmenorrhea severity, investigate if pain is worsening over time or not responding to NSAIDs (secondary causes — endometriosis may be present), record pain severity and days
■ WHEN TO GO TO HOSPITAL	Pain not controlled with NSAIDs and oral contraceptives, pain getting worse over years (secondary dysmenorrhea — possible endometriosis), pain outside menstrual period, deep pain with intercourse, pelvic examination and ultrasound needed
■■ RECOVERY TIME	Primary dysmenorrhea: improves with age and after pregnancy; managed each cycle with NSAIDs; Secondary: depends on treating underlying cause

Mental Health Disorders

12 Conditions Covered — 8 Guidance Fields Each

Depression (Major Depressive Disorder)

Category: Mental Health

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Persistent sad/empty/hopeless mood (2+ weeks), loss of interest in activities, changes in appetite/weight, sleep disturbances, fatigue, feelings of worthlessness/guilt, difficulty concentrating, thoughts of death or suicide
■ MEDICATIONS	SSRIs (Sertraline, Fluoxetine, Escitalopram - first line), SNRIs (Venlafaxine, Duloxetine), TCAs (Amitriptyline), MAOIs, Bupropion, Mirtazapine, Ketamine/Esketamine (treatment-resistant), Psychotherapy (CBT)
■ HOME REMEDIES	Regular aerobic exercise (as effective as medication for mild-moderate depression), sunlight exposure, social connection, omega-3 supplementation, St. John's Wort (mild depression, consult doctor), mindfulness meditation
■ DIET & NUTRITION	Mediterranean diet, omega-3 rich foods, avoid excessive alcohol, B-vitamin rich foods (especially B12 and folate), magnesium-rich foods, gut-healthy foods (probiotics)
■■ PRECAUTIONS	Never stop antidepressants abruptly (taper with doctor guidance), therapy combined with medication is most effective, monitor for suicidal ideation especially when starting/changing medication, tell doctor about all supplements
■ WHEN TO GO TO HOSPITAL	Suicidal thoughts or plans, inability to care for oneself, psychotic symptoms, severe self-neglect, not responding to outpatient treatment
■■ RECOVERY TIME	Mild-moderate: 6-12 weeks for treatment response; treatment continued 6-12 months to prevent relapse

Anxiety Disorder (Generalized)

Category: Mental Health

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Excessive uncontrollable worry about everyday matters, restlessness, fatigue, difficulty concentrating, irritability, muscle tension, sleep problems, physical symptoms (racing heart, sweating, GI issues)
■ MEDICATIONS	SSRIs (first-line for chronic anxiety: Escitalopram, Sertraline), SNRIs (Venlafaxine), Buspirone (non-addictive), Benzodiazepines (short-term only: Alprazolam, Clonazepam), Beta-blockers (situational), Hydroxyzine
■ HOME REMEDIES	Deep breathing exercises (4-7-8 technique), progressive muscle relaxation, mindfulness meditation, regular exercise, yoga, limit caffeine and alcohol, cold water splash (activates dive reflex)

■ DIET & NUTRITION	Limit caffeine (major anxiety trigger), avoid alcohol (rebound anxiety), omega-3 fatty acids, magnesium-rich foods, chamomile tea, avoid processed foods
■■ PRECAUTIONS	Avoid benzodiazepine dependence, therapy (CBT is most effective), identify triggers, establish consistent sleep routine, limit news and social media, build support network
■ WHEN TO GO TO HOSPITAL	Panic attacks that are frequent/disabling, anxiety with suicidal thoughts, not functioning in daily life, co-occurring substance abuse, new onset in elderly (rule out medical causes)
■■ RECOVERY TIME	Highly variable; CBT with medication: 12-20 weeks for significant improvement; anxiety management is ongoing

Panic Disorder

Category: Mental Health

■ APPLICABLE TO	Adults, Teens
■ SYMPTOMS	Recurrent unexpected panic attacks: sudden intense fear, racing heart, sweating, trembling, shortness of breath, chest pain, nausea, dizziness, feeling of unreality, fear of dying or going crazy; persistent worry about future attacks
■ MEDICATIONS	SSRIs/SNRIs (first-line preventive), Benzodiazepines (acute attack only, short-term), Cognitive Behavioral Therapy (CBT - most effective), Exposure therapy
■ HOME REMEDIES	Diaphragmatic breathing during attack (4 counts in, hold 4, out 8), grounding techniques (5-4-3-2-1 senses), cold water on face, regular exercise, yoga, progressive muscle relaxation
■ DIET & NUTRITION	Eliminate caffeine (major trigger), avoid alcohol, blood sugar maintenance (hypoglycemia can trigger attacks), magnesium-rich foods
■■ PRECAUTIONS	CBT is most effective treatment, avoid avoidance behaviors (maintains disorder), track attack patterns, consider when attacks began, rule out cardiac/thyroid causes
■ WHEN TO GO TO HOSPITAL	First panic attack (to rule out cardiac/medical causes), attacks more than once a week, significant functional impairment, agoraphobia developing
■■ RECOVERY TIME	With CBT: significant improvement in 12-20 sessions

Insomnia

Category: Mental Health / Sleep

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Difficulty falling asleep (taking more than 30 minutes), difficulty staying asleep, early morning awakening, non-restorative sleep, daytime fatigue/sleepiness, irritability, difficulty concentrating, minimum 3 nights/week for 3+ months

■ MEDICATIONS	CBT-I (cognitive behavioral therapy for insomnia - GOLD STANDARD), Melatonin (sleep onset), Zolpidem (Ambien - short-term), Eszopiclone, Ramelteon, Doxepin (low dose), Suvorexant (Belsomra); address underlying causes
■ HOME REMEDIES	Strict sleep-wake schedule (even weekends), dark/cool/quiet room, no screens 1 hour before bed, chamomile tea, lavender aromatherapy, warm bath before bed, progressive muscle relaxation, white noise
■ DIET & NUTRITION	Avoid caffeine after noon, limit alcohol (disrupts sleep architecture), avoid large meals near bedtime, tart cherry juice (melatonin), kiwi, warm milk
■■ PRECAUTIONS	Sleep hygiene is foundational treatment, avoid naps during day, get out of bed if not asleep in 20 minutes, reserve bed for sleep and sex only, address anxiety and depression
■ WHEN TO GO TO HOSPITAL	Persistent insomnia affecting daily functioning, suspected sleep apnea, RLS, shift work disorder, mood disorder assessment needed
■■ RECOVERY TIME	CBT-I: improvement in 6-8 sessions; lifestyle changes: weeks to months

Bipolar Disorder

Category: Mental Health

■ APPLICABLE TO	Adults, Teens
■ SYMPTOMS	Cycles of mania (elevated mood, decreased need for sleep, racing thoughts, impulsivity, grandiosity, risky behaviors) and depression (low mood, hopelessness, fatigue); severe mood episodes can be psychotic
■ MEDICATIONS	Mood stabilizers: Lithium (gold standard), Valproate, Lamotrigine; Antipsychotics (Quetiapine, Olanzapine, Aripiprazole); Never antidepressants alone (can trigger mania); Psychotherapy adjunct
■ HOME REMEDIES	Strict sleep regulation (irregular sleep triggers episodes), daily routine, stress management, omega-3 supplementation (adjunct), avoid alcohol and drugs (major triggers)
■ DIET & NUTRITION	Regular meal timing, omega-3 rich foods, limit caffeine and alcohol, avoid substances
■■ PRECAUTIONS	Never stop mood stabilizers abruptly, lifestyle monitoring for prodromal signs, involve family/support person, psychiatric advance directive, driving safety during episodes
■ WHEN TO GO TO HOSPITAL	Manic episode (person often lacks insight, may need involuntary treatment), psychotic symptoms, suicidal ideation, not sleeping for days, dangerous impulsive behaviors
■■ RECOVERY TIME	Lifelong condition; mood stabilizers reduce frequency and severity of episodes; 70% achieve significant improvement with proper treatment

OCD (Obsessive-Compulsive Disorder)

Category: Mental Health

■ APPLICABLE TO	Adults and Children (often starts in childhood/early adulthood)
■ SYMPTOMS	Obsessions (intrusive unwanted thoughts, images, or urges — contamination, harm, symmetry, religious/sexual themes) causing significant anxiety; Compulsions (repetitive behaviors to reduce anxiety — handwashing, checking, counting, arranging) that are time-consuming and interfere with daily life; person recognizes obsessions as irrational but cannot control them
■ MEDICATIONS	SSRIs at high doses (first-line): Fluvoxamine (Luvox), Fluoxetine, Sertraline, Paroxetine, Escitalopram; Clomipramine (TCA — effective but more side effects); Augmentation: Antipsychotics (Risperidone, Aripiprazole) for partial responders; ERP therapy (gold standard behavioral treatment)
■ HOME REMEDIES	ERP (Exposure and Response Prevention) exercises with therapist guidance, mindfulness (not avoidance), delay compulsions progressively, NAC (N-Acetylcysteine) supplement (limited evidence), exercise, stress management
■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids, inositol (may have mild benefit), reduce caffeine (worsens anxiety), adequate sleep, avoid alcohol
■■ PRECAUTIONS	ERP (Exposure Response Prevention) therapy is gold-standard treatment — more effective than medication alone; do not accommodate compulsions (increases OCD); family psychoeducation; high SSRIs doses needed (higher than depression); treatment takes 12-20 weeks to work; do not stop abruptly
■ WHEN TO GO TO HOSPITAL	OCD causing inability to function (eating, leaving house, work), concurrent suicidal ideation, severe depression alongside OCD, not responding to first-line treatment (consider intensive OCD programs or TMS)
■■ RECOVERY TIME	OCD is chronic but very treatable; ERP + SSRI: 60-70% achieve significant response in 12-20 weeks; relapse prevention important

PTSD (Post-Traumatic Stress Disorder)

Category: Mental Health

■ APPLICABLE TO	Adults and Children (after trauma exposure)
■ SYMPTOMS	Re-experiencing trauma (flashbacks, nightmares, intrusive memories), avoidance of trauma reminders, hyperarousal (startle response, sleep problems, irritability, hypervigilance), negative thoughts and mood changes, emotional numbing; symptoms present more than 1 month after traumatic event
■ MEDICATIONS	SSRIs (Sertraline, Paroxetine — FDA-approved for PTSD), SNRIs (Venlafaxine); Prazosin (for PTSD nightmares); Trauma-focused therapies: EMDR (Eye Movement Desensitization and Reprocessing), Prolonged Exposure (PE), Cognitive Processing Therapy (CPT); Avoid benzodiazepines (worsen long-term PTSD)
■ HOME REMEDIES	Safe calming environments, grounding techniques (5-4-3-2-1 senses), diaphragmatic breathing, regular aerobic exercise (reduces hyperarousal), yoga and mindfulness, journaling, trusted social support, nature exposure

■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids (brain healing), avoid alcohol (major PTSD trigger and worsens outcome), adequate sleep, reduce caffeine (worsens hyperarousal), magnesium-rich foods (calming)
■■ PRECAUTIONS	Avoid alcohol and drugs (worsen PTSD and cause dependence), build safe support network, do not isolate, trauma-informed care approach, treatment of co-occurring depression and anxiety, workplace accommodations if needed
■ WHEN TO GO TO HOSPITAL	Suicidal ideation, self-harm, substance use disorder concurrent, complete inability to function, dissociative episodes, PTSD in first responders or combat veterans (specialized programs available)
■■ RECOVERY TIME	With EMDR or Prolonged Exposure: significant improvement in 8-15 sessions; PTSD can be fully resolved with proper treatment in many individuals

Eating Disorders (Anorexia and Bulimia)

Category: Mental Health

■ APPLICABLE TO	Adolescents and Young Adults (predominantly women)
■ SYMPTOMS	Anorexia Nervosa: extreme food restriction, intense fear of weight gain, severely underweight (BMI below 17.5), distorted body image, absence of menstruation, lanugo hair, low blood pressure, fatigue, cold intolerance. Bulimia Nervosa: recurrent binge eating followed by compensatory behaviors (purging, laxatives, excessive exercise), dental erosion, swollen cheeks (parotid gland enlargement), normal or near-normal weight
■ MEDICATIONS	Anorexia: no specific medication (focus on weight restoration and therapy); antidepressants after weight restoration; Bulimia: SSRIs (Fluoxetine 60mg — FDA-approved), CBT (most effective); Binge Eating Disorder: Lisdexamfetamine (Vyvanse — FDA-approved), SSRIs; medical management of complications (electrolytes, cardiac monitoring)
■ HOME REMEDIES	Support (NOT food monitoring which worsens control issues), mealtime companionship, challenging negative body talk, non-food rewards, body-neutral activities, yoga and gentle movement, support groups (NEDA); NOTE: professional treatment is essential — eating disorders have the highest mortality of any mental illness
■ DIET & NUTRITION	Structured meal plan developed with dietitian specializing in eating disorders; regular eating pattern to normalize hunger cues; no 'good or bad' foods approach (reduce rigid food rules); nutritional rehabilitation; re-feeding protocol for anorexia (medical supervision required)
■■ PRECAUTIONS	Never comment on appearance or food intake (triggers), involve whole family in treatment (FBT — Family-Based Treatment for adolescents), cardiac monitoring for anorexia (arrhythmias from electrolyte imbalance — major cause of death), dental care for bulimia, monitor bone density
■ WHEN TO GO TO HOSPITAL	BMI below 15, severe electrolyte imbalances (cardiac arrhythmia risk), fainting, muscle weakness (proximal myopathy), refusing all treatment, suicidal ideation — may require inpatient medical stabilization then residential treatment

■■ RECOVERY TIME	Anorexia: average 5-6 years to recovery; 50% fully recover; Bulimia: better prognosis — 70% recover; both conditions highly treatable with specialized care

Substance Use Disorder (Alcohol Addiction)

Category: Mental Health

■ APPLICABLE TO	Adults
■ SYMPTOMS	Inability to control alcohol intake, strong craving for alcohol, tolerance (needing more for same effect), withdrawal symptoms (tremor, sweating, anxiety, seizures when stopping), continuing to drink despite negative consequences, giving up other activities for alcohol, secretive drinking, relationship and work problems
■ MEDICATIONS	Acute withdrawal (hospital): Benzodiazepines (Diazepam — CIWA protocol), Thiamine (vitamin B1 prevention of Wernicke's); Maintenance sobriety: Naltrexone (reduces craving), Acamprosate (reduces withdrawal symptoms), Disulfiram (Antabuse — aversion therapy), Gabapentin; liver disease treatment if present
■ HOME REMEDIES	Alcoholics Anonymous (AA) — 12-step program (strong evidence); motivational interviewing; identify and address triggers; healthy activities replacement; strong social support network; mindfulness; yoga; medical detox MUST precede home-based recovery (withdrawal can be fatal)
■ DIET & NUTRITION	Thiamine (B1) and B-vitamin supplementation (severe deficiency in alcoholics), high protein diet for liver recovery, antioxidant-rich foods, hydration, balanced nutrition, folate-rich foods, zinc, magnesium; NO ALCOHOL
■■ PRECAUTIONS	NEVER stop alcohol abruptly if heavily dependent — supervised medical detox required (seizures can be fatal); carry naltrexone prescription; remove alcohol from home; attend support groups; treat co-occurring mental health conditions; regular liver function tests
■ WHEN TO GO TO HOSPITAL	Alcohol withdrawal seizures, Delirium Tremens (DTs — fever, confusion, hallucinations — LIFE THREATENING), Wernicke's encephalopathy (confusion, ataxia, eye problems), liver failure, suicidal ideation
■■ RECOVERY TIME	Recovery is lifelong; most people relapse 1-3 times before sustained sobriety; relapse is part of recovery, not failure; quality of life significantly improves with sobriety

Schizophrenia

Category: Mental Health

■ APPLICABLE TO	Adults (typically onset late teens to early 30s)
■ SYMPTOMS	Positive symptoms: hallucinations (hearing voices most common — auditory), delusions (fixed false beliefs — persecution, grandiosity), disorganized thinking/speech; Negative symptoms: flat affect, alogia (reduced speech), avolition (lack of motivation), anhedonia, social withdrawal; Cognitive: impaired memory and executive function; onset typically gradual

■ MEDICATIONS	Antipsychotics (essential — lifelong in most): Atypical (second-generation — preferred): Risperidone, Olanzapine, Quetiapine, Aripiprazole, Clozapine (treatment-resistant schizophrenia — gold standard but requires blood monitoring); Typical (first-generation): Haloperidol, Chlorpromazine; Long-acting injectable (LAI) formulations improve adherence
■ HOME REMEDIES	Structured daily routine, family psychoeducation (family therapy improves outcomes), social skills training, supported employment, peer support, reduce cannabis use (major trigger and worsener), omega-3 supplementation (adjunct in early psychosis), reduce stress
■ DIET & NUTRITION	Regular balanced meals (antipsychotics cause weight gain — healthy diet critical), omega-3 fatty acids, Mediterranean diet, reduce sugar and ultra-processed foods, monitor for metabolic syndrome (common with antipsychotics), no cannabis or alcohol
■■ PRECAUTIONS	Never stop antipsychotics without psychiatrist guidance (relapse within 6 months in 80%), community mental health team involvement, crisis plan, carer support and education, smoking cessation (very high rates in schizophrenia — increases antipsychotic metabolism), regular metabolic monitoring (weight, glucose, lipids)
■ WHEN TO GO TO HOSPITAL	First psychotic episode (needs assessment and safe initiation of treatment), acute relapse (agitation, threatening behavior, self-neglect), suicidal ideation (significantly elevated suicide risk in schizophrenia), medication non-adherence leading to relapse
■■ RECOVERY TIME	Chronic condition requiring lifelong management; 25% achieve good outcomes; 50% have partial recovery with ongoing symptoms; early treatment intervention improves long-term prognosis significantly

Dementia (Vascular Dementia)

Category: Mental Health / Neurological

■ APPLICABLE TO	Elderly (usually 65+)
■ SYMPTOMS	Stepwise cognitive decline (each stroke causes a step-down in function — distinguishes from Alzheimer's gradual decline), memory loss (working memory early), slowed thinking, difficulty with concentration and planning, mood changes (depression, emotional lability), physical symptoms (weakness, balance problems, urinary incontinence), focal neurological signs from strokes
■ MEDICATIONS	No specific approved treatment for vascular dementia itself; secondary prevention (preventing further strokes is key): Antiplatelet therapy (Aspirin, Clopidogrel), strict hypertension control (ACEi, ARBs, CCBs), statin therapy, anticoagulation if AFib; Donepezil/Memantine (often used off-label, modest benefit); treat depression, sleep disturbance, behavioral symptoms
■ HOME REMEDIES	Mental stimulation and cognitive rehabilitation, physical exercise (improves cerebral blood flow), social engagement, music therapy, reminiscence therapy, structured routine, safe home environment, support for carers

■ DIET & NUTRITION	MIND diet (Mediterranean-DASH hybrid — associated with reduced vascular dementia risk), strict sodium restriction for blood pressure control, Mediterranean diet, no alcohol (major stroke risk factor), adequate hydration, maintain healthy weight
■■ PRECAUTIONS	Aggressive cardiovascular risk factor management (hypertension control is most important preventive measure), treat AFib (major stroke cause), quit smoking, control diabetes, anticoagulation for AFib, regular neurology follow-up, driving assessment
■ WHEN TO GO TO HOSPITAL	Acute stroke (FAST — EMERGENCY), sudden stepwise worsening, behavioral emergencies, caregiver burnout, safety concerns at home, advanced care planning
■■ RECOVERY TIME	Progressive condition; vascular dementia progresses in steps rather than gradually; prevention of further strokes most important intervention to slow progression

Chronic Fatigue Syndrome (ME/CFS)

Category: Mental Health / Systemic

■ APPLICABLE TO	Adults (predominantly women, often after infection)
■ SYMPTOMS	Profound debilitating fatigue not improved by rest; post-exertional malaise (PEM — any physical or cognitive exertion worsens symptoms for days to weeks — hallmark symptom); cognitive impairment (brain fog — memory, concentration, word-finding); unrefreshing sleep; orthostatic intolerance (worsening symptoms standing up); pain (muscle and joint); headaches; often triggered by viral infection
■ MEDICATIONS	No curative treatment; symptom management: Low-dose Amitriptyline (sleep and pain), Low-dose Naltrexone (emerging evidence), Mestinon (pyridostigmine — for orthostatic intolerance), Fludrocortisone (POTS), Abilify (Aripiprazole — small studies show benefit); CBT and graded exercise therapy are CONTROVERSIAL and may worsen PEM (pacing is preferred)
■ HOME REMEDIES	Pacing (energy envelope management — most important: never push beyond energy limits), heart rate monitoring during activity (stay below anaerobic threshold), sleep hygiene, stress reduction, cool environment, compression garments for orthostatic symptoms, rest in horizontal position
■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids, magnesium glycinate, CoQ10 (mitochondrial support), vitamin D, small frequent meals (large meals worsen fatigue), adequate hydration with added electrolytes (especially sodium for POTS), avoid caffeine and alcohol
■■ PRECAUTIONS	AVOID PUSH-CRASH CYCLE (overexerting on good days leads to severe relapse lasting weeks); strict pacing is the most evidence-based management; do not push through fatigue; keep activity diary; avoid all unnecessary stressors; occupational support for work adjustments

■ WHEN TO GO TO HOSPITAL	Initial diagnosis to exclude other conditions (thyroid, anemia, sleep apnea, malignancy, depression), severe relapse requiring acute management, significant weight loss, suicidal ideation from impact on quality of life
■■ RECOVERY TIME	Highly variable; 25-50% improve over years; some remain severely disabled; no predictable recovery; early pacing gives better long-term prognosis

Neurological Conditions

9 Conditions Covered — 8 Guidance Fields Each

Epilepsy

Category: Neurological

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Recurrent unprovoked seizures; types: tonic-clonic (grand mal - stiffening and jerking), absence (brief staring episodes), focal (affecting one side), myoclonic jerks; post-ictal confusion after
■ MEDICATIONS	Levetiracetam (Keppra), Valproate, Carbamazepine, Lamotrigine, Phenytoin, Topiramate; choice based on seizure type; Emergency: Diazepam (Valium), Lorazepam IV for status epilepticus
■ HOME REMEDIES	Ketogenic diet (high fat, low carb - especially effective in children), avoid seizure triggers (flashing lights, sleep deprivation, alcohol, fever, stress), regular sleep
■ DIET & NUTRITION	Ketogenic diet (4:1 fat:protein+carb ratio) under medical supervision for drug-resistant epilepsy; regular meals (hypoglycemia can lower seizure threshold); avoid alcohol
■■ PRECAUTIONS	Never drive until seizure-free for required period (varies by region), shower not bath (drowning risk), helmet if frequent falls, wear medical alert ID, seizure action plan, sleep adequately
■ WHEN TO GO TO HOSPITAL	Seizure lasting more than 5 minutes (STATUS EPILEPTICUS - EMERGENCY), first seizure, seizure in water, injury during seizure, series of seizures without recovery, pregnancy
■■ RECOVERY TIME	Chronic condition; 70% achieve good seizure control with medication; seizure-free 2+ years may allow medication tapering

Alzheimer's Disease / Dementia

Category: Neurological

■ APPLICABLE TO	Elderly (usually 65+)
■ SYMPTOMS	Progressive memory loss (especially recent events), confusion about time/place, language problems, difficulty with familiar tasks, personality/behavioral changes, poor judgment, getting lost, withdrawal from social activities
■ MEDICATIONS	Cholinesterase inhibitors (Donepezil, Rivastigmine, Galantamine) for mild-moderate; Memantine for moderate-severe; Lecanemab (Leqembi) new disease-modifying therapy; Medications for behavioral symptoms
■ HOME REMEDIES	Mental stimulation (puzzles, reading), physical exercise, social engagement, music therapy, reminiscence therapy, regular routine and structure, simplify environment

■ DIET & NUTRITION	MIND diet (Mediterranean-DASH hybrid): leafy greens, nuts, berries, beans, whole grains, fish, poultry, olive oil, wine (limited); shown to reduce Alzheimer's risk
■■ PRECAUTIONS	Safety modifications at home (locks, stove guards, remove driving privileges), caregiver support and respite, advance care planning, legal and financial planning while capacity present, wandering prevention
■ WHEN TO GO TO HOSPITAL	New confusion or rapid cognitive decline, behavioral emergencies, caregiver burnout, falls, aspiration pneumonia, evaluation of new symptoms, medication adjustment
■■ RECOVERY TIME	Progressive incurable condition; slowed with current medications; care transitions from home to memory care facilities as disease progresses

Parkinson's Disease

Category: Neurological

■ APPLICABLE TO	Adults (usually 60+)
■ SYMPTOMS	Tremor at rest (pill-rolling), muscle rigidity, bradykinesia (slowness of movement), postural instability, small shuffling steps, reduced facial expression (mask face), soft speech, micrographia
■ MEDICATIONS	Levodopa/Carbidopa (Sinemet - gold standard), Dopamine agonists (Pramipexole, Ropinirole), MAO-B inhibitors (Selegiline, Rasagiline), COMT inhibitors, Anticholinergics; Deep Brain Stimulation (DBS) for advanced
■ HOME REMEDIES	Regular exercise (boxing, dance, Tai Chi proven beneficial), speech therapy, physical therapy, occupational therapy, music therapy, social engagement
■ DIET & NUTRITION	High fiber diet (constipation prevention), adequate protein but space from levodopa dosing (protein reduces absorption), Mediterranean diet, adequate hydration
■■ PRECAUTIONS	Fall prevention critical (remove rugs, grab bars, good footwear), medication timing is critical (take levodopa consistently), regular neurology follow-ups, address non-motor symptoms (depression, sleep, constipation)
■ WHEN TO GO TO HOSPITAL	Sudden worsening of symptoms (medication adjustment), severe falls, aspiration pneumonia, medication-induced complications, psychosis, surgical consultation for DBS
■■ RECOVERY TIME	Progressive chronic condition; symptoms managed for years-decades; progression varies widely

Multiple Sclerosis (MS)

Category: Neurological

■ APPLICABLE TO	Adults (usually 20-40 years, women more than men)

■ SYMPTOMS	Episodes of neurological dysfunction (relapses): vision loss (optic neuritis — painful vision loss in one eye), weakness or numbness in limbs, difficulty walking, balance problems, bladder dysfunction, fatigue (most common symptom — often disabling), cognitive changes, Lhermitte's sign (electric shock down spine on neck flexion), Uhthoff's (symptoms worsen in heat)
■ MEDICATIONS	Disease-modifying therapies (DMTs): Interferon beta (Avonex, Rebif), Glatiramer acetate (Copaxone), Natalizumab (Tysabri), Ocrelizumab (Ocrevus — high efficacy), Siponimod, Cladribine; Acute relapse: High-dose IV Methylprednisolone 1g/day for 3-5 days; Symptom management: Amantadine (fatigue), Baclofen (spasticity), Dalfampridine (walking)
■ HOME REMEDIES	Regular aerobic exercise (proven to improve MS outcomes — yoga, swimming, cycling), adequate sleep, heat avoidance strategies (cooling vests, cool showers), stress management (stress may trigger relapses), vitamin D supplementation (important in MS)
■ DIET & NUTRITION	Vitamin D (strong evidence for benefit in MS — supplement 2000-4000 IU/day, sun exposure), Mediterranean diet, omega-3 fatty acids, Swank diet (low saturated fat), ensure adequate B12, avoid smoking (accelerates disability progression)
■■ PRECAUTIONS	Start DMT early (prevents disability accumulation), regular MRI monitoring, annual flu vaccine (avoid live vaccines on some DMTs), fall prevention (walking aids, physical therapy), bladder management program, regular ophthalmology review, sun safety (Vitamin D-sun balance)
■ WHEN TO GO TO HOSPITAL	New neurological symptoms (possible relapse — IV steroids within 48-72 hours), severe relapse causing disability, suspected PML (rare brain infection from Natalizumab), yearly neurological review and MRI
■■ RECOVERY TIME	Relapsing-remitting MS: partial to full recovery from relapses over weeks; DMTs reduce relapse rate by 30-70%; disability accumulates over decades

Migraine with Aura

Category: Neurological

■ APPLICABLE TO	Adults (Women more than Men)
■ SYMPTOMS	Aura (20-30 minutes before headache): visual disturbances (zigzag lines, flashing lights, blind spot), sensory changes (numbness, tingling spreading up arm to face), speech difficulty; followed by: severe unilateral throbbing headache lasting 4-72 hours, nausea and vomiting, extreme sensitivity to light and sound, worsened by movement
■ MEDICATIONS	Acute: Triptans (Sumatriptan, Rizatriptan, Zolmitriptan — most effective), NSAIDs (Naproxen, Ibuprofen), Ergotamines, Anti-nausea (Metoclopramide, Prochlorperazine); Preventive: Propranolol, Topiramate, Amitriptyline, Venlafaxine, Candesartan, CGRP monoclonal antibodies (Erenumab/Aimovig — new)

■ HOME REMEDIES	Dark quiet cool room, cold pack on forehead/neck, caffeine (small amount can help if used sparingly), ginger tea for nausea, peppermint oil on temples, magnesium glycinate supplement (400mg daily — preventive), riboflavin (Vitamin B2 400mg daily — preventive), CoQ10
■ DIET & NUTRITION	Avoid dietary triggers: aged cheese, red wine, beer, chocolate, MSG, processed meats (nitrates), artificial sweeteners, skipped meals, dehydration; maintain hydration; regular meal times; moderate caffeine; magnesium-rich foods (leafy greens, nuts, seeds)
■■ PRECAUTIONS	Triptan use limited to 10 days per month (more causes medication overuse headache — MOH); migraine diary to identify triggers; note that combined oral contraceptive pill increases stroke risk in women with migraine with aura; adequate sleep; stress management
■ WHEN TO GO TO HOSPITAL	Worst headache of life (thunderclap headache — rule out subarachnoid hemorrhage), headache with fever and stiff neck, new neurological deficit that does not fully resolve, headache after head injury, first migraine after 50, status migrainosus (migraine lasting more than 72 hours)
■■ RECOVERY TIME	Individual attack: 4-72 hours; with early Triptan use: 1-2 hours; CGRP antibodies reduce monthly migraine days by 50% in 70% of patients

Peripheral Neuropathy

Category: Neurological

■ APPLICABLE TO	Adults (common in Diabetics)
■ SYMPTOMS	Numbness and tingling starting in feet and hands (glove-and-stocking distribution), burning or sharp stabbing pain in feet (worse at night), extreme sensitivity to touch (allodynia), weakness in hands and feet, loss of balance and coordination, loss of reflexes; autonomic: dizziness on standing, constipation, bladder dysfunction
■ MEDICATIONS	Treat underlying cause (diabetes control, B12 replacement, thyroid treatment, alcohol cessation); Neuropathic pain: Pregabalin (Lyrica), Gabapentin, Duloxetine (Cymbalta — approved for diabetic neuropathy), Amitriptyline low-dose, Topical capsaicin, Lidocaine patches, Tramadol (moderate-severe); Vitamin B1, B6, B12 supplementation
■ HOME REMEDIES	Foot care (inspect daily for injury — sensation reduced), warm (not hot) foot soaks, massage, transcutaneous electrical nerve stimulation (TENS), alpha-lipoic acid supplement (antioxidant, reduces diabetic neuropathy symptoms), magnesium, B-complex vitamins
■ DIET & NUTRITION	Strict blood sugar control (if diabetic — most important), B12-rich foods (meat, dairy, eggs) or B12 supplements (especially if vegan or on Metformin), folate, thiamine (B1) rich foods, omega-3 fatty acids, avoid alcohol strictly
■■ PRECAUTIONS	Daily foot inspection (mirror for soles), well-fitting footwear (no tight shoes), test bath water temperature before entering (burn risk with reduced sensation), fall prevention measures (handrails, good lighting), regular podiatry if diabetic

■ WHEN TO GO TO HOSPITAL	Rapidly progressive weakness (Guillain-Barré syndrome?), breathing difficulty with weakness, severe uncontrolled pain, foot ulcer in diabetic, loss of bladder or bowel control
■■ RECOVERY TIME	Depends entirely on cause; diabetic neuropathy: stabilizes but rarely fully reverses; B12 deficiency: often reverses with treatment; Guillain-Barré: weeks to months recovery

Bell's Palsy

Category: Neurological

■ APPLICABLE TO	Adults (All Ages)
■ SYMPTOMS	Sudden onset (hours to days) weakness or complete paralysis of one side of face; inability to close eye on affected side; drooping corner of mouth; loss of forehead wrinkles; difficulty smiling, chewing, or speaking; taste changes (front two-thirds of tongue); ear pain; hypersensitivity to sound; excessive tearing or dry eye on affected side
■ MEDICATIONS	Prednisolone oral (high dose: 50-60mg/day for 10 days — most effective if started within 72 hours of onset); Acyclovir or Valacyclovir added to steroids (if started early — benefit is debated); Eye care: lubricating eye drops every 2-4 hours and eye ointment at night, tape eye closed at night (corneal protection critical); Physiotherapy for facial exercises
■ HOME REMEDIES	Eye protection (most critical — tape eyelid shut during sleep, sunglasses outdoors), gentle facial massages, warm compress on affected side, facial exercises in front of mirror (helped by physiotherapist guidance), protect eye from wind and dust, artificial tears every 1-2 hours while awake
■ DIET & NUTRITION	Soft foods initially if difficulty chewing (mashed potato, yogurt, soup), chew on unaffected side, use straw if needed for drinking, adequate nutrition for nerve recovery, B12 and B-complex vitamins
■■ PRECAUTIONS	Eye protection is paramount (corneal abrasion and ulceration can occur from inability to blink — can cause permanent vision loss), start Prednisolone within 72 hours for best results, ophthalmology review if eye problems develop, physiotherapy for facial exercises
■ WHEN TO GO TO HOSPITAL	Immediately when facial palsy develops (start steroids within 72 hours — time-sensitive), if eye becomes red or painful (corneal complications), if no improvement after 3-4 weeks, to exclude other causes of facial nerve palsy (brain tumor, Lyme disease, Ramsay Hunt syndrome)
■■ RECOVERY TIME	70% recover fully within 6 months; 85% with treatment; 15% have residual weakness; corneal complications preventable with proper eye care

Carpal Tunnel Syndrome

Category: Neurological / Musculoskeletal

■ APPLICABLE TO	Adults (Women 3x more than Men, Pregnant Women, Diabetics)

■ SYMPTOMS	Tingling, numbness, and burning in thumb, index, middle, and half of ring finger (median nerve distribution); weakness and dropping things; symptoms worse at night (waking from sleep to shake hands); wrist and hand pain; atrophy of thenar muscle (base of thumb) in advanced cases; positive Phalen's test (wrists flexed — symptoms reproduced in 60 seconds) and Tinel's sign (tapping over wrist — tingling)
■ MEDICATIONS	Wrist splint in neutral position (especially at night — first-line for mild-moderate); Corticosteroid injection (short-term relief, can be repeated); NSAIDs (limited benefit); Surgical: Carpal Tunnel Release (open or endoscopic — gold standard for moderate-severe, highly effective long-term); Vitamin B6 (pyridoxine) — limited evidence but safe
■ HOME REMEDIES	Wrist splint in neutral position (available OTC — wear at night and when symptomatic), avoid repetitive wrist flexion/extension, ice pack for pain and swelling, gentle wrist stretches and nerve gliding exercises, take breaks from keyboard work (wrist exercises every 30 minutes), elevated position for swollen wrists
■ DIET & NUTRITION	Anti-inflammatory diet, vitamin B6 (poultry, fish, banana, chickpeas), omega-3 fatty acids (fish, walnuts), reduce salt if swelling prominent (pregnancy-related), maintain healthy weight, vitamin D
■■ PRECAUTIONS	Wrist-friendly ergonomic setup at work (neutral wrist position when typing), ergonomic keyboard and mouse, wrist rest supports, regular breaks, avoid tight grip, treat underlying conditions (hypothyroidism, diabetes, pregnancy-related swelling often causes transient CTS)
■ WHEN TO GO TO HOSPITAL	Weakness developing (picking up objects), constant numbness (not just at night), thenar atrophy visible (severe — surgery needed urgently), failure of conservative treatment after 3 months, nerve conduction study for diagnosis and severity
■■ RECOVERY TIME	Wrist splint: improvement in weeks to months; Steroid injection: 3-6 months relief; Surgical release: symptom relief in 90% within weeks; full nerve recovery: up to 1 year

Trigeminal Neuralgia

Category: Neurological

■ APPLICABLE TO	Adults (usually over 50, Women more than Men)
■ SYMPTOMS	Sudden brief (seconds to 2 minutes) severe electric-shock-like or stabbing facial pain following the trigeminal nerve distribution (usually one cheek, jaw, teeth, gums, or lips); triggered by touch (eating, speaking, brushing teeth, cold air, washing face); pain-free intervals between episodes; patient may avoid eating/speaking/touching face; one of the most severe pains known
■ MEDICATIONS	Carbamazepine (Tegretol — first-line, 70% effective); Oxcarbazepine; Lamotrigine; Baclofen; Gabapentin; Surgical options for refractory cases: Microvascular Decompression (MVD — most durable), Percutaneous procedures (balloon compression, glycerol rhizotomy), Gamma Knife radiosurgery

■ HOME REMEDIES	Identify and avoid triggers (eating on unaffected side, use room-temperature foods, avoid facial air exposure, electric shaver instead of razor, gentle face washing), vitamin B12 supplementation, stress management, warm compress over trigeminal area between attacks
■ DIET & NUTRITION	Soft foods when having frequent attacks (mashed, liquid foods to minimize chewing pain), foods at room temperature (avoid very cold or very hot triggers), adequate nutrition despite eating difficulties, vitamin B12 rich foods
■■ PRECAUTIONS	Carbamazepine requires blood count monitoring (rare aplastic anemia), sodium monitoring, drug interactions; dental procedure avoidance unless absolutely necessary (may trigger attacks); MRI to exclude secondary causes (MS, tumor compressing nerve), ensure adequate pain management (severe psychological impact of the pain)
■ WHEN TO GO TO HOSPITAL	For diagnosis (to rule out secondary causes), medication initiation and monitoring, surgical consultation for refractory cases, for support if pain is causing depression or inability to eat
■■ RECOVERY TIME	Chronic condition with spontaneous remissions possible; medications control pain in 75% initially, but efficacy often decreases with time; surgical procedures highly effective (90%+ pain relief with MVD)

Skin Conditions

10 Conditions Covered — 8 Guidance Fields Each

Eczema (Atopic Dermatitis)

Category: Skin

■ APPLICABLE TO	All Ages (most common in children)
■ SYMPTOMS	Dry itchy inflamed skin, rash on face/scalp/neck/arms/behind knees, patches of rough leathery skin, oozing/crusting when scratched, darkening/thickening with chronic scratching, worsens with triggers
■ MEDICATIONS	Topical corticosteroids (Hydrocortisone, Triamcinolone) for flares, Tacrolimus/Pimecrolimus (steroid-sparing), Dupilumab (Dupixent - severe), Antihistamines for itch relief, Moisturizers (cornerstone of therapy)
■ HOME REMEDIES	Oatmeal baths (colloidal oatmeal), coconut oil moisturizing, sunflower seed oil, wet wrap therapy, avoid triggers (wool, harsh soaps, certain foods), identify and eliminate triggers
■ DIET & NUTRITION	Identify food triggers (dairy, eggs, soy, wheat, nuts common triggers), elimination diet under guidance, omega-3 fatty acids, probiotics, avoid inflammatory foods
■■ PRECAUTIONS	Moisturize at least twice daily (within 3 minutes of bathing), use fragrance-free products, lukewarm short baths, wear cotton clothing, identify and avoid triggers, don't scratch
■ WHEN TO GO TO HOSPITAL	Signs of skin infection (red, warm, oozing, fever), widespread severe flare, affecting eyes/eyelids, not responding to topical treatments, significant quality of life impact
■■ RECOVERY TIME	Chronic relapsing condition; 50% of childhood eczema improves significantly by teen years

Psoriasis

Category: Skin

■ APPLICABLE TO	Adults (teens and children less common)
■ SYMPTOMS	Raised red patches covered with silvery-white scales (plaques), dry cracked skin that may bleed, itching/burning/soreness, thickened/pitted/ridged nails, swollen/stiff joints (psoriatic arthritis)
■ MEDICATIONS	Topical: Corticosteroids, Vitamin D analogues (Calcipotriol), Retinoids, Coal tar; Phototherapy (UVB); Systemic: Methotrexate, Cyclosporine, Acitretin; Biologics (Adalimumab, Secukinumab, Ustekinumab)
■ HOME REMEDIES	Dead Sea salt baths, aloe vera gel (topical), fish oil supplements, turmeric (curcumin), stress management (major trigger), appropriate sun exposure, moisturizing

■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids, avoid alcohol (major trigger), avoid smoking, maintain healthy weight, limit refined carbs and red meat
■■ PRECAUTIONS	Identify and avoid triggers (stress, infections, certain medications, injury, alcohol), regular dermatology follow-up, screen for psoriatic arthritis and metabolic syndrome, mental health support
■ WHEN TO GO TO HOSPITAL	Erythrodermic psoriasis (entire body covered - medical emergency), pustular psoriasis, severe joint involvement, infection signs, new widespread outbreak
■■ RECOVERY TIME	Chronic relapsing remitting condition; biologics can achieve clear skin in 70-80% of patients

Acne Vulgaris

Category: Skin

■ APPLICABLE TO	Teens, Young Adults (all ages possible)
■ SYMPTOMS	Blackheads (open comedones), whiteheads (closed comedones), papules (small red bumps), pustules (pus-filled), nodules (large painful lumps under skin), cysts, commonly on face/back/chest
■ MEDICATIONS	Topical: Benzoyl peroxide, Salicylic acid, Retinoids (Tretinoin, Adapalene), Clindamycin; Oral: Antibiotics (Doxycycline, Minocycline), Oral contraceptives (women), Isotretinoin (Accutane) for severe nodular acne
■ HOME REMEDIES	Tea tree oil (diluted), aloe vera gel, witch hazel, green tea extract, niacinamide, zinc supplements, ice on cysts, avoid picking and popping
■ DIET & NUTRITION	Low glycemic index diet (reduces acne), avoid dairy (especially skim milk in some people), reduce sugar, omega-3 fatty acids, zinc-rich foods, avoid whey protein supplements
■■ PRECAUTIONS	Wash face twice daily with gentle cleanser, non-comedogenic moisturizer and sunscreen, don't pick/pop (scarring), sun protection especially on Tretinoin, regular dermatology follow-up for moderate-severe
■ WHEN TO GO TO HOSPITAL	Severe nodular/cystic acne (scarring risk), isotretinoin consideration, suspected hormonal cause in women (PCOS), acne not responding to OTC treatments after 8 weeks
■■ RECOVERY TIME	Mild: 8-12 weeks with treatment; moderate-severe may take 3-6 months; isotretinoin: 4-5 month course, often curative

Ringworm / Fungal Skin Infections (Tinea)

Category: Skin

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Ringworm (body): circular scaly ring-shaped red rash, itchy. Athlete's foot (feet): scaling, itching, burning between toes. Jock itch (groin): red itchy rash. Nail fungus: thickened, yellow, brittle nails

■ MEDICATIONS	Topical antifungals: Clotrimazole, Miconazole, Terbinafine cream (2 weeks); Oral antifungals (nail/severe): Terbinafine, Itraconazole, Fluconazole; Selenium sulfide shampoo (tinea versicolor)
■ HOME REMEDIES	Coconut oil (antifungal properties), tea tree oil (diluted), garlic paste, apple cider vinegar, keep affected area clean and dry, change socks daily
■ DIET & NUTRITION	Reduce sugar (fungus thrives on sugar), probiotics (Lactobacillus), avoid processed foods, antifungal foods (garlic, coconut oil, oregano)
■■ PRECAUTIONS	Keep skin clean and dry, dry between toes after bathing, wear breathable footwear, don't share personal items, treat pets (ringworm is zoonotic), wash clothes and bedding frequently
■ WHEN TO GO TO HOSPITAL	Fungal infection on scalp/face/nails (needs prescription oral medication), widespread infection, not responding to OTC treatment after 2 weeks, immunocompromised patient
■■ RECOVERY TIME	Skin: 2-4 weeks with topical treatment; Nail: 3-6 months oral treatment

Vitiligo

Category: Skin

■ APPLICABLE TO	All Ages
■ SYMPTOMS	White depigmented patches on skin (milky white), particularly noticeable on darker skin; often appears on sun-exposed areas (face, lips, hands), armpits, groin, around body openings; patches may spread or remain stable; premature whitening of hair (poliosis); psychological impact often significant
■ MEDICATIONS	Topical corticosteroids (first-line for limited disease), Topical calcineurin inhibitors (Tacrolimus, Pimecrolimus — face and folds), NB-UVB phototherapy (best evidence for repigmentation), PUVA therapy, Ruxolitinib cream (Opzelura — new FDA-approved JAK inhibitor cream), surgical options (grafting) for stable vitiligo
■ HOME REMEDIES	Sun protection (depigmented skin burns easily — SPF 50+ sunscreen), high quality self-tanner for camouflage, Ginkgo biloba (some evidence for halting progression), Polypodium leucotomos extract, khellin (from Ammi visnaga plant — caution), cosmetic camouflage makeup
■ DIET & NUTRITION	Anti-inflammatory diet, antioxidant-rich foods (vitamin C, E, beta-carotene), vitamin B12 and folate (deficiency associated with vitiligo), zinc and copper (tyrosinase cofactors needed for melanin production), avoid excess vitamin C (may inhibit melanin synthesis)
■■ PRECAUTIONS	Year-round SPF 50+ sunscreen on depigmented patches (no natural sun protection in white skin), regular eye examinations (eye melanocytes may be affected), thyroid function testing (associated autoimmune condition), psychological support important
■ WHEN TO GO TO HOSPITAL	Rapidly spreading vitiligo, associated thyroid symptoms, psychological distress, to access phototherapy, considering surgical repigmentation

■■ RECOVERY TIME	Variable; stable disease may persist indefinitely; repigmentation possible with treatment (best results on face); NB-UVB phototherapy: 3-6 months for visible response; requires long-term maintenance

Urticaria (Hives)

Category: Skin

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Sudden raised red itchy wheals on skin (can appear anywhere); individual lesions last less than 24 hours but new ones may keep appearing; may be accompanied by angioedema (deeper swelling — lips, eyelids, throat); triggers include foods, medications, infections, exercise, cold, stress; chronic urticaria lasts more than 6 weeks
■ MEDICATIONS	Non-sedating antihistamines (first-line): Cetirizine, Loratadine, Fexofenadine — regular daily dosing (not just when needed for chronic); Sedating: Chlorphenamine for nighttime itch; Omalizumab (Xolair) for chronic refractory urticaria — biologic; Corticosteroids (short course for severe acute); Epinephrine (if anaphylaxis)
■ HOME REMEDIES	Cool bath or cool damp compress (antihistamine-like effect), aloe vera gel, calamine lotion, avoid known triggers, loose cool cotton clothing, keep room cool (heat worsens itch), oatmeal bath
■ DIET & NUTRITION	Identify and eliminate food triggers (common: shellfish, nuts, eggs, dairy, wheat, soy, additives), low-histamine diet for chronic urticaria (avoid aged cheese, wine, processed meat, fermented foods), keep a food and symptom diary
■■ PRECAUTIONS	Carry epinephrine auto-injector (EpiPen) if previous anaphylaxis, medical alert bracelet, identify triggers (allergy testing), avoid NSAIDs (worsen urticaria in 30% of cases), inform doctors of history before procedures
■ WHEN TO GO TO HOSPITAL	Throat swelling or breathing difficulty (anaphylaxis — EMERGENCY — use EpiPen and call 108), facial/tongue swelling, urticaria not responding to antihistamines after 24 hours, severe widespread urticaria with systemic symptoms
■■ RECOVERY TIME	Acute urticaria: resolves in days to weeks; Chronic urticaria: 50% resolve within 1 year; may recur; Omalizumab works within 1-3 months

Seborrheic Dermatitis (Dandruff)

Category: Skin

■ APPLICABLE TO	All Ages (Infants as Cradle Cap, Adults Common)
■ SYMPTOMS	Flaky white or yellowish oily scales on scalp (dandruff), face (eyebrows, nose folds, beard), ears, chest, and back; redness and itching; greasy appearance of skin; in infants: thick crusty yellow-brown scales on scalp (cradle cap — not itchy)

■ MEDICATIONS	Antifungal shampoos: Ketoconazole 2% (Nizoral), Selenium sulfide, Zinc pyrithione, Ciclopirox; leave-in 5-10 minutes before rinsing; Topical corticosteroids (hydrocortisone or betamethasone) for inflamed skin; Topical calcineurin inhibitors for face; Infant cradle cap: baby oil then gentle combing
■ HOME REMEDIES	Tea tree oil shampoo (diluted), coconut oil scalp massage (leave overnight, comb flakes, then shampoo), apple cider vinegar rinse (diluted 1:1 with water), aloe vera gel on scalp, regular shampooing, avoid styling products on scalp
■ DIET & NUTRITION	Anti-inflammatory diet, reduce sugar (feeds Malassezia yeast), omega-3 fatty acids, zinc-rich foods, probiotics (gut-skin connection), biotin, vitamin B complex, reduce alcohol (worsens seborrheic dermatitis)
■■ PRECAUTIONS	Chronic relapsing condition — maintenance treatment required; use medicated shampoo 1-2 times per week for maintenance; avoid harsh hair products; manage stress (worsens flares); sun exposure may improve or worsen (individual variation); HIV and Parkinson's disease are associated with severe seborrheic dermatitis
■ WHEN TO GO TO HOSPITAL	Severe skin involvement beyond scalp, not responding to OTC antifungal shampoos after 4 weeks, secondary bacterial skin infection, widespread red inflamed skin, in immunocompromised patients
■■ RECOVERY TIME	Chronic relapsing condition; with antifungal treatment: improvement in 4-6 weeks; maintenance treatment required indefinitely to prevent recurrence

Shingles (Herpes Zoster)

Category: Skin / Infectious

■ APPLICABLE TO	Adults (especially over 50, Immunocompromised)
■ SYMPTOMS	Pain, burning, or tingling in a band-like pattern on one side of body (prodrome — 1-5 days before rash); then characteristic unilateral dermatomal rash: red spots progressing to fluid-filled blisters then crusting; most common on chest/abdomen (thoracic dermatome) or face; severe pain; post-herpetic neuralgia (persistent pain after rash heals) in 10-30% especially elderly
■ MEDICATIONS	Antivirals (start within 72 hours of rash onset — reduce duration and post-herpetic neuralgia): Valacyclovir (Valtrex) 1g 3x daily for 7 days, Famciclovir, Acyclovir; Pain management: Paracetamol, NSAIDs, Opioids (severe), Amitriptyline/Gabapentin (nerve pain); Shingles vaccine (Shingrix — 2 doses, 97% effective prevention, recommended 50+)
■ HOME REMEDIES	Cool wet compress on rash (reduces heat and pain), calamine lotion, loose lightweight clothing (nothing touching rash), oatmeal bath, keep rash clean and dry to prevent secondary infection, avoid scratching, keep covered around immunocompromised people and pregnant women (who haven't had chickenpox)
■ DIET & NUTRITION	Lysine-rich foods (dairy, fish, poultry, legumes — inhibits viral replication), Vitamin C (immune support), zinc, B-complex vitamins, avoid arginine-rich foods during outbreak (chocolate, nuts, gelatin — arginine promotes herpes virus replication), stay well hydrated

■■ PRECAUTIONS	Shingrix vaccine (most important — get vaccinated), keep rash covered, avoid contact with pregnant women (especially first trimester), immunocompromised, and people who never had chickenpox until all blisters crust over; start antivirals as early as possible (within 72 hours); ophthalmology if rash on forehead (zoster ophthalmicus — can cause blindness)
■ WHEN TO GO TO HOSPITAL	Rash on forehead or near eye (Herpes Zoster Ophthalmicus — same-day ophthalmology), immunocompromised patient, severe or widespread rash, pain not controlled, Ramsay Hunt syndrome (ear pain, facial palsy, rash in ear canal — urgent ENT and antivirals)
■■ RECOVERY TIME	Rash: 2-4 weeks; Pain: most resolves in 1-3 months; Post-herpetic neuralgia: 10-30% have pain for months to years; prevented by Shingrix vaccine

Rosacea

Category: Skin

■ APPLICABLE TO	Adults (30-60 years, Fair-skinned Women)
■ SYMPTOMS	Chronic redness of central face (nose, cheeks, forehead, chin); flushing easily; visible blood vessels (telangiectasia); acne-like bumps and pus-filled spots (papulopustular rosacea); burning and stinging sensation; sensitive skin; Rhinophyma (bulbous enlarged red nose — more in men); eye involvement in 50% (redness, grittiness — ocular rosacea)
■ MEDICATIONS	Topical: Metronidazole gel/cream (Rozex), Azelaic acid 15% (Finacea), Ivermectin cream (Soolantra — papulopustular), Brimonidine gel (redness), Oxymetazoline (redness); Oral: Doxycycline 40mg (Oracea — sub-antimicrobial dose, papulopustular), Isotretinoin (rhinophyma); Laser for telangiectasia and rhinophyma
■ HOME REMEDIES	Identify and avoid triggers (sun, heat, spicy food, alcohol, exercise, stress, hot drinks, certain skincare products), sun protection SPF 30+ daily, cool water compress during flush, green-tinted primer to neutralize redness, gentle fragrance-free skincare, avoid harsh exfoliants
■ DIET & NUTRITION	Identify personal triggers (common: alcohol especially red wine, spicy foods, hot drinks, chocolate, cinnamaldehyde-containing foods), anti-inflammatory diet, omega-3 fatty acids, avoid foods that cause flushing, keep food diary to identify personal triggers
■■ PRECAUTIONS	Daily broad-spectrum SPF 30-50 sunscreen (sun is the most common trigger), avoid skincare products with alcohol, fragrance, menthol, witch hazel; use gentle pH-balanced cleansers; treat ocular rosacea (eyelid hygiene, tetracyclines if needed); manage stress; record triggers in diary
■ WHEN TO GO TO HOSPITAL	Ocular rosacea affecting vision, rhinophyma requiring laser or surgical treatment, severe papulopustular rosacea unresponsive to topicals, differentiate from lupus (butterfly rash), initial diagnosis and treatment planning
■■ RECOVERY TIME	Chronic condition with flares and remissions; no cure but excellent control with trigger avoidance and topical treatment; complete clearance in some patients; laser treatment for vascular changes is semi-permanent

Alopecia Areata

Category: Skin / Autoimmune

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Sudden patchy hair loss (round or oval smooth bald patches anywhere on scalp), exclamation mark hairs at patch edges (pathognomonic), pitting or ridging of nails (in 20%), scalp usually non-inflamed; Alopecia Totalis (entire scalp); Alopecia Universalis (entire body hair loss); can regrow spontaneously but recurrence common; often triggered by stress
■ MEDICATIONS	Intralesional corticosteroid injections (most effective for patchy — repeated every 4-6 weeks); Topical corticosteroids (moderate potency for scalp); Minoxidil 5% topical (stimulates regrowth); JAK inhibitors (Baricitinib, Ruxolitinib — FDA-approved for severe alopecia areata, excellent results); Topical immunotherapy (DPCP — specialist centers); PUVA phototherapy
■ HOME REMEDIES	Scalp massage with rosemary oil (some evidence for hair growth), onion juice topically (small studies show benefit), aloe vera gel on scalp, adequate sleep, stress management techniques (major trigger), avoid hair trauma (tight hairstyles, heat styling, chemical treatments), protect scalp from sun
■ DIET & NUTRITION	Zinc (pumpkin seeds, beef, oysters), iron, biotin, vitamin D, selenium, omega-3 fatty acids (anti-inflammatory), folate, protein (hair is protein), avoid crash diets and nutritional deficiencies
■■ PRECAUTIONS	Screen for associated autoimmune conditions (thyroid disease most common — thyroid function tests), support for significant psychological impact, wigs and scalp coverings if desired, sun protection for exposed scalp, JAK inhibitors require blood monitoring
■ WHEN TO GO TO HOSPITAL	Extensive alopecia (Alopecia Totalis/Universalis), for JAK inhibitor treatment (prescription required), thyroid and other autoimmune screening, psychological support assessment, dermatology specialist for treatment planning
■■ RECOVERY TIME	Patchy alopecia areata: 50% regrow spontaneously within 1 year; 85% eventually regrow; 30% develop Totalis/Universalis; JAK inhibitors: 30-40% achieve complete scalp coverage in 6-12 months

Bone & Joint Diseases

10 Conditions Covered — 8 Guidance Fields Each

Arthritis (Osteoarthritis)

Category: Musculoskeletal

■ APPLICABLE TO	Adults (especially 45+)
■ SYMPTOMS	Joint pain (worsens with activity, improves with rest), stiffness (especially morning stiffness under 30 min), decreased range of motion, joint swelling, bone spurs, crepitus (cracking), most common in knees, hips, hands, spine
■ MEDICATIONS	Acetaminophen (first-line), NSAIDs (Ibuprofen, Naproxen, Celecoxib), Topical NSAIDs/Capsaicin, Corticosteroid injections (short-term relief), Hyaluronic acid injections, Joint replacement (advanced)
■ HOME REMEDIES	Warm/cold therapy (heat for stiffness, cold for swelling), Epsom salt baths, turmeric with black pepper, fish oil supplements, ginger, massage with essential oils, low-impact exercise
■ DIET & NUTRITION	Anti-inflammatory diet: omega-3 fatty acids (fatty fish, walnuts, flaxseed), colorful fruits and vegetables, olive oil, avoid processed foods, sugar, red meat; maintain healthy weight
■■ PRECAUTIONS	Weight management (each lb lost = 4 lb less knee stress), low-impact exercise (swimming, cycling), physical therapy, assistive devices, ergonomic modifications, joint protection techniques
■ WHEN TO GO TO HOSPITAL	Sudden severe joint pain (infection/gout?), joint swelling with fever, significant joint deformity, failure of conservative treatment, considering joint replacement
■■ RECOVERY TIME	Chronic progressive condition; proper management maintains function for decades

Rheumatoid Arthritis

Category: Musculoskeletal/Autoimmune

■ APPLICABLE TO	Adults (women more than men)
■ SYMPTOMS	Symmetrical joint inflammation (both hands, both wrists), morning stiffness lasting more than 1 hour, warm swollen joints, fatigue, fever, weight loss, rheumatoid nodules, systemic complications (heart, lungs, eyes)
■ MEDICATIONS	DMARDs (Disease-Modifying): Methotrexate (gold standard), Hydroxychloroquine, Sulfasalazine; Biologics (if DMARDs insufficient): TNF inhibitors (Adalimumab, Etanercept), JAK inhibitors (Baricitinib); Corticosteroids (bridge therapy)

■ HOME REMEDIES	Heat therapy for stiffness (morning), cold for acute inflammation, omega-3 supplementation (reduces inflammation), turmeric, regular gentle exercise, stress management
■ DIET & NUTRITION	Mediterranean diet, omega-3 fatty acids, vitamin D and calcium (osteoporosis prevention - methotrexate/steroids affect bone), anti-inflammatory spices, avoid high-purine foods
■■ PRECAUTIONS	Never stop DMARDs without rheumatologist approval, monitor for medication toxicity (Methotrexate: liver/lung, regular blood tests), infection vigilance (immunosuppressed), vaccinations before biologics, regular eye exams (Hydroxychloroquine)
■ WHEN TO GO TO HOSPITAL	Disease flare not responding to medications, suspected joint infection, breathing difficulty (lung involvement), new neurological symptoms, fever with joint swelling
■■ RECOVERY TIME	Chronic autoimmune condition; early aggressive treatment achieves remission in many patients

Osteoporosis

Category: Musculoskeletal

■ APPLICABLE TO	Postmenopausal women, Elderly men
■ SYMPTOMS	Usually silent until fracture. Fractures from minimal trauma (vertebral, hip, wrist), loss of height, stooped posture (kyphosis), back pain from vertebral fractures. Diagnosed by DEXA scan (T-score below -2.5)
■ MEDICATIONS	Bisphosphonates (Alendronate, Risedronate, Zoledronic acid - first-line), Denosumab, Teriparatide (severe), Hormone therapy (menopause), Calcium (1200mg/day) + Vitamin D (800-2000 IU/day) supplements
■ HOME REMEDIES	Weight-bearing exercise (walking, dancing), resistance training, balance exercises (fall prevention), adequate calcium and vitamin D from diet, limit alcohol, quit smoking
■ DIET & NUTRITION	Calcium-rich foods (dairy, fortified foods, sardines, tofu, kale), vitamin D (fatty fish, fortified milk), adequate protein, limit caffeine and alcohol, avoid excessive vitamin A
■■ PRECAUTIONS	Fall prevention paramount (remove hazards, good lighting, grab bars), DEXA scan screening (women 65+, men 70+, or earlier if risk factors), calcium and vitamin D supplements, quit smoking
■ WHEN TO GO TO HOSPITAL	Fracture with minimal trauma, acute back pain (vertebral fracture), DEXA scan for diagnosis, medication initiation and monitoring
■■ RECOVERY TIME	Chronic condition; fractures can heal with proper management; bisphosphonates take 1-2 years to show full effect

Gout

Category: Musculoskeletal

■ APPLICABLE TO	Adults (men more than women)
■ SYMPTOMS	Sudden severe joint pain (especially big toe - podagra), joint swelling, redness, warmth, extreme tenderness (even touch of bedsheet painful), attacks typically at night, resolve in 7-10 days if untreated
■ MEDICATIONS	Acute attack: NSAIDs (Indomethacin, Naproxen), Colchicine (most effective if started early), Corticosteroids; Chronic prevention: Allopurinol, Febuxostat (uric acid-lowering), Probenecid
■ HOME REMEDIES	Ice pack on affected joint (20 min several times daily), elevate affected joint, cherry juice (decreases uric acid), rest affected joint, hydration (flush uric acid)
■ DIET & NUTRITION	AVOID: high-purine foods (red meat, organ meats, shellfish, anchovies), alcohol (especially beer), fructose-sweetened beverages; PROMOTE: low-fat dairy, cherries, coffee, adequate hydration (2-3L water/day)
■■ PRECAUTIONS	Maintain uric acid below 6 mg/dL with medication, dietary modifications crucial, avoid dehydration, avoid aspirin (raises uric acid), protect joints during acute attacks
■ WHEN TO GO TO HOSPITAL	First gout attack (confirm diagnosis, rule out infection), fever with joint inflammation (septic arthritis?), joint aspiration for diagnosis, kidney stones (uric acid stones)
■■ RECOVERY TIME	Acute attack: 7-14 days; with uric acid-lowering therapy, attacks decrease and eventually stop

Lower Back Pain (Lumbar)

Category: Musculoskeletal

■ APPLICABLE TO	Adults (most common cause of disability worldwide)
■ SYMPTOMS	Pain in lower back (lumbosacral region), may radiate down one or both legs (sciatica — pain travels below knee following nerve path), muscle spasm, stiffness especially after rest, pain worsened by bending, twisting, lifting; acute: sudden onset; chronic: persisting more than 3 months
■ MEDICATIONS	NSAIDs (Ibuprofen, Naproxen, Diclofenac — most effective), Acetaminophen (less effective than NSAIDs for back pain), Muscle relaxants (Cyclobenzaprine, Baclofen — short-term), Opioids (only for severe refractory short-term), Epidural steroid injections (radiculopathy/sciatica), surgery for herniated disc with neurological deficit
■ HOME REMEDIES	Active movement (NOT bed rest — movement is medicine), heat for muscle spasm (NOT ice for chronic), ice for acute injury first 48-72 hours, TENS machine, yoga (proven effective for chronic back pain), swimming, stretching, core strengthening exercises
■ DIET & NUTRITION	Anti-inflammatory diet (omega-3 rich foods, colorful vegetables, turmeric), maintain healthy weight (most important modifiable factor — every kg reduces lumbar load), calcium and vitamin D for bone health, magnesium for muscle relaxation

■■ PRECAUTIONS	Proper lifting technique (bend knees, not back), ergonomic workstation setup, maintain healthy weight, exercise regularly (core strength is protective), avoid prolonged sitting (hourly breaks), appropriate mattress support, physiotherapy for persistent pain
■ WHEN TO GO TO HOSPITAL	Cauda equina syndrome (saddle area numbness, bladder/bowel dysfunction — EMERGENCY), new neurological deficit (foot drop), back pain with fever (spinal infection), severe unrelenting night pain (cancer?), pain after significant trauma
■■ RECOVERY TIME	Acute non-specific back pain: 90% recover within 6 weeks; chronic back pain: multidisciplinary approach with physiotherapy, psychology, medication most effective; surgery rarely needed

Frozen Shoulder (Adhesive Capsulitis)

Category: Musculoskeletal

■ APPLICABLE TO	Adults (40-60 years, Women more than Men, Diabetics high risk)
■ SYMPTOMS	Gradual onset shoulder pain and progressive stiffness in three stages: 1) Freezing (painful stage — increasing pain, 6 weeks to 9 months); 2) Frozen (stiff stage — pain decreases but severe stiffness, 4-6 months); 3) Thawing (gradual improvement, 6 months to 2 years); difficulty with behind-back movements, hair combing, dressing
■ MEDICATIONS	NSAIDs (Ibuprofen, Naproxen) for pain; Corticosteroid injection into shoulder joint (most effective short-term); Hydrodilatation (saline injection to stretch capsule); Physical therapy (primary treatment); Manipulation under anesthesia (MUA); Arthroscopic capsular release (surgery) for refractory cases
■ HOME REMEDIES	Gentle pendulum exercises (swing arm in circles using gravity — Codman exercises), heat before exercises, ice after exercises, stretching into pain tolerance, Epsom salt warm shoulder baths, regular gentle range of motion exercises daily, no complete rest
■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids, vitamin D and calcium for joint health, turmeric and ginger (anti-inflammatory), strict diabetes control if diabetic (primary risk factor), magnesium for muscle relaxation
■■ PRECAUTIONS	Strict diabetes control (diabetics have much worse outcomes), physical therapy is essential — home exercises must be done daily, expect slow progress (2-3 years for full resolution), corticosteroid injection most effective in freezing stage, do not completely immobilize
■ WHEN TO GO TO HOSPITAL	Severe pain not controlled with medication, for corticosteroid injection, physiotherapy referral, surgical consultation for frozen shoulder lasting more than 2-3 years without improvement
■■ RECOVERY TIME	Self-limiting condition but slow: complete resolution in 2-3 years without treatment; with treatment: 1-2 years; full recovery expected in 90%

Plantar Fasciitis

Category: Musculoskeletal

■ APPLICABLE TO	Adults (especially Runners, Overweight, People who Stand Long Hours)
■ SYMPTOMS	Stabbing heel pain worst with first steps in morning or after prolonged rest (post-static dyskinesia), pain that warms up during activity but returns after rest, pain on the bottom of the heel, tenderness on pressing the bottom of heel, tight Achilles tendon
■ MEDICATIONS	NSAIDs (Ibuprofen, Naproxen — first-line), Corticosteroid injection into plantar fascia (caution: risk of fascia rupture with multiple injections), PRP (Platelet-Rich Plasma) injection (emerging evidence), Extracorporeal Shockwave Therapy (ESWT), surgery (plantar fascia release — last resort)
■ HOME REMEDIES	Calf and plantar fascia stretching exercises (most important home treatment — Achilles stretch, towel stretch, marble pickup), icing heel with frozen water bottle rolling for 20 minutes, good supportive footwear, silicone heel cups, night splints (stretch fascia while sleeping), avoid barefoot walking on hard floors
■ DIET & NUTRITION	Anti-inflammatory diet, maintain healthy weight (reduces heel load), omega-3 fatty acids, reduce uric acid foods if gout co-exists, vitamin D and calcium for bone health
■■ PRECAUTIONS	Stretching exercises BEFORE getting out of bed in morning (before first steps), supportive shoes with arch support always (no flat shoes or flip-flops), gradual return to exercise, proper running shoes, correct biomechanical issues (overpronation) with orthotics
■ WHEN TO GO TO HOSPITAL	Severe pain not improving after 6 weeks of conservative treatment, for corticosteroid injection, shockwave therapy, podiatric assessment for orthotics, surgical consultation for chronic refractory plantar fasciitis (more than 12 months)
■■ RECOVERY TIME	90% resolve with conservative treatment within 6-12 months; stretching exercises are key — do 3 times daily consistently

Fibromyalgia

Category: Musculoskeletal / Chronic Pain

■ APPLICABLE TO	Adults (Women 7x more than Men)
■ SYMPTOMS	Widespread body pain (above and below waist, both sides, axial skeleton) for 3+ months; tender points at specific sites; fatigue (often debilitating); non-restorative sleep (waking unrefreshed); cognitive impairment (fibro fog — memory and concentration); headaches; IBS symptoms; sensitivity to touch, temperature, and light; anxiety and depression often co-occur
■ MEDICATIONS	Duloxetine (Cymbalta — FDA-approved, SNRIs effective for pain and fatigue), Milnacipran (Savella), Pregabalin (Lyrica — FDA-approved for fibromyalgia), Amitriptyline low dose (sleep and pain), Cyclobenzaprine (muscle relaxant at night); Aerobic exercise is the single most effective treatment; CBT; Tramadol for severe pain (avoid opioids — worsen long-term)

■ HOME REMEDIES	Aerobic exercise (most evidence-based: start low, progress slowly — swimming, cycling, walking), warm baths and hydrotherapy, Tai Chi and yoga (proven benefit in studies), pacing (energy management), sleep hygiene, heat therapy, gentle massage, mindfulness-based stress reduction
■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids, vitamin D (deficiency very common in fibromyalgia), magnesium glycinate (muscle relaxation and sleep), avoid artificial sweeteners (especially aspartame — can worsen pain in some patients), avoid gluten (some patients have non-celiac gluten sensitivity), adequate sleep-promoting foods
■■ PRECAUTIONS	Diagnosis of exclusion (rule out lupus, RA, hypothyroidism, anemia), multidisciplinary approach most effective (physiotherapy + psychology + medication), avoid opioids and benzodiazepines (worsen fibromyalgia long-term), realistic expectations (management not cure), work accommodations often needed
■ WHEN TO GO TO HOSPITAL	For initial evaluation and diagnosis (exclude other conditions), medication initiation, pain management specialist, psychological support, multidisciplinary pain management program
■■ RECOVERY TIME	Chronic condition; improvement in 30-40% with multimodal treatment; aerobic exercise gives best sustained benefit; quality of life significantly improvable

Scoliosis

Category: Musculoskeletal

■ APPLICABLE TO	Children and Adolescents (also Adults with degenerative scoliosis)
■ SYMPTOMS	Visible curve of spine (visible when bending forward — Adam's forward bend test); uneven shoulders, waist, or hips; one shoulder blade more prominent; back pain in adults; in severe cases: breathing difficulty (chest curve compresses lungs), cardiac complications; Adolescent Idiopathic Scoliosis (most common) — unknown cause; detected on school screening
■ MEDICATIONS	Observation (curves below 25 degrees in growing child); Bracing (TLSO brace — Boston or Milwaukee for 20-degree+ curves in growing adolescents, worn 18-23 hours/day); Surgical correction: Spinal fusion with instrumentation (Harrington rod or pedicle screws) for curves above 45-50 degrees or rapidly progressive
■ HOME REMEDIES	Scoliosis-specific exercise (Schroth method — evidence-based physiotherapy), swimming (excellent for back muscles), Pilates-based core strengthening, avoid heavy backpacks (one-shoulder especially), posture awareness, compliance with brace if prescribed (critical for effectiveness)
■ DIET & NUTRITION	Calcium and Vitamin D for bone health (prevent osteoporosis which worsens adult scoliosis), adequate protein for growth, anti-inflammatory diet, vitamin K2 (directs calcium to bones)
■■ PRECAUTIONS	Regular X-ray monitoring during adolescent growth (every 6 months in growing phase), brace compliance is essential (must be worn 18-23 hours daily to be effective), Schroth physiotherapy in addition to bracing, screen all family members (genetic component), post-surgical mobility and physiotherapy

■ WHEN TO GO TO HOSPITAL	Curve above 25 degrees (bracing evaluation), curve above 45 degrees (surgical evaluation), rapidly progressing curve, breathing problems, back pain in child with scoliosis, adult with progressive scoliosis
■■ RECOVERY TIME	Bracing: stabilizes curve in 72% of compliant patients; Surgical fusion: excellent long-term results; curve correction 50-70%; adult degenerative scoliosis: pain management ongoing

Tendinitis (Tendinopathy)

Category: Musculoskeletal

■ APPLICABLE TO	Adults (Athletes and Overuse Workers)
■ SYMPTOMS	Pain and tenderness at or around a tendon (most common: Achilles tendinitis, patellar tendinitis, rotator cuff tendinopathy, tennis elbow, golfer's elbow, De Quervain's tenosynovitis); pain that worsens with movement and repetitive activity; morning stiffness; swelling; local warmth; typically develops gradually with overuse
■ MEDICATIONS	NSAIDs (Ibuprofen, Naproxen) for short-term pain relief; Topical NSAIDs (Diclofenac gel — effective with fewer systemic side effects); Corticosteroid injection (short-term relief — multiple injections weaken tendon, risk of rupture); PRP (Platelet-Rich Plasma) injections (growing evidence); Physiotherapy (primary treatment); Surgery for chronic refractory cases
■ HOME REMEDIES	RICE protocol: Rest, Ice (20 min, 4-5 times daily), Compression, Elevation; Eccentric exercise program (gold standard for Achilles and patellar tendinopathy — lengthening contractions heal tendinopathy); massage with Tiger Balm or arnica gel; TENS machine; gradual return to sport
■ DIET & NUTRITION	Anti-inflammatory diet, omega-3 fatty acids, vitamin C (collagen synthesis for tendon healing), gelatin/collagen supplements (Type 1 collagen — emerging evidence for tendon repair), adequate protein for tissue repair, turmeric and ginger
■■ PRECAUTIONS	Warm up properly before exercise, gradual training load increases (10% per week rule), correct technique and biomechanics, appropriate supportive footwear, avoid cortisone injection directly into tendon (rupture risk), eccentric loading program is essential for recovery
■ WHEN TO GO TO HOSPITAL	Tendon rupture (sudden snap, severe pain, inability to use muscle — e.g., Achilles rupture: cannot rise on tiptoe — URGENT ORTHOPAEDIC), failure to improve after 6 weeks of physiotherapy, corticosteroid injection assessment, surgical evaluation for chronic tendinopathy
■■ RECOVERY TIME	Acute tendinitis: 4-6 weeks with rest and physiotherapy; Chronic tendinopathy: 3-6 months with eccentric exercise program; Tendon rupture: surgical repair with 6-9 months rehabilitation

Eye, Ear & Nose Conditions

8 Conditions Covered — 8 Guidance Fields Each

Conjunctivitis (Pink Eye)

Category: Eye Condition

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Red/pink eye, itching (allergic), discharge (watery-viral, thick yellow/green-bacterial), crusting on eyelids especially morning, swollen eyelids, sensitivity to light, feeling of grit in eye
■ MEDICATIONS	Viral: Supportive (artificial tears, cold compress) - self-limiting; Bacterial: Antibiotic eye drops (Ciprofloxacin, Tobramycin, Moxifloxacin); Allergic: Antihistamine drops (Olopatadine), Ketorolac
■ HOME REMEDIES	Warm compress (loosens crusts), cold compress (allergic - reduces swelling), saline eye wash, clean discharge with clean cotton, chamomile tea bags cooled on eyes (folk remedy)
■ DIET & NUTRITION	Vitamin A rich foods (protective for eyes), avoid rubbing eyes, adequate sleep, vitamin C for immune support
■■ PRECAUTIONS	Highly contagious (viral/bacterial): frequent handwashing, don't share towels/pillowcases, don't rub eyes, complete antibiotic course, stay home from school/work (bacterial until treated 24 hours)
■ WHEN TO GO TO HOSPITAL	Severe eye pain, vision changes, sensitivity to light (photophobia), severe swelling around eye, suspected corneal involvement, newborn with eye discharge (urgent), immune-compromised patient
■■ RECOVERY TIME	Viral: 7-14 days; Bacterial: 5-7 days with antibiotics

Cataracts

Category: Eye Condition

■ APPLICABLE TO	Elderly (usually 60+)
■ SYMPTOMS	Cloudy/blurry vision, difficulty seeing at night, sensitivity to light and glare, seeing halos around lights, fading/yellowing of colors, double vision in one eye, frequent prescription changes
■ MEDICATIONS	No medication prevents or treats cataracts; Surgery (phacoemulsification with intraocular lens implant) is the only treatment; Anti-VEGF drugs are for AMD not cataracts
■ HOME REMEDIES	Better lighting for reading, anti-glare sunglasses, magnifying lenses (temporary measures only), protect eyes from UV with sunglasses

■ DIET & NUTRITION	Antioxidant-rich diet (lutein/zeaxanthin - leafy greens, eggs), vitamin C (citrus, berries), omega-3 fatty acids, avoid smoking (increases risk), limit alcohol
■■ PRECAUTIONS	UV-protective sunglasses (prevent progression), annual eye exams, control diabetes (accelerates cataracts), avoid steroid eye drops long-term if possible, surgery discussion when affecting quality of life
■ WHEN TO GO TO HOSPITAL	Significant vision impairment affecting daily activities (driving, reading), for surgical consultation, sudden vision changes (may not be cataract)
■■ RECOVERY TIME	Cataract surgery: outpatient; vision improves within days; full recovery 4-6 weeks; success rate 99%

Glaucoma

Category: Eye Condition

■ APPLICABLE TO	Adults (especially over 60, High risk: family history, Diabetes, Myopia, Afro-Caribbean ethnicity)
■ SYMPTOMS	Open-angle glaucoma (most common): no symptoms initially (silent — regular screening critical), gradual peripheral vision loss, tunnel vision in advanced stages. Acute angle-closure glaucoma: sudden severe eye pain, headache, nausea, vomiting, blurred vision with halos, red eye — EMERGENCY
■ MEDICATIONS	Eye drops (long-term): Prostaglandin analogues (Latanoprost, Bimatoprost — first-line, once daily), Beta-blockers (Timolol), Alpha-agonists (Brimonidine), Carbonic anhydrase inhibitors (Dorzolamide, Brinzolamide); Laser trabeculoplasty (SLT); Surgery (Trabeculectomy, Tube shunt) for uncontrolled IOP
■ HOME REMEDIES	Regular eye pressure lowering eye drops as prescribed (never skip doses), regular aerobic exercise (lowers IOP), avoid inverted yoga positions (raise IOP), adequate sleep (head elevated 30 degrees), avoid tight neckwear, manage stress
■ DIET & NUTRITION	Dark leafy greens (nitrates convert to nitric oxide — improves optic nerve blood flow), omega-3 fatty acids, vitamin C and E antioxidants, adequate hydration (avoid large volumes of water quickly — raises IOP transiently), limit caffeine, no excessive alcohol
■■ PRECAUTIONS	Annual eye pressure and optic nerve checks for at-risk individuals, regular visual field testing, never miss eye drops, inform dentist and anesthesiologist about glaucoma medications, regular glaucoma specialist follow-up, genetic counseling for hereditary cases
■ WHEN TO GO TO HOSPITAL	Acute angle-closure glaucoma: IMMEDIATELY (eye pain, nausea, halos around lights, very red eye — this is an emergency and can cause permanent blindness within hours); regular glaucoma checks as recommended
■■ RECOVERY TIME	Open-angle glaucoma: lifelong condition managed with drops; vision already lost is permanent; drops prevent further loss; acute angle-closure: emergency laser iridotomy is curative

Macular Degeneration (AMD)

Category: Eye Condition

■ APPLICABLE TO	Elderly (usually 55+)
■ SYMPTOMS	Dry AMD (85%): gradual central vision blurring, drusen (yellow deposits under retina on exam), mild visual distortion, difficulty reading and recognizing faces, need for more light; Wet AMD (15% but causes 90% of severe vision loss): rapid central vision distortion, straight lines appear wavy (Amsler grid test), central scotoma (blank spot in central vision)
■ MEDICATIONS	Dry AMD: no proven treatment to halt progression; AREDS2 supplements (Lutein 10mg, Zeaxanthin 2mg, Vitamin C 500mg, Vitamin E 400IU, Zinc 80mg, Copper 2mg) reduce progression risk; Wet AMD: Anti-VEGF injections (Ranibizumab/Lucentis, Bevacizumab/Avastin, Aflibercept/Eylea, Faricimab/Vabysmo) injected into the eye monthly initially
■ HOME REMEDIES	Amsler grid monitoring at home (self-monitoring for wet AMD conversion — check daily), UV-blocking sunglasses, magnifying aids for reading, brighter lighting, anti-VEGF treatment promptly if wet AMD suspected, smoking cessation
■ DIET & NUTRITION	Lutein and zeaxanthin rich foods (kale, spinach, eggs, corn — protective), omega-3 fatty acids (fatty fish), vitamin C and E antioxidants (colorful fruits and vegetables), AREDS2 supplement formulation, no smoking (major risk factor — doubles AMD risk), limit processed foods
■■ PRECAUTIONS	Amsler grid daily self-monitoring (report any new distortion immediately — wet AMD can worsen in days), annual retinal examination, UV-blocking sunglasses, no smoking, AREDS2 supplements for intermediate AMD, low vision rehabilitation services
■ WHEN TO GO TO HOSPITAL	Any sudden change in central vision, new distortion on Amsler grid, dark spots in central vision — same-day ophthalmology assessment for wet AMD (anti-VEGF within days maximizes outcome)
■■ RECOVERY TIME	Dry AMD: slowly progressive, no reversal; Wet AMD: anti-VEGF injections halt or improve vision in 90% if started early; monthly to quarterly injections long-term

Tinnitus

Category: Ear Condition

■ APPLICABLE TO	Adults (especially with noise exposure or hearing loss)
■ SYMPTOMS	Perception of sound (ringing, buzzing, hissing, humming, clicking) in one or both ears with no external source; may be constant or intermittent; mild and ignorable or severely distressing; often associated with hearing loss; can cause sleep disturbance, anxiety, depression, and concentration problems; pulsatile tinnitus (heartbeat sound) may indicate vascular cause

■ MEDICATIONS	No specific medication cures tinnitus; treat underlying cause (ear wax, infection, hearing loss, hypertension, thyroid); Cognitive Behavioral Therapy (CBT) — most evidence-based treatment for distress; sound therapy (masking), hearing aids if hearing loss present; antidepressants or anxiolytics if severe anxiety or depression
■ HOME REMEDIES	Sound masking (white noise machine, fan, nature sounds especially at night), reduce caffeine and alcohol (may temporarily worsen tinnitus), protect hearing from loud noise (earplugs at concerts), relaxation techniques (tinnitus worsens with anxiety), ginkgo biloba (limited evidence)
■ DIET & NUTRITION	Reduce salt (especially if Meniere's disease associated), reduce caffeine and alcohol, magnesium supplementation (may help noise-induced tinnitus), zinc (deficiency associated with tinnitus), avoid quinine (in tonic water — can worsen tinnitus)
■■ PRECAUTIONS	Hearing protection in loud environments, keep ear canal clean (wax impaction worsens tinnitus), manage stress and anxiety (strongest predictor of tinnitus severity), avoid silence (use background sound to reduce perception), tinnitus retraining therapy (TRT), limit exposure to loud noise
■ WHEN TO GO TO HOSPITAL	Pulsatile tinnitus (may indicate vascular or intracranial cause — needs imaging), sudden hearing loss with tinnitus (emergency — steroids within 24-48 hours), unilateral tinnitus (rule out acoustic neuroma), associated vertigo, tinnitus causing severe depression or suicidal ideation
■■ RECOVERY TIME	Chronic condition for most; some tinnitus resolves spontaneously; CBT reduces distress significantly in 6-12 months; habituation (tinnitus less noticed) achievable in most

Vertigo (BPPV — Benign Paroxysmal Positional Vertigo)

Category: Ear/Neurological

■ APPLICABLE TO	Adults (most common cause of dizziness)
■ SYMPTOMS	Sudden brief episodes of intense spinning sensation (vertigo) triggered by specific head movements (rolling over in bed, looking up, bending over); lasts less than 1 minute per episode; associated nausea and unsteadiness; Dix-Hallpike test is diagnostic; recurrent episodes
■ MEDICATIONS	Epley maneuver (canalith repositioning — primary treatment — 80% effective in one to two sessions); Semont maneuver (alternative); Vestibular suppressants for acute nausea: Prochlorperazine, Betahistine (for Meniere's), Meclizine (short-term only — avoid prolonged use, delays central compensation); Vestibular rehabilitation therapy (VRT)
■ HOME REMEDIES	Sleep with head elevated (45 degrees) for 2 nights after Epley maneuver; avoid trigger positions for 48 hours; Brandt-Daroff exercises (self-treatment exercises done at home 3 times/day for 2 weeks); stay hydrated; fall prevention measures during episodes

■ DIET & NUTRITION	Low sodium diet if Meniere's disease associated (reduces endolymph fluid); adequate hydration; vitamin D supplementation (BPPV associated with vitamin D deficiency — supplement if deficient); avoid caffeine and alcohol (Meniere's trigger)
■■ PRECAUTIONS	Do not drive during active vertigo episodes, fall prevention (grab rails, avoid ladders), Brandt-Daroff exercises for recurrent BPPV, identify and treat vitamin D deficiency (reduces BPPV recurrence rate), caution when getting up from bed
■ WHEN TO GO TO HOSPITAL	New severe vertigo with headache (rule out cerebellar stroke — EMERGENCY), vertigo with double vision, weakness, numbness, difficulty swallowing, unilateral hearing loss, vertigo lasting hours (Meniere's?), first episode without obvious trigger
■■ RECOVERY TIME	BPPV with Epley maneuver: 80% resolve in 1-2 sessions; 50% recurrence rate within 5 years; Brandt-Daroff exercises prevent recurrence

Otitis Externa (Swimmer's Ear)

Category: Ear Condition

■ APPLICABLE TO	All Ages (Common in Swimmers)
■ SYMPTOMS	Ear canal pain (worse when pulling outer ear or pressing the small bump in front of ear canal — tragus), itching inside ear canal, redness and swelling of ear canal, drainage (clear, white, yellow, or pus), muffled hearing, feeling of fullness in ear, tenderness; caused by bacterial (usually Pseudomonas) or fungal infection of outer ear canal
■ MEDICATIONS	Antibiotic ear drops (Ciprofloxacin + Dexamethasone — Ciloxan, or Gentamicin, or Neomycin/polymyxin B/hydrocortisone combination); Antifungal ear drops if fungal (Clotrimazole); Keep ear dry; If severe: oral antibiotics, ear canal debridement; Pain relief: oral Ibuprofen or Paracetamol; Ear wick for swollen canal (helps drops penetrate)
■ HOME REMEDIES	Keep ear dry (avoid swimming, use cotton wool with Vaseline or swimmer's ear plug when bathing), diluted white vinegar drops in ear (restores normal acidic pH — preventive especially after swimming), warm compress for pain, do not put fingers or cotton buds in ear (trauma worsens infection)
■ DIET & NUTRITION	No specific dietary restrictions; adequate hydration for immune support; vitamin C and zinc for infection fighting; avoid sharing earbuds during infection
■■ PRECAUTIONS	Dry ears thoroughly after swimming (tip head each side, use clean towel, hairdryer on low setting at distance), avoid cotton buds (push wax deeper and traumatize canal skin), use acetic acid preventive drops after swimming if recurrent, swimmer's ear plugs for regular swimmers
■ WHEN TO GO TO HOSPITAL	Severe pain, fever, spreading cellulitis of outer ear and cheek (malignant otitis externa in diabetics — serious infection requiring IV antibiotics), no improvement after 3 days of drops, canal too swollen to insert drops, hearing loss
■■ RECOVERY TIME	With appropriate antibiotic drops: improvement in 3-5 days; complete resolution in 7-10 days; keep ear dry for 2-3 weeks to prevent recurrence

Presbyopia (Age-Related Vision Changes)

Category: Eye Condition

■ APPLICABLE TO	Adults (typically over 40)
■ SYMPTOMS	Difficulty reading small print (holding phone or book at arm's length), difficulty seeing near objects clearly, eyestrain and headache with near work, need for brighter light for reading, improved distance vision in people who previously needed reading glasses for distance (minus glasses), blurred vision when switching between near and far
■ MEDICATIONS	Reading glasses (OTC or prescription); Bifocal, trifocal, or progressive lenses (also correct distance); Multifocal contact lenses; Monovision contact lenses (one eye distance, one near); Corneal inlays (Kamra — surgical); LASIK monovision (refractive surgery); Increasing accommodation surgical options (AcuFocus)
■ HOME REMEDIES	Adequate lighting for reading (LED lamp pointing at page), increase phone font size, e-reader with adjustable text size, magnifying glass for occasional use, take breaks from near work (20-20-20 rule), eye exercises (may slightly delay progression but do not reverse presbyopia)
■ DIET & NUTRITION	Vitamin A (carrots, sweet potatoes, leafy greens), lutein and zeaxanthin (kale, eggs — protective for retina), omega-3 fatty acids, vitamin C and E antioxidants, adequate hydration, no smoking (accelerates eye aging)
■■ PRECAUTIONS	Annual eye examinations after 40 (to monitor presbyopia progression and detect other age-related eye conditions — glaucoma, AMD, cataracts), update prescription regularly, reading glasses are not harmful, UV-blocking sunglasses
■ WHEN TO GO TO HOSPITAL	Annual eye examination for prescription update, new symptoms beyond presbyopia (floaters, flashes, sudden vision changes — not normal), co-existing eye conditions (glaucoma, AMD, diabetic retinopathy screening)
■■ RECOVERY TIME	Progressive condition; presbyopia worsens until about age 65 then stabilizes; corrective lenses needed lifelong; reading glasses or multifocals provide excellent function

Infectious Diseases

12 Conditions Covered — 8 Guidance Fields Each

Malaria

Category: Infectious Disease

■ APPLICABLE TO	All Ages (high risk: children, pregnant women, travelers)
■ SYMPTOMS	Cyclical fever (every 48-72 hours), chills and rigors, sweating, headache, muscle aches, nausea/vomiting, anemia, enlarged spleen, in severe malaria: cerebral malaria, respiratory distress, organ failure
■ MEDICATIONS	Depends on species and region: Artemisinin-based Combination Therapy (ACT) - gold standard (Artemether-Lumefantrine, Artesunate-Amodiaquine); Chloroquine (P. vivax in sensitive areas); IV Artesunate (severe malaria); Prevention: Hydroxychloroquine, Doxycycline, Mefloquine
■ HOME REMEDIES	Supportive: fever management with Acetaminophen (not Aspirin), hydration, rest; neem and artemisinin are traditional sources of antimalarials but MUST use proper medications
■ DIET & NUTRITION	High calorie and protein to fight infection, plenty of fluids to prevent dehydration, easily digestible foods, iron-rich foods for anemia recovery, avoid alcohol
■■ PRECAUTIONS	Malaria prevention in endemic areas: DEET-based insect repellent, permethrin-treated clothing and bed nets, chemoprophylaxis when traveling, eliminate standing water (mosquito breeding), window screens
■ WHEN TO GO TO HOSPITAL	All suspected malaria cases need diagnosis confirmation (blood smear/RDT); Any malaria in children and pregnant women is urgent; severe malaria symptoms (confusion, breathing difficulty, jaundice, dark urine) = emergency
■■ RECOVERY TIME	Uncomplicated malaria: 3-7 days with ACT; P. vivax/ovale may need primaquine to prevent relapse

Dengue Fever

Category: Infectious Disease

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Sudden high fever (102-104°F), severe headache, pain behind eyes, severe joint and muscle pain (breakbone fever), rash, mild bleeding (nose, gums), fatigue; Warning signs: severe abdominal pain, persistent vomiting, bleeding, rapid breathing (Dengue Hemorrhagic Fever)
■ MEDICATIONS	NO specific antiviral; Acetaminophen for fever (AVOID NSAIDs and Aspirin - increase bleeding risk), IV fluid replacement (critical in severe dengue), platelet transfusion if critically low, hospitalization for monitoring

■ HOME REMEDIES	Papaya leaf juice (raises platelets - evidence mixed but commonly used), coconut water for hydration, goat milk (folk remedy), plenty of fluids, rest; Caution: avoid high-dose vitamin E or fish oil (anti-platelet effects)
■ DIET & NUTRITION	Plenty of fluids (ORS, coconut water, fruit juices), papaya leaf juice, easily digestible soft foods, high protein diet for recovery, avoid spicy and fatty foods
■■ PRECAUTIONS	Aedes mosquito control (eliminate standing water, use repellent), wear long sleeves/trousers, dengue mosquitoes bite during DAY, seek medical care promptly if warning signs develop, daily platelet monitoring if severe
■ WHEN TO GO TO HOSPITAL	Warning signs: severe abdominal pain, persistent vomiting, bleeding, rapid breathing, restlessness, pallor, or any dengue in children/pregnant women/elderly
■■ RECOVERY TIME	Uncomplicated: 7-10 days; severe dengue may require 2+ weeks and intensive care

COVID-19

Category: Infectious Disease

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Fever, cough, shortness of breath, fatigue, body aches, headache, loss of taste/smell, sore throat, nasal congestion, diarrhea; Severe: difficulty breathing, chest pain, confusion, bluish lips
■ MEDICATIONS	Antivirals: Nirmatrelvir-Ritonavir (Paxlovid - within 5 days for high-risk), Remdesivir (hospitalized), Molnupiravir; Dexamethasone (severe/oxygen-requiring), Baricitinib/Tocilizumab (ICU); Vaccination (primary prevention)
■ HOME REMEDIES	Rest, hydration, steam inhalation, honey-ginger-lemon tea, zinc lozenges, vitamin C and D supplementation, monitor oxygen saturation with pulse oximeter
■ DIET & NUTRITION	Plenty of fluids, nutritious foods, vitamin C and zinc rich foods, anti-inflammatory diet, high protein for recovery, light easily digestible foods if nausea
■■ PRECAUTIONS	Vaccination (most important), isolation when infected, mask in high-risk settings, hand hygiene, ventilation, monitor oxygen saturation at home (below 94% seek care), high-risk individuals seek early antiviral treatment
■ WHEN TO GO TO HOSPITAL	Oxygen saturation below 94%, persistent chest pain/pressure, confusion, inability to stay awake, bluish lips, trouble breathing, worsening symptoms after 5 days
■■ RECOVERY TIME	Mild: 5-14 days; Long COVID symptoms may persist weeks to months

Typhoid Fever

Category: Infectious Disease

■ APPLICABLE TO	All Ages (especially children and young adults)

■ SYMPTOMS	Sustained high fever (rising stepwise 99-104°F over first week), relative bradycardia, headache, abdominal pain, constipation or diarrhea, rose spots on trunk, splenomegaly, liver enlargement, general malaise
■ MEDICATIONS	Antibiotics: Azithromycin (mild-moderate, oral), Ceftriaxone (IV, severe), Fluoroquinolones (Ciprofloxacin - check resistance), Chloramphenicol (older drug); Acetaminophen for fever; Complete full course (10-14 days)
■ HOME REMEDIES	Complete bed rest (intestinal perforation risk with activity), plenty of fluids, ORS, fever management, light easily digestible diet
■ DIET & NUTRITION	High calorie easily digestible diet: soft cooked foods, soups, mashed potatoes, rice, bananas, boiled vegetables; avoid raw vegetables, high fiber foods, hard to digest foods; small frequent meals; no spicy or fatty foods
■■ PRECAUTIONS	Safe water and food (typhoid is fecal-oral): drink only boiled/bottled water, eat thoroughly cooked foods, hand hygiene before eating, typhoid vaccination for travelers to endemic areas
■ WHEN TO GO TO HOSPITAL	All suspected typhoid needs medical evaluation; complications: intestinal perforation (sudden severe abdominal pain - emergency), internal bleeding, meningitis; fever not responding to antibiotics
■■ RECOVERY TIME	2-3 weeks with antibiotics; without treatment: 3-4 weeks with significant mortality risk

Hepatitis A

Category: Infectious Disease

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Fatigue, nausea, vomiting, abdominal pain (right upper quadrant), low-grade fever, loss of appetite, dark urine (tea-colored), pale stools, jaundice (yellowing of skin and eyes); 2-6 week incubation
■ MEDICATIONS	No antiviral treatment; supportive care only: rest, fluids, avoid liver-damaging substances; Hepatitis A vaccine for prevention; IVIG for post-exposure prophylaxis within 2 weeks
■ HOME REMEDIES	Complete bed rest, high fluid intake, avoid alcohol strictly, avoid medications metabolized by liver (unless essential), herbal support: milk thistle (silymarin) to support liver
■ DIET & NUTRITION	Small frequent meals, high carbohydrate and protein, low fat, plenty of fruits and vegetables, avoid fatty/fried foods, NO ALCOHOL for 6 months, avoid hepatotoxic medications
■■ PRECAUTIONS	Vaccination (2 doses - complete protection), hand hygiene (especially after toilet), safe food and water, don't share utensils, avoid raw shellfish in endemic areas, notify close contacts for vaccination
■ WHEN TO GO TO HOSPITAL	Persistent vomiting/unable to eat, severe abdominal pain, confusion (liver failure), prolonged jaundice, very high liver enzymes, elderly or immune-compromised with hepatitis A

■■ RECOVERY TIME	Most recover completely in 1-2 months; no chronic infection; rarely leads to acute liver failure

HIV/AIDS

Category: Infectious Disease

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Acute HIV (2-4 weeks after infection): flu-like illness (fever, swollen lymph nodes, sore throat, rash, muscle aches). Chronic HIV: may be asymptomatic for years. AIDS (CD4 below 200): weight loss, night sweats, chronic diarrhea, oral thrush, recurrent infections, opportunistic infections (PCP pneumonia, CMV, toxoplasmosis, cryptococcal meningitis)
■ MEDICATIONS	Antiretroviral Therapy (ART) — lifelong: modern regimens are single-pill once-daily (Dolutegravir + Tenofovir + Emtricitabine, or Bictegravir combinations); PrEP (Pre-Exposure Prophylaxis): Tenofovir + Emtricitabine (Truvada) for HIV-negative high-risk individuals; PEP (Post-Exposure Prophylaxis): started within 72 hours; treat opportunistic infections
■ HOME REMEDIES	ART adherence (most important — never miss doses), healthy lifestyle supporting immune function, emotional wellbeing, peer support groups, adequate sleep, stress management; no herbal remedies treat HIV — use only alongside ART
■ DIET & NUTRITION	High protein diet (muscle wasting prevention), food safety (immunocompromised patients avoid raw/undercooked foods, unpasteurized dairy), micronutrient-rich diet, B12 and folate, avoid alcohol, adequate caloric intake
■■ PRECAUTIONS	Take ART daily (undetectable = Untransmittable — U=U, when viral load undetectable, cannot transmit HIV), use condoms, PrEP for partners, regular CD4 and viral load monitoring, opportunistic infection prophylaxis, annual flu and pneumonia vaccine, no live vaccines
■ WHEN TO GO TO HOSPITAL	All HIV diagnoses need specialist management; opportunistic infections (PCP, cryptococcal meningitis, CMV retinitis), rapid CD4 decline, virological failure on ART, HIV-related malignancies
■■ RECOVERY TIME	With ART: normal life expectancy; viral load becomes undetectable in 3-6 months; CD4 count recovers over 1-2 years; ART is lifelong

Cholera

Category: Infectious Disease

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Profuse watery diarrhea (rice-water stools — odorless, cloudy), sudden onset with large volumes, vomiting without nausea, rapid severe dehydration (sunken eyes, dry mouth, decreased skin turgor, rapid heart rate, low BP), muscle cramps, dehydration shock — can lead to death within hours if untreated

■ MEDICATIONS	ORS (Oral Rehydration Solution) — PRIMARY TREATMENT — can save lives when IV not available; IV Fluids (Ringer's Lactate) for severe dehydration; Antibiotics (shorten illness duration): Doxycycline (single dose adults), Azithromycin (children and pregnant women); Zinc supplementation for children
■ HOME REMEDIES	ORS preparation: 1 liter safe water + 6 teaspoons sugar + 1/2 teaspoon salt — drink as much as possible; coconut water; rice water; must continue drinking even while vomiting (small sips); cholera can be fatal within hours without fluid replacement
■ DIET & NUTRITION	ORS as primary nutrition during acute phase, then gradual reintroduction of plain foods (rice, banana, boiled potato), normal diet as tolerated; avoid raw fruits, dairy, and fatty foods initially; continue breastfeeding in infants
■■ PRECAUTIONS	Safe water and food is the ONLY prevention: boil or purify all drinking water, eat only cooked hot food, wash hands with soap and water especially before eating and after toilet, Cholera vaccine (Shanchol) for endemic areas and travelers
■ WHEN TO GO TO HOSPITAL	ALL suspected cholera cases need urgent medical attention; moderate-severe dehydration needs IV fluids; infants and elderly are especially vulnerable; do not wait for hospital — start giving ORS immediately
■■ RECOVERY TIME	With adequate rehydration: recovery in 3-5 days; mortality without treatment: 25-50%; with treatment: below 1%

Leptospirosis (Weil's Disease)

Category: Infectious Disease

■ APPLICABLE TO	All Ages (High risk: farmers, sewer workers, flood exposure)
■ SYMPTOMS	Phase 1 (acute, 5-7 days): sudden fever, severe headache, myalgia (especially calf muscles), red eyes (conjunctival suffusion), rash; apparent improvement; Phase 2 (Weil's disease, 1-3 days later): jaundice, kidney failure, bleeding, meningitis, respiratory distress; exposure to water contaminated with rat urine
■ MEDICATIONS	Mild-moderate: Doxycycline 100mg twice daily for 7 days; Severe/hospitalized: IV Penicillin G or Ceftriaxone; Supportive: IV fluids, dialysis for kidney failure, blood transfusion for bleeding; Prophylaxis for high-risk exposure: Doxycycline 200mg once weekly
■ HOME REMEDIES	Avoid wading in floodwater without protection, rubber boots in endemic areas, rat control, protective clothing, protective eyewear in high-risk settings; NO specific home remedies treat established leptospirosis
■ DIET & NUTRITION	During illness: adequate fluid intake and nutrition; kidney failure: renal diet (restrict potassium, phosphorus, fluid as directed); liver involvement: low fat, high carbohydrate; no alcohol
■■ PRECAUTIONS	Rubber boots and gloves when working in potentially contaminated water or soil, rat control around homes, avoid swimming in potentially contaminated freshwater, cover wounds, early antibiotic treatment critical for outcomes

■ WHEN TO GO TO HOSPITAL	All confirmed or suspected cases need medical evaluation; severe leptospirosis (Weil's disease) requires hospitalization; signs of kidney or liver failure, bleeding, respiratory distress need intensive care
■■ RECOVERY TIME	Mild: 1-3 weeks with antibiotics; severe Weil's disease: weeks in hospital with potential permanent organ damage

Chickenpox Prevention and Varicella Complications

Category: Infectious Disease

■ APPLICABLE TO	Unvaccinated Children and Adults
■ SYMPTOMS	Primary infection: itchy vesicular rash, fever, fatigue. Complications in adults: pneumonia (varicella pneumonia — most common serious complication in adults), encephalitis, hepatitis, secondary bacterial infection. In immunocompromised: severe disseminated disease. In pregnancy: risk of congenital varicella syndrome (limb defects, skin scarring, eye and brain abnormalities) if infection before 20 weeks
■ MEDICATIONS	Healthy children: Calamine lotion, antihistamines, Paracetamol (NEVER Aspirin — Reye's syndrome); At-risk patients: Acyclovir IV (immunocompromised), oral Acyclovir (adults, pregnant women in 2nd/3rd trimester, severe disease); VZIG (Varicella Zoster Immunoglobulin) — post-exposure prophylaxis for immunocompromised, pregnant non-immune women, newborns
■ HOME REMEDIES	Oatmeal baths for itching, baking soda paste, neem leaf water bath, keep nails trimmed (prevent scarring from scratching), cool compress, loose breathable cotton clothing, isolate until all lesions crusted
■ DIET & NUTRITION	Soft foods if oral lesions present, cold soothing foods, adequate hydration, vitamin C and zinc for immune support, adequate calories and protein for healing
■■ PRECAUTIONS	Two-dose Varicella vaccine schedule (highly effective prevention), keep unvaccinated children away from high-risk individuals (pregnant women, immunocompromised, newborns), report to healthcare provider if exposed during pregnancy (VZIG within 96 hours of exposure)
■ WHEN TO GO TO HOSPITAL	Breathing difficulty (pneumonia), confusion (encephalitis), bacterial superinfection of skin (spreading redness, warmth, fever), immunocompromised patient with chickenpox, pregnant woman exposed to chickenpox (VZIG), newborn with chickenpox
■■ RECOVERY TIME	Healthy children: 5-10 days; Adults: 10-14 days; complications can significantly prolong recovery

Urinary Schistosomiasis (Bilharzia)

Category: Infectious Disease

■ APPLICABLE TO	All Ages (Endemic areas: Sub-Saharan Africa, Middle East)

■ SYMPTOMS	Acute (cercarial dermatitis): itchy rash after wading in infected water; Katayama fever (6-8 weeks later): fever, cough, urticaria, eosinophilia; Chronic urinary schistosomiasis: blood in urine (terminal hematuria — blood at end of urination), frequency, pain, calcified bladder on X-ray, bladder cancer (long-term), hydronephrosis, kidney failure; Chronic intestinal: bloody diarrhea, hepatosplenomegaly, portal hypertension
■ MEDICATIONS	Praziquantel (single oral dose 40mg/kg — highly effective, safe, cheap — drug of choice); Repeat after 4-6 weeks in heavy infections; Mass drug administration programs in endemic areas; Treat complications (bladder cancer surveillance, portal hypertension management)
■ HOME REMEDIES	Prevention: avoid freshwater swimming, wading, or bathing in endemic areas; if unavoidable: DEET repellent (limited effectiveness against cercariae), towel dry vigorously immediately after exposure (cercariae can penetrate in seconds), swim in certified chlorinated pools only
■ DIET & NUTRITION	Adequate nutrition to support immune response, iron-rich foods for anemia (chronic blood loss), high protein diet for liver disease if hepatic schistosomiasis, adequate fluids
■■ PRECAUTIONS	Avoid all contact with freshwater lakes, rivers, and ponds in endemic areas, use safe tap water for bathing, WHO mass treatment programs, annual urine dipstick test for blood in endemic areas, bladder cancer surveillance in those with heavy long-term infection
■ WHEN TO GO TO HOSPITAL	Hematuria (blood in urine), fever after freshwater exposure, bladder ultrasound and urine tests for diagnosis, Praziquantel treatment, bladder cancer screening in chronic cases
■■ RECOVERY TIME	Acute infection: resolves with Praziquantel within weeks; established chronic infection: eggs are killed but calcification and structural damage may persist; bladder cancer risk persists with heavy infection history

Appendicitis

Category: Infectious Disease / Surgical

■ APPLICABLE TO	All Ages (Peak: 10-30 years)
■ SYMPTOMS	Classic presentation: periumbilical pain migrating to right lower quadrant (McBurney's point) over 12-24 hours; nausea and vomiting (after pain onset — distinguishes from gastroenteritis where vomiting comes first); loss of appetite; low-grade fever; rebound tenderness (worse removing hand than pressing); guarding; Rovsing's sign; atypical in young children, elderly, and pregnant women
■ MEDICATIONS	SURGICAL EMERGENCY: Appendicectomy (laparoscopic or open) — gold standard; IV antibiotics pre-and post-operatively (Cefuroxime + Metronidazole, or Piperacillin-Tazobactam); Non-operative management with IV antibiotics: appropriate in selected uncomplicated cases (growing evidence) — 70% success rate but 20-30% recurrence in 5 years

■ HOME REMEDIES	NO home treatment — suspected appendicitis is a surgical emergency. Do not give pain killers or laxatives and do not apply heat to abdomen (may accelerate perforation). Go to emergency department immediately
■ DIET & NUTRITION	Nil by mouth from time of suspected appendicitis (potential surgery); post-operatively: clear fluids then light diet as tolerated; full diet within 24-48 hours of uncomplicated laparoscopic appendicectomy
■■ PRECAUTIONS	Seek emergency care immediately for right lower quadrant pain with nausea and fever; do NOT delay; perforated appendicitis has much higher mortality and morbidity; atypical presentations in young children (diarrhea, generalized pain), elderly (minimal pain — can go unnoticed), and pregnant women (appendix displaced upward)
■ WHEN TO GO TO HOSPITAL	IMMEDIATELY for any suspected appendicitis — time to surgery before perforation (within 24 hours ideal) significantly improves outcomes
■■ RECOVERY TIME	Laparoscopic appendicectomy: discharge 24-48 hours, full recovery 1-2 weeks; open or perforated: 5-7 days hospital, 3-4 weeks recovery

Herpes Simplex (Cold Sores and Genital Herpes)

Category: Infectious Disease

■ APPLICABLE TO	All Ages
■ SYMPTOMS	HSV-1 (oral herpes/cold sores): tingling/burning at lip, then painful fluid-filled blisters on or around lips, crusting over 7-10 days; triggered by stress, sun, illness, menstruation. HSV-2 (genital herpes): painful blisters and ulcers on genitals, buttocks, thighs; flu-like symptoms in primary infection; recurrent attacks milder; asymptomatic shedding can transmit virus
■ MEDICATIONS	Antiviral medications: Acyclovir, Valacyclovir (Valtrex), Famciclovir; Treatment reduces duration and severity; Suppressive therapy (daily Valacyclovir) for frequent recurrences or partners; Topical Acyclovir cream (cold sores — moderately effective if started at tingling stage); IV Acyclovir for severe primary genital herpes or immunocompromised
■ HOME REMEDIES	Ice pack on cold sore (reduces pain and swelling), aloe vera gel topically, lemon balm (Melissa officinalis) cream/tea bags (antiviral properties, reduces healing time in studies), lysine topical or oral supplement, tea tree oil diluted, keep lesion clean and dry
■ DIET & NUTRITION	Lysine-rich foods (dairy, fish, chicken, legumes — inhibits herpes replication), limit arginine-rich foods during outbreaks (chocolate, nuts, protein shakes — arginine promotes viral replication), vitamin C and zinc for immune support, avoid triggers (alcohol, excessive sun)
■■ PRECAUTIONS	Avoid contact when lesions present (kissing, oral/sexual contact), condoms reduce but don't eliminate transmission (asymptomatic shedding), disclose to sexual partners, sunscreen on lips (prevents sun-triggered cold sores), avoid sharing lip products, razors, towels; neonatal herpes prevention (Caesarean section if active genital lesions at delivery)

■ WHEN TO GO TO HOSPITAL	Primary genital herpes (severe, extensive — may need IV antivirals), eye herpes (Herpes Keratitis — vision-threatening, needs urgent ophthalmology), herpes in newborn (emergency), herpes encephalitis (fever, confusion, seizures — EMERGENCY), herpes in immunocompromised
■■ RECOVERY TIME	Primary cold sore: 7-14 days; Recurrent: 3-7 days; With antiviral treatment: duration reduced by 1-2 days; suppressive therapy prevents 70-80% of recurrences

Digestive Disorders

10 Conditions Covered — 8 Guidance Fields Each

GERD (Gastroesophageal Reflux Disease)

Category: Digestive

■ APPLICABLE TO	Adults
■ SYMPTOMS	Heartburn (burning sensation in chest), acid regurgitation, difficulty swallowing, chest pain, chronic cough, hoarseness, sensation of lump in throat, worse after eating, lying down, bending
■ MEDICATIONS	PPIs (Omeprazole, Pantoprazole, Lansoprazole) - most effective; H2 blockers (Famotidine, Ranitidine); Antacids (Tums, Maalox) for quick relief; Prokinetics (Metoclopramide); Alginate (Gaviscon)
■ HOME REMEDIES	Elevate head of bed 6-8 inches, don't eat within 3 hours of bedtime, ginger tea, apple cider vinegar (diluted), aloe vera juice, chewing gum after meals (increases saliva), licorice root (DGL form)
■ DIET & NUTRITION	Avoid trigger foods (coffee, alcohol, chocolate, spicy/fatty foods, tomatoes, citrus, mint), eat smaller meals, lose weight if overweight, avoid carbonated drinks, eat slowly
■■ PRECAUTIONS	Weight loss (most effective lifestyle intervention), elevate head of bed, no smoking, avoid tight clothing, avoid eating before lying down, regular endoscopy if symptoms >5 years (Barrett's surveillance)
■ WHEN TO GO TO HOSPITAL	Difficulty swallowing, unexplained weight loss, vomiting blood, black stools, chest pain (rule out cardiac), symptoms not responding to PPIs, recurrent symptoms off medications
■■ RECOVERY TIME	Chronic condition requiring ongoing management; well-controlled with PPIs in most patients

Irritable Bowel Syndrome (IBS)

Category: Digestive

■ APPLICABLE TO	Adults
■ SYMPTOMS	Abdominal pain or cramping (relieved by defecation), altered bowel habits (constipation, diarrhea, or alternating), bloating, gas, mucus in stool; symptoms triggered by stress, food, hormones
■ MEDICATIONS	IBS-D (diarrhea): Loperamide, Rifaximin, Alosetron (women), Eluxadoline; IBS-C (constipation): Laxatives, Linaclotide, Lubiprostone; Antispasmodics (Dicyclomine, Hyoscine); Low-dose antidepressants; Psyllium husk
■ HOME REMEDIES	Peppermint oil capsules (enteric-coated, reduces spasm), probiotics, ginger tea, stress management, heat pad for cramps, yoga, regular exercise

■ DIET & NUTRITION	Low FODMAP diet (most evidence-based): avoid fermentable carbs (certain fruits, wheat, dairy, legumes, onion, garlic); reintroduce systematically; regular meal timing; adequate fiber (soluble preferred)
■■ PRECAUTIONS	Identify personal triggers (food diary), stress management crucial (brain-gut axis), rule out organic disease (colonoscopy if alarm features), psychological support (CBT effective)
■ WHEN TO GO TO HOSPITAL	Rectal bleeding, unexplained weight loss, fever, new symptoms over 50, nocturnal symptoms waking from sleep, family history of IBD or colon cancer, alarm features
■■ RECOVERY TIME	Chronic condition; symptom control achievable in 50-80% with appropriate treatment

Peptic Ulcer Disease

Category: Digestive

■ APPLICABLE TO	Adults
■ SYMPTOMS	Burning/gnawing epigastric pain (between meals, at night - duodenal; worse with eating - gastric), nausea, bloating, heartburn, loss of appetite; complications: bleeding (vomiting blood, black stools), perforation (sudden severe pain)
■ MEDICATIONS	H. pylori eradication (if positive): Triple therapy - PPI + Amoxicillin + Clarithromycin for 14 days OR Quadruple therapy; PPI (Omeprazole, Pantoprazole) 4-8 weeks; Avoid NSAIDs; Sucralfate
■ HOME REMEDIES	Cabbage juice (sulforaphane), probiotic yogurt, honey (especially Manuka - anti-H.pylori), ginger, avoid alcohol and smoking, stress reduction
■ DIET & NUTRITION	Avoid NSAIDs on empty stomach, avoid spicy/acidic foods during acute phase, small frequent meals, avoid alcohol and caffeine, probiotic-rich foods
■■ PRECAUTIONS	Test and treat H. pylori, avoid NSAIDs (use Acetaminophen instead), avoid alcohol and smoking, complete full antibiotic course for H. pylori, PPI protection if NSAIDs essential
■ WHEN TO GO TO HOSPITAL	Vomiting blood (hematemesis), black tarry stools (melena), sudden severe abdominal pain (perforation - EMERGENCY), significant weight loss, anemia
■■ RECOVERY TIME	Duodenal ulcer heals in 4-6 weeks; gastric ulcer 6-8 weeks; H. pylori eradication cures most cases

Liver Cirrhosis

Category: Digestive

■ APPLICABLE TO	Adults

■ SYMPTOMS	Fatigue, weakness, easy bruising/bleeding, jaundice, loss of appetite, nausea, weight loss/muscle wasting, ascites (abdominal fluid - distended abdomen), ankle swelling, confusion (hepatic encephalopathy), spider veins, enlarged breasts in men
■ MEDICATIONS	Diuretics (Furosemide + Spironolactone) for ascites, Propranolol (portal hypertension/varices prevention), Lactulose and Rifaximin (hepatic encephalopathy), Vitamin K, TIPSS procedure, Liver transplantation (definitive)
■ HOME REMEDIES	ABSOLUTE alcohol abstinence, adequate nutrition (calorie and protein), salt restriction (ascites management), avoid hepatotoxic medications, vaccinate against hepatitis A and B
■ DIET & NUTRITION	Adequate calories and protein (malnutrition common), sodium restriction (under 2g/day for ascites), small frequent meals, avoid raw shellfish (Vibrio infection risk), high protein (unless encephalopathy)
■■ PRECAUTIONS	Absolute alcohol abstinence is most important, regular gastroscopy (variceal screening), liver cancer surveillance (ultrasound + AFP every 6 months), all infections treated aggressively, transplant evaluation
■ WHEN TO GO TO HOSPITAL	Hematemesis (variceal bleeding - emergency), ascites needing drainage, encephalopathy, jaundice worsening, any infection, acute deterioration (acute-on-chronic liver failure)
■■ RECOVERY TIME	Irreversible damage; progression can be halted; compensated cirrhosis: median survival 12+ years; decompensated: much poorer without transplant

Inflammatory Bowel Disease (Crohn's and Colitis)

Category: Digestive

■ APPLICABLE TO	Adults (often diagnosed 15-35 years)
■ SYMPTOMS	Crohn's Disease: abdominal pain (often right lower quadrant), diarrhea (may be bloody), weight loss, fatigue, mouth ulcers, perianal disease (fistulas, abscesses); can affect any part of GI tract. Ulcerative Colitis: bloody diarrhea (hallmark), urgency, rectal bleeding, abdominal cramping, tenesmus (feeling of incomplete evacuation), confined to colon
■ MEDICATIONS	Aminosalicylates (Mesalazine/Asacol — UC first-line mild-moderate); Corticosteroids (Prednisolone — acute flares); Immunosuppressants (Azathioprine, 6-Mercaptopurine); Biologics (TNF inhibitors: Infliximab, Adalimumab; IL-12/23: Ustekinumab; Integrin inhibitors: Vedolizumab); Surgery (colectomy curative for UC; Crohn's — last resort)
■ HOME REMEDIES	Stress management (major trigger), identify food triggers (food diary), soluble fiber for mild UC, low-residue diet during flares, probiotics (VSL#3 has evidence for UC), omega-3 fatty acids, adequate sleep, smoking cessation (smoking worsens Crohn's but UC shows paradoxically protective effect with smoking — not recommended)

■ DIET & NUTRITION	Low-residue diet during flares, soluble fiber (oats, bananas, peeled fruits) between flares, well-cooked vegetables, avoid raw or hard-to-digest foods during flares, adequate protein and calories, vitamin D and calcium supplementation, B12 monitoring (Crohn's with terminal ileum involvement)
■■ PRECAUTIONS	Never stop medications without gastroenterologist guidance, regular monitoring for bowel cancer (colonoscopy surveillance after 8 years of colitis), osteoporosis prevention (steroids deplete calcium), routine vaccinations (flu, pneumonia) before biologics, IBD specialist team
■ WHEN TO GO TO HOSPITAL	Severe flare (more than 6 bloody stools per day, fever, rapid heart rate, abdominal rigidity), toxic megacolon (EMERGENCY), obstruction or perforation signs, significant rectal bleeding, suspected abscess or fistula
■■ RECOVERY TIME	Chronic relapsing remitting condition; 50% of patients have at least one relapse per year; biologics can achieve remission in 50-70%

Celiac Disease

Category: Digestive

■ APPLICABLE TO	All Ages
■ SYMPTOMS	Classic: chronic diarrhea, bloating, abdominal pain, weight loss, nutritional deficiencies (iron, B12, folate, calcium, Vitamin D); Atypical (increasingly recognized): constipation, anemia, osteoporosis, fatigue, dermatitis herpetiformis (itchy blistering skin rash), infertility, neurological symptoms; triggered by gluten ingestion
■ MEDICATIONS	Strict gluten-free diet (the ONLY treatment) — complete gluten elimination including hidden sources; treat nutritional deficiencies (Iron, B12, Folate, Calcium, Vitamin D supplementation); Dapsone for dermatitis herpetiformis; no medication treats celiac — diet is the medicine
■ HOME REMEDIES	Strict gluten-free lifestyle: dedicated gluten-free kitchen equipment, reading all food labels, aware of cross-contamination, connect with celiac support groups, gluten-free grains (rice, corn, quinoa, millet, sorghum, teff), naturally gluten-free whole foods
■ DIET & NUTRITION	STRICT gluten-free diet: eliminate ALL wheat, barley, rye and anything derived from them; safe grains: rice, corn, quinoa, millet, buckwheat, oats (certified GF — regular oats cross-contaminated); read labels carefully (hidden gluten in sauces, soups, condiments, medications, lipstick)
■■ PRECAUTIONS	Regular dietitian review, test for associated autoimmune conditions (thyroid, Type 1 diabetes), annual blood tests (tissue transglutaminase — TTG antibodies to confirm compliance), bone density scan (osteoporosis risk), family members should be screened (genetic condition)
■ WHEN TO GO TO HOSPITAL	Diagnosis confirmation (blood tests + endoscopic biopsy), nutritional deficiency management (severe anemia, osteoporosis), refractory celiac disease not responding to strict gluten-free diet, suspected complications

■■ RECOVERY TIME	Intestinal healing: 1-2 years of strict gluten-free diet; symptoms improve much faster (weeks); lifelong gluten-free diet required — even trace amounts cause damage

Hemorrhoids (Piles)

Category: Digestive

■ APPLICABLE TO	Adults (common in Pregnancy)
■ SYMPTOMS	Internal hemorrhoids: painless bright red rectal bleeding after defecation (blood on toilet paper or in bowl), prolapsing tissue; External hemorrhoids: painful swollen lump at anus, itching, burning, discomfort especially when sitting, possible thrombosis (very painful hard lump); constipation worsens symptoms
■ MEDICATIONS	Topical preparations: Hydrocortisone cream (Anusol HC), Lidocaine topical anesthetic, Zinc oxide, Witch hazel; Systemic: Diosmin + Hesperidin (Daflon) — reduces swelling; Stool softeners; Procedures: Rubber band ligation (most effective non-surgical), Sclerotherapy, Hemorrhoidectomy (large/prolapsed hemorrhoids)
■ HOME REMEDIES	Warm sitz baths 10-15 minutes 2-3 times daily and after bowel movements, cold ice pack for external thrombosis, witch hazel wipes, coconut oil (soothing), aloe vera gel, psyllium husk to soften stools, avoid straining, do not sit on toilet more than needed
■ DIET & NUTRITION	High fiber diet (25-35g per day): whole grains, legumes, fruits, vegetables to soften stools; plenty of water (8-10 glasses daily); avoid straining by keeping stools soft; avoid constipating foods; limit alcohol and spicy foods (irritate rectum); avoid prolonged sitting on toilet
■■ PRECAUTIONS	Do not strain during defecation, use toilet when urge comes (do not delay), high fiber and water intake prevents recurrence, exercise regularly, do not sit on toilet reading (increases straining), consider squatting position (Squatty Potty)
■ WHEN TO GO TO HOSPITAL	Significant rectal bleeding (always rule out colorectal cancer especially over 45 years old), severe thrombosed external hemorrhoid (excision needed within 72 hours), prolapsed hemorrhoid that cannot be pushed back, heavy bleeding with anemia
■■ RECOVERY TIME	With conservative management: improvement in 1-2 weeks; rubber band ligation: highly effective, recurrence less common; hemorrhoidectomy: 2-4 week recovery

Pancreatitis (Acute)

Category: Digestive

■ APPLICABLE TO	Adults

■ SYMPTOMS	Sudden severe upper abdominal pain (epigastric, may radiate to back — boring through to back is classic), nausea and vomiting, fever, rapid heart rate, abdominal tenderness, distension; most caused by gallstones (biliary) or alcohol; Ranson's criteria determine severity; severe pancreatitis: organ failure, necrosis, systemic inflammatory response
■ MEDICATIONS	Hospitalization essential; IV fluid resuscitation (aggressive hydration — cornerstone of treatment), adequate pain control (Morphine or Fentanyl IV, NSAIDs), nasojejunal feeding (early enteral nutrition superior to parenteral nutrition), antibiotics only if infected necrosis; ERCP for gallstone pancreatitis with cholangitis; cholecystectomy after recovery (if gallstones)
■ HOME REMEDIES	No home treatment for acute pancreatitis — requires hospitalization. After recovery: alcohol abstinence (most important), low-fat diet, small frequent meals, address gallstones to prevent recurrence
■ DIET & NUTRITION	Nil by mouth initially (hospital); early enteral feeding preferred over parenteral; after recovery: very low fat diet (under 30g fat/day), small frequent meals (6 small meals vs 3 large), no alcohol (lifelong if alcohol-related), avoid fatty/fried/rich foods, high carbohydrate foods, adequate protein
■■ PRECAUTIONS	Abstain from alcohol completely if alcohol-related pancreatitis (even one drink can trigger recurrence), cholecystectomy for gallstone pancreatitis (prevents recurrence in 70%+), small frequent low-fat meals, lipase enzyme supplements if pancreatic exocrine insufficiency develops
■ WHEN TO GO TO HOSPITAL	All cases of acute pancreatitis require hospitalization for IV fluids and monitoring; severe pancreatitis needs ICU level care; ANY severe upper abdominal pain with nausea requires urgent medical assessment
■■ RECOVERY TIME	Mild pancreatitis: 3-5 days hospital, full recovery 1-2 weeks; Severe pancreatitis: weeks to months; Chronic pancreatitis: lifelong management needed

Gallstones (Cholelithiasis)

Category: Digestive

■ APPLICABLE TO	Adults (Women 4x more than Men, Obese, over 40 — 4 Fs: Fat, Forty, Female, Fertile)
■ SYMPTOMS	Often asymptomatic (80% silent gallstones); Biliary colic: sudden severe right upper quadrant or epigastric pain radiating to right shoulder, lasting 1-5 hours, triggered by fatty meals, nausea and vomiting; Acute cholecystitis (infected gallbladder): persistent severe right upper quadrant pain, fever, Murphy's sign positive; Jaundice if stone blocks common bile duct
■ MEDICATIONS	Symptomatic gallstones: Cholecystectomy (laparoscopic — gold standard); Ursodeoxycholic acid (dissolves small cholesterol stones — only if unfit for surgery, takes years); Antispasmodics for biliary colic (Buscopan); Antibiotics for cholecystitis (Co-amoxiclav or Piperacillin-Tazobactam); ERCP for stones in bile duct; Adequate pain relief during colic

■ HOME REMEDIES	Avoid fatty meals (triggerPrevention triggers biliary colic), low-fat diet, olive oil and lemon juice (traditional gallbladder flush — NOT recommended by evidence, can cause stone migration and pancreatitis), maintain healthy weight, gradual weight loss (rapid loss increases gallstone formation risk)
■ DIET & NUTRITION	Low-fat diet (under 30% calories from fat), high fiber diet, regular meals (skipping meals increases gallstone risk — bile stagnates), avoid rapid weight loss, adequate water, fruits and vegetables, lean protein, reduce refined carbohydrates and sugar
■■ PRECAUTIONS	Symptomatic gallstones should be removed (cholecystectomy), low-fat diet to prevent colic attacks while awaiting surgery, weight loss should be gradual (not more than 1-2 lbs/week), Ursodeoxycholic acid prophylaxis during rapid weight loss (bariatric surgery patients)
■ WHEN TO GO TO HOSPITAL	Severe biliary colic not settling, fever with right upper quadrant pain (cholecystitis or cholangitis — URGENT), jaundice (bile duct stone), pale stools + dark urine + jaundice (obstructive jaundice — ERCP needed), pancreatitis
■■ RECOVERY TIME	Laparoscopic cholecystectomy: same-day or overnight; full return to activity in 1-2 weeks; risk of post-cholecystectomy syndrome (loose stools) in 10-15%

Diverticulitis

Category: Digestive

■ APPLICABLE TO	Adults (usually over 40)
■ SYMPTOMS	Left lower quadrant abdominal pain (most common — sigmoid colon most affected), fever, nausea and vomiting, change in bowel habits (constipation or diarrhea), tenderness on left side, rectal bleeding (in diverticular bleeding — painless heavy bleeding, different from diverticulitis); Diverticulosis (pouches without inflammation) is usually asymptomatic
■ MEDICATIONS	Uncomplicated mild diverticulitis (outpatient): Liquid diet + oral antibiotics (Co-amoxiclav or Metronidazole + Ciprofloxacin) for 7-10 days; Complicated or severe: Hospital admission, IV antibiotics, IV fluids; CT-guided drainage for abscess; Surgery (sigmoidectomy) for recurrent/complicated cases; High-fiber diet for prevention
■ HOME REMEDIES	Clear liquid diet during acute attack (rests bowel), warm compress on painful area, adequate rest, paracetamol for pain, gradual return to normal diet as pain resolves; prevention: high fiber diet, adequate hydration, regular exercise, avoid constipation
■ DIET & NUTRITION	During attack: clear liquids (broth, water, diluted juice), then low-fiber soft foods, then gradually increase fiber over weeks; Prevention: high-fiber diet (25-35g daily), adequate water (8-10 glasses), regular dietary fiber from fruits, vegetables, whole grains, legumes; avoid constipation
■■ PRECAUTIONS	High-fiber diet for long-term prevention, adequate hydration, regular exercise, maintain healthy weight, stop smoking, avoid NSAIDs (increase bleeding and perforation risk), recognize and report worrying symptoms early

■ WHEN TO GO TO HOSPITAL	Severe pain not improving in 48-72 hours, high fever, rigid abdomen (perforation — EMERGENCY), heavy rectal bleeding, unable to keep fluids down, new symptoms in immunocompromised or diabetic patients
■■ RECOVERY TIME	Mild uncomplicated: improvement in 2-3 days, complete recovery 1 week; 30% will have recurrence; sigmoid colon resection for recurrent or complicated disease

Kidney & Urological Conditions

4 Conditions Covered — 8 Guidance Fields Each

Kidney Stones (Nephrolithiasis)

Category: Kidney/Urological

■ APPLICABLE TO	Adults
■ SYMPTOMS	Sudden severe colicky pain (loin to groin - renal colic), pain radiating to lower abdomen and groin, bloody urine (hematuria), nausea/vomiting, frequent urination, painful urination, fever/chills if infected
■ MEDICATIONS	Pain relief: NSAIDs (Ketorolac - preferred), Opioids (severe pain), Alpha-blockers (Tamsulosin - helps stone passage), Hydration (IV fluids); Procedures: Ureteroscopy, ESWL (lithotripsy), PCNL (large stones); Prevention: based on stone type
■ HOME REMEDIES	Hydration (2-3L water daily), lemon juice (citrate prevents calcium oxalate stones), warm compress for pain, basil juice (kidney tonic), apple cider vinegar
■ DIET & NUTRITION	High fluid intake (most important), limit sodium (increases calcium excretion), moderate calcium intake from food (not supplements), limit oxalate foods (spinach, nuts, chocolate) for oxalate stones, limit animal protein
■■ PRECAUTIONS	Stay hydrated always (pale urine goal), 24-hour urine stone risk profile, strain urine to catch stone for analysis, follow stone type-specific prevention diet, treat underlying metabolic causes
■ WHEN TO GO TO HOSPITAL	Pain not controlled at home, fever with stone (infected kidney - emergency), only one kidney, inability to keep fluids down, stone larger than 6mm (less likely to pass spontaneously), complete urinary obstruction
■■ RECOVERY TIME	Small stones (less than 4mm) pass spontaneously in 1-4 weeks; larger stones need procedures

Chronic Kidney Disease

Category: Kidney/Urological

■ APPLICABLE TO	Adults (especially diabetic and hypertensive)
■ SYMPTOMS	Early: usually asymptomatic. Later: fatigue, swollen ankles/legs/feet, shortness of breath, nausea, vomiting, loss of appetite, itching, decreased urine output, confusion, pale skin (anemia), muscle cramps
■ MEDICATIONS	Treat underlying cause (BP control with ACE inhibitors/ARBs protective of kidneys), SGLT2 inhibitors (Empagliflozin - new evidence), Erythropoietin (anemia), Phosphate binders, Vitamin D analogs, Bicarbonate (acidosis), Dialysis or Transplant (end-stage)

■ HOME REMEDIES	Strictly follow kidney diet, blood pressure and sugar control, stay hydrated but not over-hydrated, exercise as tolerated, avoid nephrotoxic drugs (NSAIDs, contrast dye, aminoglycosides)
■ DIET & NUTRITION	KIDNEY DIET (crucial): limit protein, potassium (avoid bananas, oranges, potatoes in later stages), phosphorus (limit dairy, dark colas, nuts), sodium; adequate calories to prevent malnutrition - work with renal dietitian
■■ PRECAUTIONS	Regular nephrology follow-up and blood tests, medication review (dose adjustment needed for many drugs), avoid NSAIDs and herbal supplements, BP control, diabetes control, prepare for dialysis/transplant when approaching ESRD
■ WHEN TO GO TO HOSPITAL	Rapidly declining kidney function, severe electrolyte imbalances, volume overload (difficulty breathing), uremic symptoms (confusion, pericarditis), starting dialysis
■■ RECOVERY TIME	Progressive chronic condition; rate of progression slowed with optimal treatment; ESRD requires dialysis or transplant

Nephrotic Syndrome

Category: Kidney

■ APPLICABLE TO	Children and Adults
■ SYMPTOMS	Massive protein in urine (foamy urine), severe edema (especially puffy eyelids — periorbital — in children; ankle and leg swelling in adults), low blood protein (albumin), high cholesterol, weight gain from fluid retention, fatigue; children: puffy morning eyes is classic presentation
■ MEDICATIONS	Children (minimal change disease — most common cause): Prednisolone oral (60mg/m2/day initially, 80% go into remission); Adults: treat underlying cause; diuretics (Furosemide) for edema; ACE inhibitors/ARBs (reduce proteinuria); anticoagulation (hypercoagulable state); statins (dyslipidemia); immunosuppressants for resistant cases (Cyclosporine, Tacrolimus, Mycophenolate)
■ HOME REMEDIES	Strict salt restriction (reduces edema), moderate fluid restriction as advised, high protein diet may be needed (but worsening proteinuria if excess), blood pressure monitoring, elevation of edematous limbs
■ DIET & NUTRITION	Sodium restriction (under 1-2g/day for edema control), avoid salt substitutes if on ACE inhibitors, adequate protein (discuss with nephrologist — controversial), low saturated fat (manage dyslipidemia), small frequent meals if abdominal fluid present
■■ PRECAUTIONS	Vaccination before steroid therapy (pneumococcal, meningococcal, varicella if not immune — live vaccines CONTRAINDICATED on immunosuppression), infection vigilance (peritonitis risk — fever with abdominal pain in nephrotic child = emergency), thrombosis prevention, BP monitoring
■ WHEN TO GO TO HOSPITAL	Severe edema affecting breathing, fever with abdominal pain (peritonitis), sudden leg pain or chest pain (thrombosis), not responding to steroids in 4 weeks, diagnosis and biopsy

■■ RECOVERY TIME	Children with minimal change disease: 80% achieve complete remission with steroids; 50-80% relapse but usually steroid-responsive; adults: depends on underlying cause

Prostatitis

Category: Kidney / Urological / Men's Health

■ APPLICABLE TO	Men (All Ages)
■ SYMPTOMS	Acute bacterial: sudden fever, chills, severe pelvic or perineal pain, urinary symptoms (frequency, urgency, burning, difficulty urinating), systemically unwell; Chronic bacterial: recurrent UTIs, milder pelvic pain, urinary symptoms; Chronic Pelvic Pain Syndrome (CPPS, most common): pelvic, perineal, testicular pain 3+ months, urinary and sexual dysfunction, no bacterial infection
■ MEDICATIONS	Acute bacterial: IV or oral Ciprofloxacin or Trimethoprim-Sulfamethoxazole for 4-6 weeks (long course needed — poor antibiotic penetration into prostate); Chronic bacterial: 4-6 weeks oral antibiotics; CPPS: Alpha-blockers (Tamsulosin), NSAIDs, Quercetin supplement, pelvic floor physiotherapy, Pregabalin/Duloxetine for chronic pain, 5-alpha reductase inhibitors
■ HOME REMEDIES	Warm sitz baths (reduces pelvic muscle tension), avoid prolonged sitting (cycling, riding — pressures prostate), avoid constipation (bowel pressure worsens symptoms), pelvic floor exercises with physiotherapist (CPPS), stress management (CPPS heavily influenced by stress), quercetin supplements (evidence for CPPS)
■ DIET & NUTRITION	Anti-inflammatory diet, quercetin-rich foods (onions, apples, kale, berries — anti-inflammatory for prostate), avoid caffeine, alcohol, spicy foods (bladder irritants), lycopene-rich tomatoes, adequate hydration, zinc-rich foods (prostate health)
■■ PRECAUTIONS	Complete full antibiotic course for bacterial prostatitis (relapse risk if stopped early), avoid bicycle riding during acute phase, warm baths daily, pelvic floor physiotherapy for CPPS, regular follow-up, prostate massage (contraindicated in acute bacterial — can cause bacteremia)
■ WHEN TO GO TO HOSPITAL	Acute bacterial prostatitis with fever (may need IV antibiotics), inability to urinate (urinary retention — needs catheterization), prostatic abscess (intense pain, fluctuant mass — drainage needed), sepsis from prostatic source
■■ RECOVERY TIME	Acute bacterial: improvement in 3-5 days with antibiotics; complete course 4-6 weeks; CPPS: chronic condition — managed over months to years; improvement in 60-70% with multimodal treatment

Thyroid & Endocrine Disorders

6 Conditions Covered — 8 Guidance Fields Each

Hypothyroidism

Category: Endocrine

■ APPLICABLE TO	Adults (women 5-10x more common)
■ SYMPTOMS	Fatigue, weight gain, cold intolerance, constipation, dry skin, hair loss, puffy face, slowed heart rate, depression, impaired memory, muscle weakness and aches, heavy or irregular periods (women)
■ MEDICATIONS	Levothyroxine (T4 - first line, once daily on empty stomach), Liothyronine (T3 - sometimes combined), regular TSH monitoring to adjust dose
■ HOME REMEDIES	Selenium-rich foods (Brazil nuts - 1-2 per day), zinc, iodine (adequate intake), limit goitrogens in raw form (cruciferous vegetables - cooking reduces goitrogenic effect), ashwagandha, address stress
■ DIET & NUTRITION	Iodine-rich foods (iodized salt, seafood), selenium-rich foods (Brazil nuts, tuna), zinc-rich foods, avoid soy and raw cruciferous vegetables close to medication time (affect absorption)
■■ PRECAUTIONS	Take Levothyroxine consistently (same time, empty stomach, away from calcium, iron, antacids), regular TSH monitoring (every 6-12 months when stable), don't adjust dose without doctor guidance
■ WHEN TO GO TO HOSPITAL	Myxedema coma (severe: extreme cold, confusion, low heart rate - EMERGENCY), severe symptoms, dose adjustment, new symptoms, pregnancy (thyroid needs change significantly)
■■ RECOVERY TIME	Symptoms improve in 2-4 weeks; TSH normalizes in 4-8 weeks; lifelong replacement therapy

Hyperthyroidism / Graves Disease

Category: Endocrine

■ APPLICABLE TO	Adults (women more common)
■ SYMPTOMS	Weight loss despite increased appetite, rapid heartbeat, anxiety and irritability, heat intolerance, sweating, tremor, fatigue, muscle weakness, goiter (enlarged thyroid), protruding eyes (Graves), diarrhea, irregular periods
■ MEDICATIONS	Antithyroid drugs: Methimazole (PTU in pregnancy 1st trimester), Radioactive Iodine therapy (destroys thyroid - most common US treatment), Surgery (thyroidectomy), Beta-blockers (Propranolol for symptom control while awaiting definitive treatment)

■ HOME REMEDIES	Avoid iodine (seaweed, iodized salt - can worsen), avoid caffeine (increases palpitations), stress management, rest, lemon balm (Melissa officinalis - reduces thyroid activity mildly)
■ DIET & NUTRITION	Avoid high iodine foods (seaweed, kelp, shellfish), calcium and vitamin D rich foods (bone protection), adequate caloric intake (increased metabolism), cool foods and environment
■■ PRECAUTIONS	Thyroid storm (life-threatening thyrotoxicosis) possible with stress, infection, or surgery - know symptoms; eye protection for Graves ophthalmopathy; regular follow-up; after radioactive iodine/surgery - will need Levothyroxine
■ WHEN TO GO TO HOSPITAL	Thyroid storm symptoms (fever, extreme rapid heartbeat, delirium - EMERGENCY), Graves eye disease, uncontrolled symptoms, atrial fibrillation
■■ RECOVERY TIME	Antithyroid drugs: 12-18 months; Radioactive iodine: hypothyroidism in 6-12 months (needs Levothyroxine lifelong)

Addison's Disease (Adrenal Insufficiency)

Category: Endocrine

■ APPLICABLE TO	Adults
■ SYMPTOMS	Fatigue and weakness (progressive), weight loss, decreased appetite, hyperpigmentation of skin and mucous membranes (particularly in skin creases, recent scars — hallmark sign), low blood pressure, salt craving, dizziness on standing, nausea, vomiting, abdominal pain, depression; Addisonian crisis: severe vomiting, collapse, shock
■ MEDICATIONS	Hydrocortisone (cortisol replacement) 15-25mg daily in divided doses (larger dose morning, smaller evening mimicking natural rhythm); Fludrocortisone (mineralocorticoid replacement); STRESS DOSING: double or triple hydrocortisone dose during illness, injury, or surgery (essential — can be fatal if not done); injectable hydrocortisone kit for emergencies
■ HOME REMEDIES	Medical ID bracelet or card (MUST wear — essential in emergency), carry emergency hydrocortisone injection kit, extra salt in hot weather or with exercise (mineralocorticoid support), adequate rest, stress management, join Addison's disease support group
■ DIET & NUTRITION	Extra salt intake (especially in hot weather or exercise), adequate fluid intake, no special restrictions otherwise; adequate calcium and vitamin D (cortisol affects bone); avoid excessive licorice (false mineralocorticoid effect may mask inadequacy)
■■ PRECAUTIONS	NEVER stop steroid treatment (can be fatal), stress dosing rules must be known by patient AND family (fever, vomiting, diarrhea, surgery, injury = increase dose), medical alert ID bracelet, injectable emergency hydrocortisone for vomiting when cannot take oral
■ WHEN TO GO TO HOSPITAL	Addisonian crisis (severe vomiting, collapse, hypotension, confusion) — EMERGENCY: IV hydrocortisone 100mg stat and IV fluids; any illness preventing oral medication; major surgery (requires perioperative stress dosing in hospital)

■■ RECOVERY TIME	Lifelong condition requiring daily replacement therapy; with proper dosing, most people lead completely normal lives; major risk is adrenal crisis during illness — education prevents this

Goiter (Enlarged Thyroid)

Category: Endocrine

■ APPLICABLE TO	All Ages (Women more than Men)
■ SYMPTOMS	Visible or palpable swelling at front of neck, feeling of tightness in throat, difficulty swallowing large pieces of food, cough or hoarseness (if large goiter compresses trachea or recurrent laryngeal nerve), rarely difficulty breathing; may be associated with hyper- or hypothyroidism, or normal thyroid function (simple/endemic goiter)
■ MEDICATIONS	Iodine deficiency goiter: iodized salt and iodine supplementation (prevention and treatment); Hypothyroid goiter: Levothyroxine; Hyperthyroid goiter: antithyroid drugs, radioactive iodine, or surgery; Nodular goiter: thyroid function tests, ultrasound, FNA if suspicious nodule, surgery if compressive or malignant
■ HOME REMEDIES	Ensure adequate dietary iodine (iodized salt, seafood, dairy), avoid excessive consumption of raw goitrogenic vegetables (cabbage, broccoli, cauliflower, kale — cooking reduces goitrogenic compounds), selenium-rich foods (Brazil nuts, tuna), zinc foods
■ DIET & NUTRITION	Adequate iodine from iodized salt (cook with it), seafood, dairy products; selenium (Brazil nuts, tuna, sunflower seeds — required for thyroid hormone conversion); avoid excessive raw goitrogens; moderate soy intake
■■ PRECAUTIONS	Regular thyroid function tests and ultrasound monitoring, FNA (fine needle aspiration) biopsy for suspicious nodules, monitor for tracheal compression symptoms (difficulty breathing = urgent surgical evaluation), thyroid cancer awareness
■ WHEN TO GO TO HOSPITAL	Rapidly growing goiter, sudden painful swelling (hemorrhage into nodule), difficulty breathing, very hoarse voice, hard fixed nodule (cancer concern), thyroid function abnormalities, FNA for diagnosis
■■ RECOVERY TIME	Iodine-deficiency goiter: reduces with iodine supplementation in weeks to months; surgical removal: outpatient or 1-2 days; radioactive iodine: effect in 3-6 months

Cushing's Syndrome

Category: Endocrine

■ APPLICABLE TO	Adults (Women more than Men)

■ SYMPTOMS	Central obesity (moon face, buffalo hump — fat between shoulder blades, abdominal fat), purple stretch marks (striae) on abdomen and thighs, thin skin that bruises easily, muscle weakness (proximal — difficulty rising from chair), high blood pressure, high blood sugar, recurrent infections, osteoporosis, irregular periods (women), erectile dysfunction (men), mood changes, acne, excessive hair growth (women)
■ MEDICATIONS	Iatrogenic (steroid-induced — most common cause): gradual steroid taper under medical supervision; Cushing's Disease (pituitary adenoma): transsphenoidal surgery (primary treatment), Pasireotide, Cabergoline; Adrenal adenoma: adrenalectomy; Ectopic ACTH: treat primary tumor; Adrenal enzyme inhibitors (Ketoconazole, Metyrapone, Osilodrostat) if surgery not possible
■ HOME REMEDIES	Supportive: exercise as tolerated (improves metabolic complications), low-sodium diet, fall prevention (osteoporosis risk), skin protection (thin bruising skin), blood pressure monitoring, blood sugar monitoring
■ DIET & NUTRITION	Low sodium (hypertension and fluid retention control), adequate calcium and vitamin D (osteoporosis prevention), low glycemic index (blood sugar control), high protein (muscle wasting prevention), potassium-rich foods (low potassium common in Cushing's), moderate calorie restriction
■■ PRECAUTIONS	Never stop steroid medications abruptly (adrenal insufficiency), bone density monitoring, hypertension and diabetes management, wound care (poor healing), infection vigilance, psychological support (mood disorder very common), regular endocrinology follow-up
■ WHEN TO GO TO HOSPITAL	For diagnosis (24-hour urine cortisol, overnight dexamethasone suppression test), MRI pituitary, adrenal CT, pre- and post-surgical management, acute adrenal crisis if steroids suddenly stopped
■■ RECOVERY TIME	Post-curative surgery: months to 1-2 years for full recovery (adrenal suppression persists, needs hydrocortisone replacement temporarily); metabolic complications (hypertension, diabetes) often improve significantly after cure

Pituitary Disorders (Prolactinoma)

Category: Endocrine

■ APPLICABLE TO	Adults (Women more common)
■ SYMPTOMS	Women: irregular or absent periods (oligomenorrhea/amenorrhea), galactorrhea (milky nipple discharge without pregnancy or breastfeeding), infertility, low libido, vaginal dryness; Men: reduced libido, erectile dysfunction, infertility, gynecomastia; Both: headache, visual field defects (bitemporal hemianopia — tumor pressing on optic chiasm) in large tumors; osteoporosis from hypogonadism
■ MEDICATIONS	Dopamine agonists (shrink tumor and normalize prolactin — first-line even for large tumors): Cabergoline (twice weekly dosing, well-tolerated, highly effective — reduces tumor size in 80%); Bromocriptine (older, more side effects, but used in pregnancy); Surgery (transsphenoidal) for dopamine agonist resistance or intolerance; Radiation for residual tumor

■ HOME REMEDIES	Compliance with dopamine agonist therapy (critical — must be taken as prescribed), avoid medications that raise prolactin (antipsychotics, metoclopramide, domperidone, some antidepressants — discuss with prescribing doctor), stress management, adequate sleep
■ DIET & NUTRITION	Calcium and vitamin D for bone health (hypogonadism causes osteoporosis), balanced nutrition, no specific dietary restrictions, adequate protein
■■ PRECAUTIONS	Regular prolactin and MRI monitoring, visual field testing if large tumor, bone density DEXA scan, pregnancy planning discussion (Cabergoline stopped once pregnant, Bromocriptine safer in pregnancy), Cabergoline associated with cardiac valve changes at very high doses (rare with standard doses — echocardiogram monitoring in long-term high-dose use)
■ WHEN TO GO TO HOSPITAL	Sudden severe headache and visual loss (pituitary apoplexy — hemorrhage into pituitary — EMERGENCY), new visual field defects, infertility investigation, initial diagnosis, medication initiation and monitoring
■■ RECOVERY TIME	Cabergoline: prolactin normalizes in weeks; tumor shrinkage begins in months; 30-40% achieve tumor remission after 2 years treatment; medication may be reduced or stopped in some after years of normal MRI and prolactin